

THE 1,000 MEDICAL PROMPTS · VOLUME 2 OF 3

SEMINARS, JOURNAL CLUBS & PRESENTATIONS.

300 prompts to build seminars on any topic, appraise and present journal articles, design slides and posters, and deliver talks that hold a room.

300

prompts · #401–700

7

sections

35 · 173 · 92

Master · Standard · Quick

2.1

Seminar preparation

2.2

Journal club

2.3

Slide design & structure

2.4

Talks & oral presentations

2.5

Posters & visual teaching aids

2.6

Case presentations & grand rounds

2.7

Delivery, Q&A & teaching

Read this first.

These prompts are written the way a research supervisor thinks, not the way a chatbot answers. Many are long on purpose: a vague prompt gets you a vague, confident, wrong answer. Before you paste anything, five non-negotiables.

01 The AI does not know your patients.

Every prompt has a context block. Fill it honestly. If you say you have 200 cases a year when you have 40, the model will design a study you cannot finish.

02 Never accept a citation you have not opened.

Language models fabricate references that look flawless: real journal, real authors, wrong or non-existent paper. Verify every reference on PubMed by PMID or DOI before it enters your work.

03 Never paste identifiable patient data.

No names, hospital IDs, MRNs or scans with burned-in identifiers. De-identify before the data touches a chatbot. This is an ethics committee issue, not a preference.

04 The AI drafts. You decide.

Use it to speed up structure, phrasing and pre-checking. The scientific judgement, the assumptions and the interpretation stay yours. Your name goes on the paper, not the model's.

05 Declare AI use where required.

ICMJE and most journals require disclosure of generative AI use. AI cannot be an author. Check your journal's policy and your university's thesis rules before submission.

How to use each prompt

- › Copy the full prompt. The length is what makes the output specific.
- › Replace every [SQUARE BRACKET] with your real details. Options inside a bracket are separated by "/"; keep the one that applies.
- › Put anything you paste (abstracts, drafts, results) where the prompt shows [PASTE ...], above the task.
- › Use a model with web search or document upload switched on wherever the prompt asks for evidence.
- › Run each stage in a fresh chat so the model doesn't carry assumptions from earlier steps.
- › Push back. If the output is generic, reply: "Be more specific. Tie every point to my context."

Every prompt is built on **P-R-O-M-P-T**.

One formula, six layers. Each layer matches what OpenAI, Anthropic, Google and Microsoft recommend in their own prompting guides.

P Persona Who the AI should be: a senior expert with a trait	R Request One clear task, as numbered steps	O Output Shape, length, tone, tables	M Material Your context and anything you paste	P Parameters Rules, guidelines, accuracy guards	T Tweak Self-critique and the next step
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See it in action

BAD	"Tell me about pneumonia."
GOOD	"Explain community-acquired pneumonia management for an MBBS exam."
BETTER	"Act as a pulmonologist. Explain CAP management for an MBBS final-year student as a 6-point answer with empirical antibiotic choices per Indian guidelines. Keep it under 150 words, exam-oriented."

Three kinds of prompt

QUICK 40–80 words One focused output in a few sentences. Each has a TRY IT WITH example.	STANDARD 100–200 words One real task done properly: context, numbered task, rules, output, a pro tip and the next prompt to run.	MASTER 250–450 words A full workflow on its own page, with when to use it, why it exists and how to get more out of it.
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Tags you will see in the outputs

TAG	WHAT TO DO
[CITE] · [CITATION NEEDED]	Find a real source on PubMed and insert it yourself.
[VERIFY] · [ASSUMPTION — VERIFY FROM PUBLISHED DATA]	Replace the number with a sourced figure before use.
[UNVERIFIED BACKGROUND]	Treat as a lead to check, not a fact.
[ASK ME] · [NEED INFO] · [NOT REPORTED]	The model is missing information. Supply it, or leave it out.

Every prompt works in ChatGPT, Claude, Gemini and Copilot. → **NEXT** points to the prompt to run after this one.

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2.1

Seminar preparation

A seminar is judged by what the audience remembers the next morning. Build it around clear objectives, current evidence and a story, not a textbook chapter read aloud.

60

prompts

- #401 Mixed-audience seminar deck, clinically rigorous
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Master

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Standard

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Quick

- #440 Review my seminar slides like a moderator
- #441 Build a seminar from our department's cases
- #442 Collect feedback on my seminar
- #443 A seminar title that's accurate and interesting
- #444 A one-line opening hook
- #445 Five learning objectives in a minute
- #446 The definition slide
- #447 A useful mnemonic for my seminar
- #448 "Why this matters" slide
- #449 How many slides do I need?
- #450 Key numbers slide
- #451 Summary slide from my outline
- #452 Transition lines between sections
- #453 A question to wake up the audience
- #454 An analogy for a hard concept
- #455 Three questions for the discussion
- #456 A short history slide
- #457 Abbreviations slide for my seminar
- #458 Future directions slide
- #459 What moderators always ask on this topic
- #460 Turn my paragraph into slide bullets

Mixed-audience seminar deck, clinically rigorous

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT

When you have a departmental seminar on any topic and want a complete, evidence-based deck.

WHY

Departmental audiences are mixed: MBBS students, interns, residents and consultants in the same room. A deck pitched at resident level, with a short foundation recap for juniors and a controversies section for consultants, keeps everyone engaged, and cites real evidence rather than textbook generalities.

COPY THE PROMPT BELOW

PERSONA	You are a senior academic in [SPECIALTY] who has delivered and moderated hundreds of departmental seminars. You are clinically rigorous, evidence-based and practical, and you never invent citations or statistics.
MY CONTEXT	Topic: [e.g. Acute Respiratory Distress Syndrome: from pathophysiology to ventilator management] Audience: mixed (MBBS students, interns, junior residents, senior residents, consultants) Time available: [e.g. 45 minutes + 15 minutes discussion] Sources I have (guidelines, reviews, textbook chapters), uploaded or listed: [UPLOAD OR LIST] Output format: [slide-by-slide text for PowerPoint / a .pptx file if your tool can create one]
TASK	1. Pitch the core content at resident level, with a 1–2 slide foundation recap early, and advanced nuance for consultants. 2. Follow this structure, adapting as needed: title slide with a one-line clinical hook; why this matters (a representative case); foundation recap (definitions, classification); pathophysiology; clinical features and diagnostic approach (criteria or scores in tables); management, stepwise, first-line versus escalation; an evidence corner (landmark trials and current guidelines); controversies and grey zones; 3–5 key takeaways; references in Vancouver style. 3. For every slide give a message title, concise bullets (one idea per slide), a table where comparisons help, and speaker notes. 4. Mark what is exam-relevant for students and what is decision-changing for residents and consultants. 5. Suggest where a figure, flowchart or algorithm should go, and describe it.
RULES	Name trials and guidelines only from my sources or with [VERIFY] beside each, because misremembered trial names and years are the most embarrassing seminar error. Mark every drug dose as [VERIFY DOSE FROM CURRENT GUIDELINE]. Show me the slide-by-slide text for review before building any file.
OUTPUT	A slide table (number, title, bullets, visual, speaker notes, audience tag), then the reference list with [VERIFY] flags.
TWEAK	End with the 5 questions a consultant is most likely to ask after this seminar, and short evidence-based answers.

How to get more out of it

- › Tuning levers: change the depth (resident ↔ consultant), the slide count (about 1 slide per minute) and the visual style.
- › Verify every trial and guideline in the evidence corner against the original source before presenting.
- › Rehearse the controversies section. It is where the discussion will go.
- › → **NEXT #408** Learning objectives with Bloom's verbs

From topic to seminar blueprint in one sitting

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT

The day your seminar topic is assigned, before you open any textbook.

WHY

Most residents start a seminar by reading everything and end with 80 slides and no structure. Starting with objectives, a time budget and a storyline tells you exactly what to read and what to leave out.

COPY THE PROMPT BELOW

PERSONA	You are a medical educator and senior faculty member who mentors residents on seminar preparation. You think backwards from what the audience must be able to do after the session.
MY CONTEXT	Topic as assigned: [e.g. Approach to hyponatraemia] Specialty and my year: [e.g. MD Medicine, year 2] Audience and its level: [brief] Duration: [minutes] Days until the seminar: [N] Moderator's known expectations or style: [brief or "unknown"]
TASK	1. Clarify the scope: what is in, what is out, and how to phrase the final seminar title. 2. Write 4–6 learning objectives using Bloom's verbs (explain, differentiate, apply, choose). 3. Build a storyline: an opening case, 5–7 content blocks mapped to the objectives, and a closing return to the case. 4. Create a time budget per block that fits the duration. 5. For each block, list the must-read sources by type (current guideline, recent review, textbook chapter) with [VERIFY LATEST EDITION]. 6. Make a day-by-day preparation plan for the days I have, including a rehearsal. 7. Predict the moderator's 5 likely questions.
RULES	Keep the content proportional to the time, about one slide per minute, and cut anything not tied to an objective, because overlong seminars lose the audience in the middle. Name no specific papers from memory; describe the source type and mark it [FIND AND VERIFY].
OUTPUT	Scope note, objectives, storyline table (block, objective, minutes, key points), the source list, the preparation plan, then the predicted questions.
TWEAK	End with the single case vignette to open with, 60 words, de-identified and realistic for Indian practice.

How to get more out of it

- › Show the objectives and storyline to your moderator a week before. It avoids a last-minute restructure.
- › Read the guideline first, the review second and the textbook last.
- › Keep the opening case alive: return to it in every block.
- › → **NEXT #401** Mixed-audience seminar deck, clinically rigorous

Evidence pack: guidelines and trials, verified from my sources

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT

When your seminar needs an evidence corner and you have gathered the guidelines and key papers.

WHY

The evidence section is where seminars go wrong: trials cited with the wrong year, outcomes overstated, or guidelines quoted from memory. Building it strictly from uploaded sources, with quotes, makes it accurate and defensible when a consultant challenges it.

COPY THE PROMPT BELOW

PERSONA	You are an evidence-based medicine specialist who prepares evidence summaries for clinical guideline panels. You quote the source for every claim and state the certainty of evidence clearly.
MY CONTEXT	Seminar topic and the clinical questions it must answer: [list 3–5 questions] Guidelines uploaded (name, organisation, year): [UPLOAD / LIST] Key trials and meta-analyses uploaded: [UPLOAD / LIST] Setting to apply the evidence to: [e.g. Indian tertiary and district hospitals] Audience and how much time the evidence section gets: [e.g. residents and consultants, 8 minutes] My specialty and year: [brief]
TASK	1. For each clinical question, summarise what the uploaded guidelines recommend, with the strength or grade, quoting the recommendation wording. 2. Build a trials table: trial, year, design, population, intervention versus comparator, primary outcome and result with CI, and the key limitation. 3. Identify where guidelines disagree with each other, and why. 4. Identify where the evidence does not apply well to Indian settings (population, resources, drug availability). 5. Write 3 evidence-corner slides with speaker notes. 6. List any major trial or guideline the audience will expect that is missing from my uploads, marked [FIND AND UPLOAD], so I can add it before the seminar.
RULES	Use only the uploaded documents; for anything else write [NOT IN MY SOURCES — VERIFY], because a single wrong trial detail can discredit the whole seminar. Reproduce effect sizes exactly. State certainty levels as the guideline states them.
OUTPUT	A recommendations table per question, the trials table, the disagreement notes, the applicability notes, then the slide text.
TWEAK	End with the one piece of evidence I should read in full before presenting, because it is most likely to be questioned.

How to get more out of it

- › Upload the guideline PDFs themselves, not summaries of them.
- › Put the year on every trial and guideline on the slides. Consultants check it.
- › If two guidelines disagree, show both. That is where the best discussion happens.
- › → NEXT #421 Compare two or more guidelines

Full speaker script for a 45-minute seminar

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT

When your slides are ready and you want to rehearse to time.

WHY

Reading slides aloud is the commonest seminar failure. A script written for speaking, with timing, transitions and planned interaction, lets you rehearse properly, then speak from notes on the day.

COPY THE PROMPT BELOW

PERSONA	You are a speaking coach for medical educators who writes natural, spoken scripts and trims them to the second.
MY CONTEXT	My slides (titles and bullets, or upload the file): [PASTE OR UPLOAD] Duration of the talk (excluding discussion): [e.g. 40 minutes] Audience: [brief] My speaking style and my nerves: [brief] Points where I want audience interaction: [brief or "suggest"] Parts of the content I find hardest to explain: [list] Language mix the audience is comfortable with: [e.g. English with occasional Hindi]
TASK	1. Write a spoken script per slide at about 130 words per minute, in conversational academic English. 2. Mark timing checkpoints (e.g. "by slide 10: 12 minutes"). 3. Add a transition sentence between sections, so the story flows. 4. Insert 3–4 interaction moments: a question to the audience, a quick show of hands, or a case pause. 5. Mark the lines to memorise word for word (opening, key message, closing). 6. Flag slides whose script runs long, and suggest cuts. 7. For the hardest parts I listed, write a simpler explanation or an analogy I can use if the audience looks lost. 8. Write a 30-second emergency version of the conclusion in case time runs out.
RULES	Keep facts exactly as on my slides and add no new data, because the script must match what the audience sees. Script the interaction moments with a fallback answer in case nobody responds.
OUTPUT	A script table (slide, script, seconds, interaction), then the checkpoints and the memorise list.
TWEAK	End with a 5-step rehearsal plan for the next 3 days.

How to get more out of it

- › Rehearse aloud with a timer. Silent reading always underestimates the time.
- › Don't read the script on the day. Speak from the checkpoints and the memorised lines.
- › Record one rehearsal on your phone. You'll hear your filler words immediately.
- › → **NEXT #434** Feedback on my rehearsal script

Case-based interactive seminar

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT

When you want a seminar built around an unfolding case rather than a lecture.

WHY

Case-based seminars keep the audience thinking instead of copying. Revealing a case in stages, with questions at each stage, teaches clinical reasoning and makes the content stick.

COPY THE PROMPT BELOW

PERSONA	You are a clinician-educator who designs case-based learning sessions for residents, and who knows how to reveal information at the right pace.
MY CONTEXT	Topic: [TOPIC] Learning objectives: [PASTE OR "suggest"] Audience and number: [brief] Duration: [minutes] A real case I can adapt (de-identified), or "create a realistic one": [PASTE OR REQUEST] Setting where the case takes place: [emergency / ward / OPD / district hospital] Common mistakes residents make with this condition: [list or "suggest"]
TASK	1. Build an unfolding case in 4–6 stages: presentation, examination, initial investigations, key result, management decision, and follow-up or complication. 2. At each stage, write 1–2 questions for the audience, the expected answers, and the teaching point to deliver. 3. Map each stage to a learning objective. 4. Add a short mini-lecture slide (3 bullets) after each stage to consolidate. 5. Close with a summary of the reasoning path, and take-home points. 6. Suggest how to run it: small groups, polls or calling on people. 7. Build at least one stage around a common mistake, so the audience can discover why it is wrong. 8. Give the timing for each stage so the whole case fits the duration.
RULES	Keep the case realistic for Indian practice and fully de-identified. Mark drug doses and guideline specifics as [VERIFY], because case-based sessions invite detailed questions.
OUTPUT	A stage table (stage, information revealed, questions, expected answers, teaching point, objective), the mini-lecture slides, then the facilitation guide.
TWEAK	End with 2 variations of the case (a different age or comorbidity) for repeat sessions.

How to get more out of it

- › Reveal information on separate slides. Showing everything at once kills the reasoning.
- › Wait 10 seconds after asking a question. Silence is thinking time.
- › Keep a "parking lot" for tangent questions and answer them at the end.
- › → NEXT #435 Make my seminar interactive

Mock moderator: grill me before the real seminar

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT

2–3 days before your seminar, once the slides are final.

WHY

The dreaded part of a seminar is the moderator's questions. A realistic mock session, one question at a time with feedback, turns fear into preparation and shows you exactly which slides need strengthening.

COPY THE PROMPT BELOW

PERSONA	You are a senior professor and seminar moderator known for probing questions on basics, recent evidence and practical application. You are strict but fair, and you give frank feedback.
MY CONTEXT	Seminar topic and my specialty and year: [brief] My slide titles and key content: [PASTE] Areas I feel weakest in: [list] My moderator's known style or favourite themes: [brief or "unknown"] Duration of the seminar and the discussion slot: [brief]
TASK	1. Ask me one question at a time, mixing basic science, clinical application, recent evidence, Indian context and "what would you do" scenarios. 2. After each answer, score it out of 10, give the ideal answer in 3–5 points, and tell me which slide should cover it better. 3. Include 2 classic trap questions moderators use on this topic. 4. Mix difficulty: start with 3 basic questions, then 4 applied questions, then 3 advanced or evidence questions. 5. After 10 questions, summarise my weak areas and the slides to strengthen, and give me a one-day plan to fix them.
RULES	Ask only one question per turn and wait for my answer, because the value is in answering under pressure. Base questions on my topic and slides. Mark any fact in your ideal answers that depends on a guideline as [VERIFY].
OUTPUT	Conversational: one question per turn, feedback in a short structured block (score, ideal answer, gap, slide to fix), then the final summary with the weak areas ranked from most to least urgent.
TWEAK	End with the 3 questions I should prepare backup slides for.

How to get more out of it

- › Answer aloud before typing. The seminar is spoken.
- › Add backup slides at the end of the deck for the questions you struggled with.
- › Ask a colleague to play the moderator with these questions for a real rehearsal.
- › → **NEXT #404** Full speaker script for a 45-minute seminar

Turn a new guideline into a seminar

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT

When a new or updated clinical guideline is released and your department wants a session on it.

WHY

"What's new in the guideline" seminars are common and often done badly: a slide per recommendation, read aloud. The useful version shows what changed, why, what it means for practice here, and what remains uncertain.

COPY THE PROMPT BELOW

PERSONA	You are a guideline methodologist and clinician who explains guideline updates to busy clinicians, focusing on what changes practice.
MY CONTEXT	The guideline (upload the full text or the recommendations section): [UPLOAD] The previous version, if available: [UPLOAD OR "not available"] Audience and duration: [brief] Local practice for this condition in my hospital: [brief] Drugs, tests or devices the guideline recommends that my hospital lacks: [list or "unsure"] My specialty and year: [brief]
TASK	1. List the key recommendations with their strength and certainty, quoting the exact wording and the section number. 2. Identify what is new or changed since the previous version, and the evidence cited for each change. 3. Classify each change as practice-changing, practice-confirming or minor. 4. Discuss applicability to Indian settings: resources, drug availability, cost and staffing. 5. Build a 20–30 minute slide plan: what changed, why, what to do on Monday, and uncertainties. 6. Write a one-page summary handout. 7. Suggest a simple audit the department could run to check adherence to the key recommendation. 8. Note which recommendations are conditional or based on low-certainty evidence, so the audience doesn't treat them as absolute.
RULES	Quote only from the uploaded guideline and mark anything else [VERIFY], because a misquoted recommendation can change patient care. Don't overstate the certainty of weak recommendations.
OUTPUT	The recommendations table, the changes table (old position, new position, evidence, classification), the applicability notes, the slide plan with timings, then the handout.
TWEAK	End with the 3 recommendations most likely to be debated in the department, and why.

How to get more out of it

- › Lead with what changes practice. Nobody remembers the minor edits.
- › Bring the guideline's own algorithm figure if it has one, and credit it.
- › Invite a senior to comment on local applicability. It turns the seminar into a discussion.
- › → NEXT #420 The Indian perspective slide

408 Learning objectives with Bloom's verbs

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT At the very start of seminar preparation.

PERSONA	You are a medical educator trained in curriculum design who writes precise, measurable learning objectives.
MY CONTEXT	Topic: [TOPIC] Audience level: [MBBS / interns / residents / mixed] Duration: [minutes]
TASK	1. Write 4–6 learning objectives, each starting with a Bloom's verb (define, explain, differentiate, interpret, apply, choose, justify). 2. Spread them across levels: recall, understanding and application. 3. Say which part of the seminar will deliver each objective. 4. Suggest one quick end-of-session check for each objective.
RULES	Avoid vague verbs such as "know", "understand" and "learn about", because they cannot be checked. Keep the number realistic for the time available.
OUTPUT	A numbered objectives list, a mapping table (objective, seminar block, check question).

PRO TIP Put the objectives on slide 2 and return to them on the last slide. Moderators notice.

TRY IT WITH Topic: approach to a patient with haemoptysis, for MD Medicine residents, 40 minutes.

→ **NEXT #409** Minute-by-minute time plan

409 Minute-by-minute time plan

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT Once your outline is ready, to make sure it fits the time.

PERSONA	You are a meticulous academic event planner who keeps speakers on time.
MY CONTEXT	Outline or slide titles: [PASTE] Total time and the split between talk and discussion: [e.g. 45 + 15 minutes] Planned interactive moments: [list]
TASK	1. Allocate minutes to each section and slide. 2. Show a running clock with checkpoints every 10 minutes. 3. Identify sections to cut if I am running late, in order of priority. 4. Estimate the total slide count against the time (about one slide per minute).
RULES	Include time for interaction, transitions and technical hiccups, because seminars always run over their plan.
OUTPUT	A timing table (section, slides, minutes, clock), then the cut list.

PRO TIP Put the checkpoint times in small text in your speaker notes, not on the slides.

TRY IT WITH A 60-slide outline for a 40-minute seminar on diabetic ketoacidosis.

→ **NEXT #404** Full speaker script for a 45-minute seminar

410 Narrow a huge seminar topic to something teachable

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT When your assigned topic is enormous ("tuberculosis", "heart failure").

PERSONA	You are a seminar moderator who helps residents choose a sharp angle on broad topics.
MY CONTEXT	Assigned topic: [TOPIC] Duration and audience: [brief] Recent seminars in the department on related topics: [list or "unknown"]
TASK	1. Propose 5 focused angles (e.g. diagnosis in a specific group, a management controversy, recent advances, an approach to a symptom, an Indian perspective). 2. For each, write a working title and 3 learning objectives. 3. Recommend one angle for my audience and time, with the reason. 4. Suggest how to acknowledge the rest of the topic in 1 slide.
RULES	Keep each angle deliverable in the time available, because covering everything superficially teaches nothing.
OUTPUT	An angles table (title, focus, objectives), then the recommendation.

PRO TIP Confirm the narrowed title with your moderator before you start preparing.

TRY IT WITH Assigned topic: "Tuberculosis", 45 minutes, MD Respiratory Medicine residents.

→ **NEXT #402** From topic to seminar blueprint in one sitting

411 Pitch the same topic to different audiences

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT When you must present one topic to students, residents or consultants, or to a mix of them.

PERSONA	You are a medical educator who adapts content depth skilfully for different audiences.
MY CONTEXT	Topic and my current outline: [PASTE] Audience: [MBBS students / interns / residents / consultants / nurses / mixed]
TASK	1. Say what each audience already knows and what they need from this topic. 2. Adjust the outline: what to expand, what to compress, what to drop. 3. Suggest the level of jargon, the depth of evidence and the type of examples for each. 4. For a mixed audience, show how to layer content (foundation recap, core, advanced corner).
RULES	Keep the clinical accuracy the same at every level; only the depth and emphasis change, because simplifying must never mean being wrong.
OUTPUT	A comparison table by audience, then the adjusted outline.

PRO TIP For a mixed audience, tell the room explicitly: "This slide is for students; this one is for the consultants."

TRY IT WITH Topic: interpretation of pulmonary function tests.

→ **NEXT #401** Mixed-audience seminar deck, clinically rigorous

412 Pathophysiology as a visual story

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / MBBS STUDENT

USE IT When your seminar needs a pathophysiology section that doesn't bore the room.

PERSONA	You are a physiology-minded clinician who explains mechanisms visually and links every step to a clinical feature.
MY CONTEXT	Condition: [CONDITION] Audience level: [brief] Slides available for this section: [e.g. 3] Sources I am using: [UPLOAD / LIST]
TASK	1. Break the pathophysiology into a sequence of 5–8 steps from trigger to clinical picture. 2. Link each step to a symptom, sign, investigation finding or treatment target. 3. Describe a flowchart or diagram for each slide. 4. Write speaker notes explaining each arrow.
RULES	Keep to established mechanisms from my sources, and label emerging or debated mechanisms as such, because contested mechanisms presented as fact invite correction.
OUTPUT	A step table (step, mechanism, clinical correlate, treatment target), diagram descriptions, then speaker notes.

PRO TIP If a pathophysiology step doesn't explain a clinical feature or a treatment, cut it from the seminar.

TRY IT WITH Condition: septic shock, 3 slides, for residents.

→ **NEXT #414** Diagnostic approach as an algorithm

413 Turn a classification into a clear table

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / MBBS STUDENT

USE IT When your topic has a classification, staging or grading system.

PERSONA	You are a medical educator who turns classification systems into clean, memorable tables.
MY CONTEXT	Classification or staging system: [name, or "the one used in guideline X"] My source for it: [UPLOAD / CITE] Audience: [brief]
TASK	1. Present the classification as a table with categories, defining criteria and clinical significance. 2. Note the year and version, and any older versions still used in papers. 3. Suggest a memory aid if it helps. 4. Write one slide of speaker notes on how the classification guides management.
RULES	Reproduce thresholds and criteria only from my source, and mark any detail I did not supply as [VERIFY FROM SOURCE], because classification cut-offs are commonly misremembered.
OUTPUT	The table, the version note, the memory aid, then the speaker notes.

PRO TIP Always show which version you are using. Moderators love asking about the previous one.

TRY IT WITH GOLD grouping for COPD assessment, from the current GOLD report.

→ **NEXT #401** Mixed-audience seminar deck, clinically rigorous

414 Diagnostic approach as an algorithm

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT When your seminar covers "approach to" a symptom, sign or abnormal result.

PERSONA	You are a clinician–teacher who builds practical, stepwise diagnostic algorithms.
MY CONTEXT	Presenting problem: [e.g. hyponatraemia, pancytopenia, pleural effusion] Setting: [ward / emergency / OPD; resource level] Sources: [UPLOAD / LIST]
TASK	1. Build a stepwise algorithm: initial assessment, first-line tests, branch points with decision criteria, and further tests. 2. Mark "can't-miss" diagnoses and red flags at the right points. 3. Describe the algorithm as a flowchart (boxes and arrows) for a slide. 4. Add a resource-limited variant for district hospitals. 5. Write speaker notes walking through one example patient.
RULES	Mark cut-off values and test choices that depend on a guideline as [VERIFY FROM SOURCE], because algorithms are often photographed and reused.
OUTPUT	The flowchart description, a branch table (decision point, criterion, next step), the resource-limited variant, then the notes.

PRO TIP Walk the audience through the algorithm with a real case. Algorithms alone are hard to follow.

TRY IT WITH Approach to a unilateral pleural effusion, tertiary and district versions.

→ **NEXT #415** Management section, stepwise and practical

415 Management section, stepwise and practical

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT For the management part of a seminar.

PERSONA	You are a senior clinician who teaches management as clear steps, from first-line care to escalation.
MY CONTEXT	Condition and severity range: [brief] Setting and resources: [brief] Guidelines or sources: [UPLOAD / LIST]
TASK	1. Lay out management in steps: immediate measures, first-line treatment, monitoring, escalation criteria and escalation options, and when to refer. 2. Present drug choices in a table with dose placeholders, key cautions and monitoring. 3. Highlight differences between tertiary and resource-limited settings. 4. Add the common mistakes in managing this condition.
RULES	Mark every dose, duration and threshold as [VERIFY DOSE FROM CURRENT GUIDELINE], because dosing errors on slides can be copied into practice.
OUTPUT	A stepwise outline, the drug table, the setting comparison, then the common mistakes.

PRO TIP A one-slide "management on a page" summary is often the most photographed slide of the seminar.

TRY IT WITH Management of acute severe asthma in adults, tertiary and district settings.

→ **NEXT #416** Drug comparison table for a seminar

416 Drug comparison table for a seminar

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / MBBS STUDENT

USE IT When your topic involves several drugs or drug classes.

PERSONA	You are a clinical pharmacologist who builds accurate, exam-useful drug tables.
MY CONTEXT	Drugs or classes to compare: [list] Aspects I want: [mechanism, indications, dose, adverse effects, interactions, monitoring, cost, availability in India] Source: [UPLOAD / LIST]
TASK	1. Build a comparison table with the aspects I listed. 2. Highlight the distinguishing features between similar drugs. 3. Add a row for special populations (pregnancy, renal and hepatic impairment, elderly). 4. Write speaker notes on how to choose between them.
RULES	Mark every dose as [VERIFY DOSE] and every availability or cost note as [VERIFY LOCALLY], because drug details change and vary by region.
OUTPUT	The drug table, then the choice guidance in notes.

PRO TIP Split big tables across two slides. A 10-column table is unreadable from the back row.

TRY IT WITH Compare DOACs with warfarin for stroke prevention in atrial fibrillation.

→ **NEXT #417** Landmark trials in one table

417 Landmark trials in one table

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT For the "evidence corner" of your seminar, once you have the trial papers or abstracts.

PERSONA	You are an evidence-based medicine teacher who summarises trials accurately and critically.
MY CONTEXT	Trials to include (upload the papers or paste the abstracts): [UPLOAD / PASTE] The clinical question they address: [one line]
TASK	1. Build a table: trial, year, design, population, intervention versus comparator, primary outcome and result, and key limitation. 2. Write a one-line "what it changed" for each trial. 3. Put the trials in a timeline showing how the evidence evolved. 4. Write speaker notes for a 3-minute evidence walkthrough.
RULES	Use only the uploaded or pasted trial details, with numbers reproduced exactly, and mark missing details [NOT IN SOURCE], because trial details are what experts in the audience check.
OUTPUT	The trials table, the timeline, then the speaker notes.

PRO TIP One slide with 4–6 trials is enough. Pick the ones that changed practice.

TRY IT WITH Trials of corticosteroids in septic shock.

→ **NEXT #418** Controversies and grey zones

418 Controversies and grey zones

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT For the section of your seminar that keeps consultants engaged.

PERSONA	You are a senior clinician who presents controversies fairly, giving both sides their strongest evidence.
MY CONTEXT	Topic: [TOPIC] Controversial questions I am aware of: [list or "suggest"] Sources for each side: [UPLOAD / LIST]
TASK	1. Identify 3–4 genuine controversies in current practice. 2. For each, present the arguments and evidence on both sides. 3. State what current guidelines say, if anything. 4. Suggest how the department might approach it, and what evidence is awaited. 5. Write one slide per controversy with a debate question.
RULES	Present both sides with equal care, and support claims only from my sources or mark them [VERIFY], because a one-sided "controversy" slide will be challenged.
OUTPUT	A controversy table (question, side A, side B, guideline position, awaited evidence), then the slides.

PRO TIP End each controversy slide with a question to the room. It starts the discussion you want.

TRY IT WITH Controversies in fluid resuscitation for sepsis.

→ **NEXT #419** Recent advances slide set

419 Recent advances slide set

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT When your seminar needs a "what's new" section.

PERSONA	You are an academic clinician who tracks new evidence and separates genuine advances from hype.
MY CONTEXT	Topic: [TOPIC] Time window: [e.g. last 3 years] New papers, guidelines or approvals I have found: [UPLOAD / LIST]
TASK	1. Categorise the advances: new diagnostics, new therapies, new guideline positions, new understanding of the disease. 2. For each, give the evidence level and whether it is ready for practice, promising, or early research. 3. Note whether each is available or relevant in India. 4. Write 2–3 slides with speaker notes.
RULES	Include only advances from the sources I gave, or mark them [VERIFY — FIND SOURCE], because AI-listed "advances" are often outdated or invented.
OUTPUT	An advances table (advance, category, evidence level, readiness, India relevance), then the slides.

PRO TIP Use a search tool with live citations (Perplexity or Consensus) to find recent advances, then verify each paper yourself.

TRY IT WITH Recent advances in the management of idiopathic pulmonary fibrosis.

→ **NEXT #420** The Indian perspective slide

420 The Indian perspective slide

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT For the section of your seminar on Indian epidemiology, guidelines and practice.

PERSONA	You are a public-health physician who knows Indian data sources, national programmes and practice realities.
MY CONTEXT	Topic: [TOPIC] Indian data and guidelines I have found (with sources): [LIST]
TASK	1. Summarise the Indian burden using my sourced figures. 2. Summarise relevant Indian guidelines or national programme recommendations from my sources. 3. Highlight how Indian practice differs from Western guidelines: resources, drug availability, cost and disease patterns. 4. Identify gaps in Indian data. 5. Write 1–2 slides with speaker notes.
RULES	Use only figures and guidelines I listed, with their sources and years, and mark anything else [VERIFY], because Indian statistics are often quoted from outdated surveys.
OUTPUT	The summary, the differences table, the data gaps, then the slides.

PRO TIP Always give the survey name and year with a prevalence figure on the slide.

TRY IT WITH Topic: COPD in India, with burden data from national surveys and the Global Burden of Disease study.

→ **NEXT #421** Compare two or more guidelines

421 Compare two or more guidelines

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT When different societies recommend different things on your topic.

PERSONA	You are a guideline analyst who compares recommendations clearly and explains why they differ.
MY CONTEXT	Guidelines to compare (upload or list with year): [UPLOAD / LIST] Clinical questions to compare on: [list]
TASK	1. Build a side-by-side table: question, guideline A recommendation (with strength), guideline B recommendation (with strength). 2. Explain the reasons for differences: evidence interpretation, population, cost, or date of publication. 3. Suggest which is more applicable to my setting, and why. 4. Write one comparison slide with speaker notes.
RULES	Quote recommendations from the uploaded documents and mark anything else [VERIFY], because paraphrased recommendations often lose their conditions.
OUTPUT	The comparison table, the reasons, the applicability note, then the slide.

PRO TIP Put the publication year of each guideline in the table header. It often explains the differences.

TRY IT WITH Compare two international hypertension guidelines on blood-pressure targets in older adults.

→ **NEXT #417** Landmark trials in one table

422 References slide and citation check

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT When finalising your seminar deck.

PERSONA	You are a reference editor who formats seminar references in Vancouver style.
MY CONTEXT	Sources I used, as I have them: [PASTE LIST] Number of reference slides allowed: [e.g. 1–2]
TASK	1. Format each reference in Vancouver style. 2. Flag references missing essential details, without inventing them. 3. Suggest how to cite sources on individual slides (small superscript numbers). 4. Select the 10–15 most important if the list is too long.
RULES	Write [MISSING — CHECK PUBMED] for missing details instead of inventing them, because invented volume or page numbers make real references look fake.
OUTPUT	The formatted list, then the flagged items and citation tips.

PRO TIP Cite figures and tables you adapt from papers on the slide itself: "Adapted from [ref]".

TRY IT WITH 18 sources, including 2 guidelines, 6 trials, 3 reviews and a textbook.

→ **NEXT #424** Take-home messages that stick

423 End-of-seminar quiz

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT To close your seminar with active recall and check the objectives.

PERSONA	You are a medical educator who writes high-quality MCQs that test application, not trivia.
MY CONTEXT	Seminar topic and learning objectives: [PASTE] Audience level: [brief] Format: [show of hands / online poll / paper]
TASK	1. Write 5–8 MCQs mapped to the objectives, mostly clinical-vignette style. 2. For each, give the answer and a 2-line explanation. 3. Include one question on a common misconception. 4. Suggest how to run the quiz in 5 minutes.
RULES	Keep every correct answer consistent with my seminar content and mark guideline-dependent answers [VERIFY], because a wrong quiz answer undermines the whole session.
OUTPUT	The numbered questions with options, then the answer key with explanations.

PRO TIP An online poll (Mentimeter, Google Forms or Slido) turns a quiz into the most engaging 5 minutes of the seminar.

TRY IT WITH Seminar on approach to acute kidney injury, for interns.

→ **NEXT #424** Take-home messages that stick

424 Take-home messages that stick

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT For the final slide of any seminar.

PERSONA	You are a medical educator who writes memorable, accurate take-home messages.
MY CONTEXT	Seminar content summary or slide titles: [PASTE] Learning objectives: [PASTE] Audience: [brief]
TASK	1. Write 3–5 take-home messages, each one sentence, specific, actionable and in plain language. 2. Map each message to a learning objective. 3. Suggest a visual layout for the slide. 4. Suggest the closing sentence to say aloud, and a follow-up resource to point people to.
RULES	Each message must be supported by the seminar content, with no new claims, because the last slide is the one people photograph.
OUTPUT	The messages, the mapping, the layout, then the closing line.

PRO TIP "Check magnesium in every hypokalaemic patient" is a take-home message. "Electrolytes are important" is not.

TRY IT WITH Seminar on management of diabetic foot infections.

→ **NEXT #425** One-page handout for the audience

425 One-page handout for the audience

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT When you want the audience to leave with a summary.

PERSONA	You are a medical educator who designs concise, printable one-page summaries.
MY CONTEXT	Seminar content: [PASTE SLIDES OR SUMMARY] Audience: [brief] Format: [printed A4 / PDF shared on WhatsApp] My name and department for the footer: [brief]
TASK	1. Create a one-page handout: title, 3–5 key points, one table or algorithm, key doses or thresholds, and 3 references. 2. Keep it under 350 words. 3. Suggest a layout with boxes or columns.
RULES	Mark doses and thresholds as [VERIFY] until I confirm them, and keep all content consistent with the slides, because handouts are kept and reused long after the seminar.
OUTPUT	The handout text with its layout plan.

PRO TIP Put your name and the date on the handout. It becomes part of your teaching portfolio.

TRY IT WITH Handout for a seminar on oxygen therapy in the ward.

→ **NEXT #424** Take-home messages that stick

426 Opening clinical vignette

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT To open any seminar with a case that grabs attention.

PERSONA	You are a clinician-storyteller who opens teaching sessions with vivid, realistic cases.
MY CONTEXT	Topic and the key teaching points: [brief] Audience: [brief] A real case to adapt (de-identified), if any: [PASTE OR "create one"]
TASK	1. Write a 60–90 word vignette, realistic for Indian practice, that raises the central question of the seminar. 2. End with one question to the audience. 3. Write 2 alternative vignettes with a different twist. 4. Say how to return to the vignette at the end.
RULES	Fully de-identify real cases, and keep the details clinically plausible, because an unrealistic case loses a clinical audience.
OUTPUT	The main vignette with its question, then the alternatives and the closing plan.

PRO TIP Present the vignette in the present tense. It creates urgency.

TRY IT WITH Topic: pulmonary embolism in pregnancy.

→ **NEXT #405** Case-based interactive seminar

427 Basic science seminar with clinical hooks

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / MBBS STUDENT

USE IT For anatomy, physiology, pharmacology, pathology or microbiology seminars.

PERSONA	You are a basic-science teacher who makes every concept clinically relevant.
MY CONTEXT	Topic: [e.g. brachial plexus, renal tubular physiology, beta-lactam pharmacology] Audience and exam relevance: [e.g. MD residents; university exam] Duration: [minutes]
TASK	1. Outline the seminar: core concepts, diagrams, and clinical correlations for each concept. 2. Suggest diagrams to draw or show, described clearly. 3. Add "clinical hook" slides linking each concept to a patient scenario. 4. Add 5 viva-style questions examiners ask on this topic.
RULES	Keep to standard textbook facts and mark anything uncertain [VERIFY FROM TEXTBOOK], because basic-science errors are easy for examiners to spot.
OUTPUT	The outline with concept-to-clinical links, diagram descriptions, then the viva questions.

PRO TIP Draw the key diagram live on a board if you can. It teaches better than any slide.

TRY IT WITH Topic: brachial plexus anatomy and lesions, for MS Orthopaedics residents.

→ **NEXT #408** Learning objectives with Bloom's verbs

428 Seminar on a procedure or surgery

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT (SURGICAL AND PROCEDURAL SPECIALTIES)

USE IT When your seminar covers a procedure: indications, technique and complications.

PERSONA	You are a senior surgeon and teacher who explains procedures step by step with safety in mind.
MY CONTEXT	Procedure: [PROCEDURE] Audience: [brief] Sources and videos available: [LIST]
TASK	1. Structure the seminar: indications and contraindications, preoperative preparation, relevant anatomy, step-by-step technique, intraoperative pitfalls, complications and their management, and outcomes. 2. Suggest images or video clips for each step. 3. Add a safety checklist slide. 4. Add 5 questions examiners ask about this procedure.
RULES	Keep technique descriptions to standard practice from my sources, and mark outcome figures as [VERIFY], because procedural details vary between techniques and centres.
OUTPUT	The structured outline with visuals, then the checklist and questions.

PRO TIP Show a short, edited video clip of the key step. Nothing teaches a procedure better.

TRY IT WITH Procedure: laparoscopic cholecystectomy, critical view of safety.

→ **NEXT #401** Mixed-audience seminar deck, clinically rigorous

429 Teaching session on interpreting a test

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT For sessions on ECG, ABG, PFT, chest X-ray, CSF or other interpretations.

PERSONA	You are a clinician-teacher famous for turning test interpretation into a simple, systematic method.
MY CONTEXT	Test: [e.g. arterial blood gas] Audience level: [brief] Duration: [minutes] Example tracings or reports I have (de-identified): [UPLOAD / DESCRIBE]
TASK	1. Teach a stepwise interpretation method in 5–7 steps. 2. Build 5–6 practice examples of graded difficulty, with worked answers. 3. Include common pitfalls and mimics. 4. Design a 5-minute practice round at the end.
RULES	Ensure every worked example is internally consistent (e.g. ABG values that are physiologically possible), and mark any formula or normal range to confirm as [VERIFY], because inconsistent examples confuse learners.
OUTPUT	The method, the examples with answers, the pitfalls, then the practice round.

PRO TIP Give the method as a mnemonic and use it on every example. Repetition builds the habit.

TRY IT WITH Test: arterial blood gas interpretation for interns, 30 minutes.

→ **NEXT #423** End-of-seminar quiz

430 Image-based seminar

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT When your seminar is built around images: X-rays, CT, MRI, ultrasound or histopathology.

PERSONA	You are a radiologist-educator who teaches systematic image reading.
MY CONTEXT	Topic: [e.g. interstitial lung disease on HRCT] Images available (de-identified, described): [LIST] Audience: [brief]
TASK	1. Teach a systematic reading approach for this modality and topic. 2. Arrange the images from classic to difficult, with key findings to point out. 3. Write the findings and differential for each image. 4. Add "spot diagnosis" slides for audience practice.
RULES	Describe findings conservatively and mark any diagnosis that needs confirmation as [CONFIRM WITH REPORT], and make sure every image is fully de-identified, because images with patient details can't be shown.
OUTPUT	The approach, an image-by-image table (findings, differential, teaching point), then the spot-diagnosis slides.

PRO TIP Show each image twice: first unmarked for the audience to look, then annotated.

TRY IT WITH HRCT patterns in interstitial lung disease, 12 images.

→ **NEXT #423** End-of-seminar quiz

431 Turn a textbook chapter into a seminar

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT When your main source is a textbook chapter and you need to transform it into a seminar.

PERSONA	You are a medical educator who turns dense chapters into focused, engaging seminars.
MY CONTEXT	The chapter (upload or paste key sections): [UPLOAD] Time available and audience: [brief]
TASK	1. Extract the 6–8 most important concepts for my audience. 2. Identify what to leave out. 3. Convert the chosen content into a slide outline with message titles. 4. Add what the chapter lacks: recent evidence, Indian context and controversies, as slots marked [ADD FROM CURRENT SOURCE].
RULES	Use only the chapter for the core content, and flag anything that may be outdated as [CHECK CURRENT GUIDELINE], because textbooks lag behind the evidence.
OUTPUT	The concepts list, the omissions, the slide outline, then the gaps to fill.

PRO TIP Check the textbook's edition year. Anything older than 3–4 years needs a guideline check.

TRY IT WITH A chapter on acute pancreatitis, for a 40-minute seminar.

→ **NEXT #419** Recent advances slide set

432 Prepare a seminar with NotebookLM

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT When you have 5–15 source documents and want to study and outline them quickly.

PERSONA	You are a research-tools trainer who uses NotebookLM for grounded seminar preparation.
MY CONTEXT	Topic: [TOPIC] Sources uploaded to NotebookLM: [list] Seminar structure planned: [PASTE OUTLINE]
TASK	1. Write 12 questions to ask NotebookLM, one or two per section of my outline. 2. Write a customisation instruction for an Audio Overview that focuses on my learning objectives. 3. Explain how to check each answer using NotebookLM's citations. 4. Suggest how to turn the answers into slide bullets.
RULES	Phrase every question so the answer must come from my sources, because NotebookLM is reliable only when grounded in them.
OUTPUT	The question list by section, the audio instruction, then the verification and conversion steps.

PRO TIP Listen to the Audio Overview on your commute the day before. It's surprisingly good revision.

TRY IT WITH 8 sources on heart failure with preserved ejection fraction.

→ **NEXT #401** Mixed-audience seminar deck, clinically rigorous

433 What have I missed?

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT When your slides are drafted and you want a completeness check.

PERSONA	You are a senior moderator who spots missing topics in seminars instantly.
MY CONTEXT	Topic and objectives: [PASTE] My slide titles and bullets: [PASTE] Audience: [brief]
TASK	1. Compare my content against what a complete seminar on this topic should cover for this audience. 2. List the important gaps and suggest where each should go. 3. List content that is excessive or off-topic. 4. Check that each objective is covered.
RULES	Prioritise gaps by importance for the audience, not completeness for its own sake, because adding everything makes seminars overlong.
OUTPUT	A gaps table (gap, importance, where to add), then the excess list and the objective check.

PRO TIP Moderators usually ask about exactly the gap you skipped. Close the important ones.

TRY IT WITH Slides for a seminar on management of upper gastrointestinal bleeding.

→ **NEXT #406** Mock moderator: grill me before the real seminar

434 Feedback on my rehearsal script

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT After a rehearsal, to improve delivery and content.

PERSONA	You are a presentation coach for medical educators.
MY CONTEXT	My script or rehearsal transcript: [PASTE] Timing achieved versus allowed: [e.g. 52 minutes versus 40 allowed] Feedback I've already received: [brief]
TASK	1. Identify sections that run long, repeat themselves or wander. 2. Identify jargon and sentences that are hard to follow when spoken. 3. Identify missing transitions and weak openings or closings. 4. Suggest specific cuts to hit the time, with the time saved by each.
RULES	Keep the key clinical content and all facts, and cut delivery padding first, because timing problems are usually words, not content.
OUTPUT	An issue table (section, problem, fix, time saved), then a revised opening and closing.

PRO TIP If you're 20% over time, cut whole slides. Speaking faster never works.

TRY IT WITH A 5,800-word rehearsal transcript for a 40-minute seminar.

→ **NEXT #409** Minute-by-minute time plan

435 Make my seminar interactive

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT When you want the audience thinking, not just listening.

PERSONA	You are an active-learning specialist in medical education.
MY CONTEXT	Seminar outline: [PASTE] Audience size and setting: [e.g. 30 people, seminar room with a projector] Tools available: [poll apps, whiteboard, none]
TASK	1. Suggest 4–6 interaction points placed across the seminar: polls, think-pair-share, case pauses, quick quizzes, and hands-up votes. 2. For each, write the exact question and the expected responses. 3. Estimate the time each takes. 4. Suggest how to handle no response, or a wrong majority answer.
RULES	Keep interaction purposeful and tied to an objective, and within the time budget, because interaction for its own sake wastes minutes.
OUTPUT	An interaction table (point, method, question, minutes), then the handling tips.

PRO TIP Asking the audience to commit to an answer before you reveal it is the single most powerful interaction.

TRY IT WITH Seminar on antibiotic stewardship for residents, 30 attendees.

→ **NEXT #423** End-of-seminar quiz

436 Check the story and flow of my outline

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT When your outline feels like a list of facts rather than a story.

PERSONA	You are a medical storyteller who structures talks for narrative flow.
MY CONTEXT	My outline or slide titles: [PASTE] Opening case, if any: [PASTE] Audience and duration: [brief]
TASK	1. Summarise the narrative the outline currently tells. 2. Identify jumps, repetitions and dead ends. 3. Propose a reordered flow with transition sentences. 4. Show where to return to the opening case. 5. Suggest one moment of tension or surprise to keep attention in the middle of the talk.
RULES	Rearrange and connect my content rather than adding new content, because the flow problem is order, not substance.
OUTPUT	The current narrative, the problems, then the new order with transitions.

PRO TIP Read your slide titles in order. If they don't tell the story, the seminar won't either.

TRY IT WITH A 35-slide outline on chronic kidney disease that jumps between staging, complications and diet.

→ **NEXT #401** Mixed-audience seminar deck, clinically rigorous

437 Adapt my seminar for nurses and allied staff

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT When teaching nurses, paramedics, physiotherapists or technicians.

PERSONA	You are a clinical educator who teaches multidisciplinary teams practically and respectfully.
MY CONTEXT	Original seminar content: [PASTE] Audience role and experience: [e.g. ICU staff nurses] What they need to do differently after the session: [brief]
TASK	1. Refocus the content on their role: recognition, monitoring, escalation, procedures and safety. 2. Remove content outside their scope and simplify the mechanisms. 3. Add practical checklists and escalation triggers. 4. Suggest a short skills demonstration if relevant.
RULES	Keep all clinical facts accurate and within their scope of practice, and mark local protocols as [CHECK HOSPITAL PROTOCOL], because nurses act on this content directly.
OUTPUT	The adapted outline, the checklists, then the demonstration idea.

PRO TIP Ask the nurses what they struggle with before you prepare. It makes the session land.

TRY IT WITH Seminar on sepsis recognition adapted for ward nurses.

→ **NEXT #411** Pitch the same topic to different audiences

438 Turn a seminar into a CME talk for practitioners

STANDARD

2.1 SEMINAR PREPARATION · FACULTY / SENIOR RESIDENT

USE IT When invited to speak to general practitioners or doctors in private practice.

PERSONA	You are a CME speaker known for practical, clinic-ready talks.
MY CONTEXT	Original seminar content: [PASTE] Audience: [e.g. general practitioners in a district town] Duration: [minutes]
TASK	1. Refocus the content on what practitioners see and decide: recognition, first-line management, when to refer, and follow-up. 2. Replace academic detail with practical pearls and red flags. 3. Add 2–3 short case scenarios from primary care. 4. Write a one-page practice summary for them.
RULES	Keep recommendations consistent with current guidelines and mark specifics [VERIFY], and avoid promoting any product, because CME content must be accurate and unbiased.
OUTPUT	The adapted outline, the cases, then the summary.

PRO TIP Ask the organisers what the audience's common referral problems are. Build the talk around them.

TRY IT WITH Seminar on thyroid disorders adapted for general practitioners.

→ **NEXT #425** One-page handout for the audience

439 Pro-con debate seminar

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT When the department wants a debate on a controversial topic.

PERSONA	You are an academic debate moderator in medicine.
MY CONTEXT	Motion: [e.g. "All patients with sepsis should receive early vasopressors"] My side: [pro / con] Time per speaker and format: [brief] Sources: [UPLOAD / LIST]
TASK	1. Build my strongest case: 3–4 arguments, each with the best evidence from my sources. 2. Anticipate the opponent's strongest arguments and prepare rebuttals. 3. Write an opening statement and a closing statement. 4. Suggest 2 slides per argument.
RULES	Use honest evidence even on my side, and acknowledge the limitations of my sources, because misrepresenting evidence loses a medical audience's trust.
OUTPUT	The arguments table, the rebuttals, the statements, then the slide plan.

PRO TIP Concede a minor point early. It makes your main arguments more credible.

TRY IT WITH Motion: "High-flow nasal oxygen should replace NIV for acute hypercapnic respiratory failure." Con side.

→ **NEXT #418** Controversies and grey zones

440 Review my seminar slides like a moderator

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT

USE IT When your deck is complete, 3–4 days before the seminar.

PERSONA	You are a senior faculty moderator who reviews residents' seminar decks for accuracy, clarity and design.
MY CONTEXT	My slides (paste the text or upload the file): [PASTE OR UPLOAD] Topic, audience and duration: [brief] Sources I used: [list]
TASK	1. Check medical accuracy slide by slide, flagging any statement that needs verification or seems outdated. 2. Check clarity: overloaded slides, vague titles, missing labels and unreadable tables. 3. Check structure against the objectives and the time. 4. Give the 10 most important fixes, in priority order, with the slide number.
RULES	Flag doubtful facts as [VERIFY] rather than correcting them from memory, because I must confirm them against my sources. Quote the slide text for every comment.
OUTPUT	A slide-by-slide comment table, then the top 10 fixes.

PRO TIP Fix the accuracy flags first, then clarity, then design. Moderators forgive plain slides, not wrong ones.

TRY IT WITH A 42-slide deck on management of chronic liver disease complications.

→ **NEXT #406** Mock moderator: grill me before the real seminar

441 Build a seminar from our department's cases

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT When you want the seminar anchored in real cases your department has managed.

PERSONA	You are a clinician-educator who turns departmental case experience into teaching.
MY CONTEXT	Topic: [TOPIC] 3–6 de-identified case summaries from our department: [PASTE] Audience and duration: [brief]
TASK	1. Identify the teaching themes the cases illustrate: presentation spectrum, diagnostic pitfalls, management decisions and outcomes. 2. Build the seminar structure around these themes, using each case as an illustration. 3. Link each case to what the guidelines or literature say, with [CITE] slots. 4. Draw 3–5 local lessons ("in our unit, we should...").
RULES	Keep the cases fully de-identified, and present outcomes honestly including errors, without blaming individuals, because a blame-free tone keeps the discussion open.
OUTPUT	A theme-to-case map, the seminar outline, then the local lessons.

PRO TIP Ask the treating consultants to comment briefly during the seminar. Their perspective adds depth.

TRY IT WITH Five cases of dengue with warning signs managed in our medicine ward this season.

→ **NEXT #405** Case-based interactive seminar

442 Collect feedback on my seminar

STANDARD

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

USE IT To improve your teaching and build a portfolio.

PERSONA	You are a medical-education evaluator who designs short, useful feedback forms.
MY CONTEXT	Seminar topic and objectives: [PASTE] Audience: [brief] Format: [Google Form / paper]
TASK	1. Design a feedback form with 6–8 items: objectives met, clarity, pace, relevance, slides, interaction, and two open questions. 2. Include one question that checks learning, not just satisfaction. 3. Explain how to summarise the responses and use them next time. 4. Suggest how to include the summary in a teaching portfolio.
RULES	Keep the form short enough to finish in 2 minutes and make it anonymous, because long or named forms give poor and biased feedback.
OUTPUT	The form items with rating scales, then the summary method and portfolio tip.

PRO TIP Share the form link on the last slide as a QR code. Response rates jump.

TRY IT WITH Seminar on antibiotic stewardship for 25 residents.

→ **NEXT #434** Feedback on my rehearsal script

QUICK PROMPTS · 2.1

443 A seminar title that's accurate and interesting

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a seminar moderator in [SPECIALTY]. Write 5 titles for my seminar on [TOPIC] for [AUDIENCE]: two descriptive, two question-style and one with a clinical hook. Each title should be under 12 words, accurate to the content and free of clichés. Mark your favourite.

TRY IT WITH Topic: approach to a patient with fever and thrombocytopenia.

444 A one-line opening hook

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a medical storyteller. Write 3 opening lines (under 25 words each) for my seminar on [TOPIC]: one clinical scenario, one striking fact with a [VERIFY] slot for the number, and one provocative question for the audience. Keep each realistic for Indian practice.

TRY IT WITH Topic: snakebite envenomation in rural India.

445 Five learning objectives in a minute

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a medical educator trained in curriculum design. Write 5 measurable learning objectives for a [DURATION]-minute seminar on [TOPIC] for [AUDIENCE], each starting with a Bloom's verb and each achievable in the time. Avoid "know" and "understand", and mark which objectives test application rather than recall.

TRY IT WITH 30-minute seminar on hyperkalaemia for interns.

446 The definition slide

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT / MBBS STUDENT

Act as a clinical academic. Write the definition slide for [CONDITION]: the current accepted definition with its source organisation and year marked [VERIFY], one line on how it changed from older definitions, and one line on why the definition matters clinically.

TRY IT WITH Condition: acute respiratory distress syndrome.

447 A useful mnemonic for my seminar

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT / MBBS STUDENT

Act as a medical educator known for memorable teaching. Create a mnemonic for [LIST TO REMEMBER] in my seminar on [TOPIC]. Check that every letter maps to an accurate, relevant item without forcing awkward words, and give a one-line explanation for each letter.

TRY IT WITH Causes of a high anion gap metabolic acidosis.

448 "Why this matters" slide

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a seminar coach. Write a "why this matters" slide for my seminar on [TOPIC]: 3 bullets covering burden (with a [VERIFY] slot for numbers), the clinical consequence of getting it wrong, and relevance to my audience [AUDIENCE] in India.

TRY IT WITH Topic: early warning scores in the general ward.

449 How many slides do I need?

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a presentation coach. For a [DURATION]-minute seminar on [TOPIC] with [NUMBER] planned interaction points, recommend the number of content slides, the backup slides, and the split between sections, and tell me which section usually becomes bloated and how to keep it in check.

TRY IT WITH 45-minute seminar with 3 polls on heart failure management.

450 Key numbers slide

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a clinical teacher. List the 6–8 key numbers a practising clinician must know about [TOPIC] (thresholds, targets, doses, time windows), each marked [VERIFY FROM CURRENT GUIDELINE], with a one-line explanation of why each number matters at the bedside.

TRY IT WITH Topic: management of diabetic ketoacidosis.

451 Summary slide from my outline

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a medical educator. From my seminar outline, [PASTE OUTLINE], write a summary slide of 5 bullets of under 15 words each that recap the key points in the order I taught them, and link the last bullet to the opening case.

TRY IT WITH Outline of a seminar on community-acquired pneumonia.

452 Transition lines between sections

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a speaking coach. Write one natural spoken transition sentence between each pair of sections in my seminar, [LIST SECTIONS IN ORDER], so that each transition links what was just covered to what comes next and sounds natural when spoken aloud.

TRY IT WITH Sections: definition → pathophysiology → diagnosis → management → controversies.

453 A question to wake up the audience

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

Act as an interactive teaching expert. Write 3 questions I can ask the audience midway through my seminar on [TOPIC]: one quick poll, one "what would you do" case question and one common-misconception check, each with its expected answer and a one-line follow-up comment.

TRY IT WITH Topic: blood transfusion thresholds in the ICU.

454 An analogy for a hard concept

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT / MBBS STUDENT

Act as a gifted medical teacher known for clear explanations. Explain [DIFFICULT CONCEPT] with 2 different everyday analogies for [AUDIENCE], each in 2–3 sentences, then state exactly where each analogy breaks down, so that students don't take it too far.

TRY IT WITH Concept: ventilation-perfusion mismatch.

455 Three questions for the discussion

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

Act as a seminar moderator. After a seminar on [TOPIC], suggest 3 open discussion questions that invite the audience to share experience or debate practice, one of them specific to Indian settings, each under 25 words, and say what a good answer would reveal.

TRY IT WITH Topic: antibiotic prescribing in upper respiratory infections.

456 A short history slide

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a medical historian. Give 5 milestones in the history of [TOPIC] for a single slide, each with the year and the person or group, and mark every date and name [VERIFY], because historical facts are often misattributed in slides.

TRY IT WITH Topic: history of insulin therapy.

457 Abbreviations slide for my seminar

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a copy-editor for medical slides. From my slide text, [PASTE TEXT], list every abbreviation with its expansion, flag any used without definition, and suggest which ones to spell out on the slides instead because the audience won't know them.

TRY IT WITH Slides on critical care nephrology full of AKI, CRRT, SLED and KDIGO.

458 Future directions slide

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT / FACULTY

Act as a research-minded clinician. Write a "future directions" slide for my seminar on [TOPIC]: 4 bullets naming specific areas of ongoing research, new drugs or tests in trials, and unanswered questions, each marked [VERIFY], without hype, and one line on when results are expected.

TRY IT WITH Topic: gene therapy in haemophilia.

459 What moderators always ask on this topic

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a senior professor who moderates seminars. List the 8 questions most commonly asked by moderators on [TOPIC] at the [AUDIENCE LEVEL] level, from basic to advanced, with a one-line model answer for each and [VERIFY] beside anything guideline-dependent.

TRY IT WITH Topic: anaemia in chronic kidney disease, MD Medicine level.

460 Turn my paragraph into slide bullets

QUICK

2.1 SEMINAR PREPARATION · PG RESIDENT

Act as a slide designer. Convert this paragraph, [PASTE PARAGRAPH], into a slide with a message-style title and no more than 3 bullets of under 12 words each, keeping every fact accurate and moving the extra detail into speaker notes.

TRY IT WITH A 120-word paragraph on the mechanism of action of SGLT2 inhibitors.

2.2

Journal club

A journal club is not a summary of a paper. It is a critical appraisal that ends with one question: should this change what we do for our patients?

60

prompts

7

Master

35

Standard

18

Quick

#461 Journal club presentation: appraisal to bottom line

#462 Choose the right paper for journal club

#463 Deep appraisal of a randomised controlled trial

#464 Appraise a cohort or case-control study

#465 Appraise a diagnostic accuracy study

#466 Appraise a systematic review or meta-analysis

#467 Decode the statistics in my paper

#468 Five-line PICO summary of the paper

#469 Does the abstract match the paper?

#470 "Why was this study done?" slide

#471 Methods on two slides

#472 Key results, clearly presented

#473 Explain a forest plot to the audience

#474 Explain the survival curves

#475 Absolute effects: ARR, RRR, NNT and NNH

#476 Can we trust these results?

#477 Does this apply to our patients?

#478 Hunt for the hidden bias

#479 Confounding in an observational paper

#480 Are these the right outcomes?

#481 Appraise a non-inferiority trial

#482 Should we believe the subgroup finding?

#483 Detect spin in the discussion and conclusion

#484 Funding and conflicts: do they matter here?

#485 Compare the paper with its editorial and letters

#486 How does this fit with previous evidence?

#487 The clinical bottom line

#488 Questions to drive the discussion

#489 The faculty questions I'll face

#490 One-page appraisal handout

#491 Fill the CASP checklist for my paper

#492 Certainty of evidence using GRADE

#493 Appraise a qualitative study

#494 Appraise an economic evaluation

#495 Appraise a prognostic or prediction-model study

#496 Appraise a clinical guideline (AGREE II)

#497 Check the trial registry for outcome switching

#498 Read a paper properly in 30 minutes

#499 Explain the paper to MBBS students

#500 Quiz questions from the journal club

#501 Turn my critique into a letter to the editor

#502 Moderating a journal club well

#503 This study's design in one line

#504 Find the real primary outcome

#505 Quick ARR and NNT

#506 What does this hazard ratio mean?

#507 Intention-to-treat or per-protocol?

#508 Was allocation really concealed?

#509 Who was blinded, and does it matter?

#510 Is this loss to follow-up a problem?

#511 The whole paper in one sentence

#512 Was this study adequately powered?

#513 Is this outcome a surrogate?

#514 Would this change my practice?

#515 Explain a methods term from my paper

#516 Does the title overclaim?

#517 Relative versus absolute risk for the audience

#518 Explain the key figure in three lines

#519 A sharp question to ask the presenter

#520 Summarise the paper for colleagues

Journal club presentation: appraisal to bottom line

2.2 JOURNAL CLUB · PG RESIDENT

USE IT

The week before you present, once your paper is chosen and you have the full text.

WHY

Most journal clubs retell the abstract. A strong one summarises the question, appraises it with a validated checklist, states the real strengths and limitations, and ends with a clear clinical bottom line for your patients. That is what faculty remember.

COPY THE PROMPT BELOW

PERSONA	You are a clinical epidemiologist who has moderated departmental journal clubs for 15 years and is known for spotting the flaw everyone else missed. You teach residents to appraise, not summarise.
MY CONTEXT	My level and specialty: [e.g. MD Medicine, year 2] Audience: [residents and faculty / mixed] Time for presentation and discussion: [e.g. 20 + 15 minutes] Checklist my department prefers: [CASP / JBI / CONSORT-based / none] [PASTE FULL TEXT OF THE PAPER OR UPLOAD THE PDF]
TASK	1. Summarise the study in 5 lines: question (PICO), design, setting, sample, and main result with numbers. 2. Explain why the study was done: the gap it addressed and the prior evidence, as described by the authors. 3. Appraise it with the appropriate checklist for its design, item by item, quoting the paper for each judgement. 4. Identify the 3 genuine strengths and the 3 limitations that most threaten the conclusion, with the likely direction of bias. 5. Interpret the results in clinical terms: absolute risk difference, NNT or NNH where applicable, and the precision of the estimates. 6. Judge applicability to Indian patients and our hospital's resources. 7. State the clinical bottom line in 2 sentences: would this change our practice, and why or why not? 8. Build a 12–15 slide outline with message titles, bullets and one-line speaker notes.
RULES	Use only the paper I provided; where a detail is not reported, write [NOT REPORTED] rather than assuming, because the audience will check the paper. Calculate NNT only from numbers in the paper and show the arithmetic. Keep judgement and fact clearly separate.
OUTPUT	The 5-line summary, the appraisal table (item, judgement, quote), the strengths and limitations, the clinical interpretation, the bottom line, then the slide outline.
TWEAK	End with the 3 questions faculty are most likely to ask me, with short answers grounded in the paper.

How to get more out of it

- › Read the methods section twice. The flaw is almost always there, not in the discussion.
- › Check the trial registry entry. Compare the registered primary outcome with the reported one.
- › Practise the bottom line aloud. It is the sentence the head of department will remember.
- › → NEXT #489 The faculty questions I'll face

Choose the right paper for journal club

2.2 JOURNAL CLUB · PG RESIDENT

USE IT

2–3 weeks before your journal club, when you need to pick a paper.

WHY

The paper decides the journal club. A recent, practice-relevant paper with a clear design and a genuine debate makes a great session. An obscure or poorly designed paper makes for a long hour. This prompt helps you choose deliberately.

COPY THE PROMPT BELOW

PERSONA	You are a senior faculty member who coordinates the department's journal club and knows which papers generate good learning and discussion.
MY CONTEXT	Specialty and my year: [brief] Recent journal clubs in the department (so I don't repeat): [list] Areas of practice that are debated in our unit: [list] Candidate papers I have found (title, journal, year, design; paste abstracts if possible): [PASTE 3–6 CANDIDATES] Departmental rules (e.g. published within the last 12 months, major journals only): [brief]
TASK	1. Score each candidate on: relevance to our practice, recency, design quality, clarity of the question, potential for discussion, and teaching value for appraisal skills. 2. Flag any candidate that is a secondary analysis, a small pilot, or has an obvious fatal flaw that would make the session one-sided. 3. Recommend one paper and a backup, with reasons. 4. Suggest what to read alongside it: the accompanying editorial, the protocol, the trial registration and key prior studies (described by type, marked [FIND]). 5. Suggest the central debate question the journal club should resolve. 6. Explain how to confirm the paper's status (published version, corrections, retractions).
RULES	Judge only from the abstracts and details I pasted, and don't claim facts about papers I didn't provide, because a wrong claim about a paper's findings will surface during the session. Prefer papers whose full text I can access legally.
OUTPUT	A scoring table (paper, criterion scores, total, flags), the recommendation with a backup, the companion-reading list, then the debate question.
TWEAK	End with a one-line pitch I can send to the coordinator to get the paper approved.

How to get more out of it

- › High-impact trials that challenge current practice make the best journal clubs.
- › Check whether the paper has an accompanying editorial. It is a free second opinion.
- › Avoid papers you can't obtain in full text. You can't appraise an abstract.
- › → NEXT #461 Journal club presentation: appraisal to bottom line

Deep appraisal of a randomised controlled trial

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT

When your journal-club paper is an RCT and you want to go beyond the checklist.

WHY

RCTs look robust, but their flaws are subtle: inadequate concealment, unblinded outcome assessment, outcome switching, post-hoc subgroups and inappropriate analysis populations. A structured deep appraisal finds what a quick read misses.

COPY THE PROMPT BELOW

PERSONA	You are a trial methodologist who appraises RCTs for guideline panels. You apply the Cochrane RoB 2 domains and the CONSORT checklist, and you explain statistics in clinical language.
MY CONTEXT	The trial (upload the full text, and supplementary material if available): [UPLOAD] Trial registration number and protocol, if available: [PASTE OR "not found"] Clinical question I care about: [one line] My comfort level with statistics: [beginner / intermediate / advanced]
TASK	1. Randomisation and allocation concealment: how they were done, and whether they were adequate (quote the methods). 2. Blinding: who was blinded, and whether the primary outcome could be influenced by knowing the allocation. 3. Baseline balance and any imbalances that matter clinically. 4. Deviations and missing data: losses, crossovers, and ITT versus per-protocol analysis. 5. Outcomes: primary outcome definition, whether it is patient-important or surrogate, composite components, and whether it matches the registration. 6. Statistics: sample-size assumptions versus observed rates, the effect estimate with CI, NNT, and multiplicity. 7. Subgroups: pre-specified or not, interaction tests, and credibility. 8. An overall RoB 2 judgement per domain, and the certainty of the conclusion.
RULES	Quote the paper for every judgement, and mark anything not reported as [NOT REPORTED] , because unsupported criticism is easily dismissed in a journal club. Show the arithmetic for NNT and absolute differences. If I haven't provided the registry entry, say what to check there rather than assuming.
OUTPUT	Eight headed sections, a RoB 2 domain table (domain, judgement, quote), then a one-paragraph verdict.
TWEAK	End with the single methodological issue most likely to change how the results should be interpreted.

How to get more out of it

- › Look for the CONSORT flow diagram first. Losses and exclusions tell you a lot.
- › A "significant" result on a surrogate outcome is not the same as a patient-important benefit.
- › Compare the event rate in the control group with the one assumed in the sample-size calculation. If it's lower, the trial was probably underpowered.
- › → **NEXT #475** Absolute effects: ARR, RRR, NNT and NNH

Appraise a cohort or case-control study

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT

When your journal-club paper is observational.

WHY

Observational studies drive many practice changes, and many false beliefs. Confounding, selection and measurement bias can create associations that vanish in trials. Appraising them properly is one of the most important skills a clinician can have.

COPY THE PROMPT BELOW

PERSONA	You are an epidemiologist who teaches the appraisal of observational research, with STROBE for reporting and ROBINS-I or Newcastle-Ottawa for bias.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] Audience and time available: [brief] The study (upload the full text): [UPLOAD] Design as stated by the authors: [cohort / case-control / cross-sectional] Clinical question I care about: [one line]
TASK	1. Confirm the real design and the direction of inquiry, and whether the authors' label is correct. 2. Selection: source population, how participants were identified, and whether selection could relate to both exposure and outcome. 3. Measurement: how the exposure and outcome were measured, and whether misclassification is differential. 4. Confounding: which confounders were measured and adjusted for, which important ones are missing, and whether the adjustment set is sensible (no mediators or colliders). 5. Analysis: model choice, events per variable, handling of missing data, and sensitivity analyses. 6. Causality: apply the Bradford Hill considerations briefly to the main association. 7. Interpretation: whether the authors' language matches the design (association versus causation). 8. An overall risk-of-bias judgement and a bottom line for practice.
RULES	Quote the paper for each judgement and mark anything missing as [NOT REPORTED], because journal-club audiences check claims against the text. Don't dismiss the study merely for being observational; judge how well it handled its own biases.
OUTPUT	Eight headed sections, a bias table (domain, concern, likely direction), then a one-paragraph verdict.
TWEAK	End with the unmeasured confounder most likely to explain the result, and how a future study could address it.

How to get more out of it

- › Draw the causal diagram for the main association. Missing confounders become obvious.
- › Large effect sizes, dose-response relationships and consistency across studies strengthen a causal reading.
- › Watch for "healthy user" effects in drug studies. They are a classic trap.
- › → **NEXT #479** Confounding in an observational paper

Appraise a diagnostic accuracy study

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT

When your journal-club paper evaluates a test, score or imaging method.

WHY

Diagnostic studies are easily biased, through the wrong spectrum of patients, partial verification or unblinded reading, and the resulting accuracy figures often collapse in real practice. QUADAS-2 thinking protects you from adopting a test that won't perform.

COPY THE PROMPT BELOW

PERSONA	You are a diagnostic-research methodologist who applies QUADAS-2 and STARD, and who explains sensitivity, specificity and likelihood ratios in bedside terms.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] The study (upload the full text): [UPLOAD] The clinical role I would use this test for: [replacement / triage / add-on] Prevalence of the condition in my setting, if known: [estimate or "unknown"]
TASK	1. Patient selection: consecutive or random, the case-control ("two-gate") design risk, and whether the spectrum matches my practice. 2. Index test: conduct, threshold (pre-specified or data-driven), and blinding to the reference standard. 3. Reference standard: whether it correctly classifies the condition, and blinding to the index test. 4. Flow and timing: the interval between tests, whether all patients received the same reference standard, and exclusions and indeterminate results. 5. Results: reconstruct the 2×2 table, then check the sensitivity, specificity, predictive values and likelihood ratios with 95% CIs. 6. Apply the results to my setting's prevalence: the post-test probabilities for a positive and a negative result. 7. A QUADAS-2 judgement per domain, with applicability concerns. 8. A bottom line: should we use this test, and for what role?
RULES	Quote the paper for judgements, and show the arithmetic for recalculated values and post-test probabilities, because the audience may check them. Mark any numbers not reported as [NOT REPORTED] instead of inferring them.
OUTPUT	Eight headed sections, the 2×2 table, the QUADAS-2 table, then the bottom line.
TWEAK	End with the patient group in whom this test would most likely mislead, and why.

How to get more out of it

- › Likelihood ratios are more useful at the bedside than sensitivity and specificity. Teach them.
- › If the threshold was chosen after seeing the data, the accuracy is optimistic.
- › A test is only useful if it changes management. Ask what you would do differently with the result.
- › → **NEXT #461** Journal club presentation: appraisal to bottom line

Appraise a systematic review or meta-analysis

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT

When your journal-club paper is a systematic review or meta-analysis.

WHY

Meta-analyses sit at the top of the evidence pyramid, but only if they are done well. Poor searches, mixing of incompatible studies, low-quality inputs and ignored heterogeneity can produce confident, wrong answers. AMSTAR 2 and GRADE thinking separate robust reviews from misleading ones.

COPY THE PROMPT BELOW

PERSONA	You are a systematic-review methodologist and GRADE-trained guideline developer who appraises reviews using AMSTAR 2 and explains forest plots clearly.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The review (upload the full text and supplements if available): [UPLOAD] Clinical question I care about: [one line] My comfort level with meta-analysis statistics: [beginner / intermediate / advanced]
TASK	1. Question and protocol: a clear PICO, and whether it was registered (e.g. PROSPERO) before the review. 2. Search: databases, dates, grey literature, language limits, and whether it was comprehensive. 3. Selection and extraction: duplicate screening and extraction, and the list of excluded studies with reasons. 4. Risk of bias of the included studies: the tool used, and how the results were incorporated. 5. Synthesis: appropriateness of pooling, fixed versus random effects, heterogeneity (I^2 , prediction interval), sensitivity and subgroup analyses. 6. Publication bias: funnel plot or tests, where appropriate. 7. Certainty of evidence: GRADE for the main outcomes, whether reported or as my estimate. 8. An AMSTAR 2 critical-domain judgement and a bottom line.
RULES	Quote the paper for each judgement, and mark missing items as [NOT REPORTED], because reporting gaps are themselves findings. Explain the forest plot using the review's own numbers, without recomputing the pooled estimates.
OUTPUT	Eight headed sections, the AMSTAR 2 table, a plain-language forest-plot explanation, then the bottom line.
TWEAK	End with whether a new trial is still needed on this question, and why.

How to get more out of it

- A meta-analysis of small, biased trials is still biased. Check the inputs before admiring the pooled result.
- High I^2 means "these studies disagree". Find out why before trusting the average.
- Look for the prediction interval. It shows the range of effects to expect in a new setting.
- → **NEXT #473** Explain a forest plot to the audience

Decode the statistics in my paper

2.2 JOURNAL CLUB · PG RESIDENT

USE IT

When the paper's statistics are beyond your comfort level and the journal club is soon.

WHY

Faculty often ask "what does this hazard ratio mean?" or "why this test?". Understanding the statistics in your own paper, in plain language, is the difference between a confident presenter and one who reads the numbers aloud.

COPY THE PROMPT BELOW

PERSONA	You are a biostatistician who teaches clinicians to read the statistics in published papers, in plain language first and technical language second.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The paper (upload, or paste the methods and results): [UPLOAD / PASTE] Statistical terms I don't understand in it: [list] My statistics background: [beginner / intermediate]
TASK	1. List every statistical method used, and explain in one line why it was chosen for this data. 2. Explain each key result in plain language: the effect measure, its CI, the p value, and what they mean together. 3. Explain the terms I listed with an example from this paper. 4. Convert relative measures to absolute ones where the data allows (ARR, NNT), showing the arithmetic. 5. Point out any statistical choice a reviewer might question (e.g. multiplicity, per-protocol analysis, stepwise selection). 6. Write 3 "if asked, say this" sentences about the statistics for my journal club. 7. Suggest one simple visual to explain the main result. 8. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Use only the numbers in the paper, and if something can't be calculated from the reported data, say so rather than estimating, because recalculated numbers will be checked by the audience.
OUTPUT	A methods table (method, purpose), the plain-language results, the glossary, the absolute measures, the questionable choices, then the scripted sentences.
TWEAK	End with the one statistics question I'm most likely to be asked, and the ideal answer.

How to get more out of it

- › Always convert relative risk reductions into absolute numbers. It changes how impressive a result looks.
- › If a CI crosses the line of no effect, say "inconclusive", not "no effect".
- › Ask a statistician colleague to check your NNT arithmetic before the session.
- › → **NEXT #461** Journal club presentation: appraisal to bottom line

468 Five-line PICO summary of the paper

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / MBBS STUDENT

USE IT As your first step with any paper, and for the opening slide.

PERSONA	You are an evidence-based medicine tutor who summarises studies precisely.
MY CONTEXT	The paper or its abstract and methods: [PASTE OR UPLOAD] My specialty: [brief] Audience: [brief]
TASK	1. Write the PICO(T): population with key inclusion features, intervention or exposure, comparator, primary outcome and time frame. 2. Add the design, the setting (country and number of centres) and the sample size. 3. State the main result with its effect estimate and CI. 4. Write the whole summary in 5 lines for a slide.
RULES	Use the paper's own definitions and numbers exactly, and write [NOT REPORTED] if something is missing, because the opening summary sets the audience's understanding of everything that follows.
OUTPUT	The PICO table, then the 5-line slide text.

PRO TIP If you can't write the PICO clearly, re-read the methods before going further.

TRY IT WITH A multicentre RCT of early versus delayed anticoagulation after ischaemic stroke with atrial fibrillation.

→ **NEXT #470** "Why was this study done?" slide

469 Does the abstract match the paper?

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT Early in your preparation, to detect spin and selective reporting.

PERSONA	You are a research-integrity reviewer who compares abstracts with full texts.
MY CONTEXT	The abstract: [PASTE ABSTRACT] The results section and tables: [PASTE OR UPLOAD] Trial registration details, if available: [PASTE]
TASK	1. Compare every number in the abstract with the results section and tables. 2. Check whether the abstract highlights secondary outcomes or subgroups over a non-significant primary outcome. 3. Check whether the abstract's conclusion matches the primary result. 4. Check whether harms reported in the paper appear in the abstract. 5. Summarise any spin found, with examples.
RULES	Quote both texts for every discrepancy, because accusations of spin must be demonstrated, not asserted.
OUTPUT	A comparison table (item, abstract, full text, match), then the spin summary.

PRO TIP Spin in abstracts is common. Finding it makes a memorable journal-club point.

TRY IT WITH A trial with a neutral primary outcome but an abstract emphasising a positive subgroup.

→ **NEXT #483** Detect spin in the discussion and conclusion

470 "Why was this study done?" slide

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT For the introduction section of your journal club.

PERSONA	You are a journal-club presenter who sets the context in 2 slides.
MY CONTEXT	The paper's introduction: [PASTE] What I know about prior evidence and guidelines (with sources): [LIST] Audience: [brief]
TASK	1. Summarise the clinical problem and the knowledge gap as the authors describe it. 2. Summarise the key prior evidence the study builds on, citing the paper's references. 3. State the hypothesis the study tested. 4. Write 1–2 slides with speaker notes.
RULES	Represent the authors' rationale accurately, and mark any claim of mine that isn't from the paper or my sources as [VERIFY] , because context slides are often where misquotation happens.
OUTPUT	The summary, then the slide text and notes.

PRO TIP One slide on "what we believed before this study" makes the results slide far more meaningful.

TRY IT WITH A trial testing conservative oxygen targets in mechanically ventilated patients.

→ **NEXT #471** Methods on two slides

471 Methods on two slides

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT For the methods section of your journal-club presentation.

PERSONA	You are an experienced presenter who condenses methods without losing the critical details.
MY CONTEXT	The paper's methods section: [PASTE OR UPLOAD] Design: [design]
TASK	1. Extract the critical methods: design, setting, participants (key inclusion and exclusion criteria), intervention and comparator details, outcomes and their measurement, sample size, randomisation and blinding (for trials), and the analysis population. 2. Organise them into 2 slides, one with a flow diagram description. 3. Mark the details that matter for the appraisal, for me to emphasise.
RULES	Report the methods exactly as the paper states them, and flag missing essentials as [NOT REPORTED] , because methods slides are where the audience looks for flaws.
OUTPUT	The extracted methods table, the 2-slide text, then the emphasis list.

PRO TIP Show the CONSORT or STROBE flow diagram from the paper. It answers half the methods questions.

TRY IT WITH A pragmatic cluster-randomised trial of a sepsis protocol in emergency departments.

→ **NEXT #472** Key results, clearly presented

472 Key results, clearly presented

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT For the results section of your journal-club presentation.

PERSONA	You are a data presenter who shows results honestly and clearly.
MY CONTEXT	The paper's results section, tables and figures: [PASTE OR UPLOAD] Primary outcome: [state]
TASK	1. Present the primary outcome with the event rates in each group, the effect estimate, the CI and the p value. 2. Present the 2–3 most important secondary outcomes and the main harms. 3. Calculate absolute risk differences and NNT or NNH where possible, showing the arithmetic. 4. Suggest which figure from the paper to show, and how to annotate it. 5. Write 2–3 results slides with speaker notes.
RULES	Reproduce numbers exactly, and keep the primary outcome first even if it is non-significant, because leading with a secondary outcome is spin.
OUTPUT	A results table, the absolute measures with arithmetic, then the slides and notes.

PRO TIP Put the primary result in large font on its own slide. It's the most important slide you'll show.

TRY IT WITH A trial reporting 28-day mortality, ventilator-free days and adverse events.

→ **NEXT #475** Absolute effects: ARR, RRR, NNT and NNH

473 Explain a forest plot to the audience

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT When your paper has a forest plot (meta-analysis or subgroups).

PERSONA	You are a statistics teacher who explains forest plots in plain language.
MY CONTEXT	A description of the forest plot, or the numbers in it: [PASTE OR UPLOAD] Outcome and effect measure: [e.g. risk ratio for mortality]
TASK	1. Explain the anatomy: study rows, squares (weight), lines (CIs), the diamond (pooled estimate) and the line of no effect. 2. Interpret this specific plot: the direction, size and precision of the pooled effect, and the consistency across studies. 3. Explain the heterogeneity statistics shown. 4. Write a 1-minute spoken explanation for the journal club.
RULES	Use only the plot's reported numbers and don't recompute the pooled estimate, because recalculations from a figure are unreliable.
OUTPUT	The anatomy explanation, the interpretation, then the spoken script.

PRO TIP Point at the diamond first, then at the studies that disagree with it.

TRY IT WITH A forest plot of 9 RCTs of corticosteroids in community-acquired pneumonia.

→ **NEXT #466** Appraise a systematic review or meta-analysis

474 Explain the survival curves

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT When your paper reports Kaplan–Meier curves and hazard ratios.

PERSONA	You are a biostatistician who explains time-to-event results clearly.
MY CONTEXT	A description of the Kaplan–Meier figure, and the reported HR, CI, p value and numbers at risk: [PASTE]
TASK	1. Explain the axes, the censoring marks and the numbers at risk. 2. Explain when the curves separate, and what that implies clinically. 3. Explain the hazard ratio in plain language, and whether proportional hazards seem reasonable from the curves. 4. Point out the uncertainty in the tail of the curves, where few patients remain at risk. 5. Write a 1-minute spoken explanation.
RULES	Base the interpretation only on the reported figure and numbers, because survival figures are easy to over-read.
OUTPUT	The explanation points, then the script.

PRO TIP Crossing curves are a warning sign that a single hazard ratio may mislead.

TRY IT WITH Overall survival curves in an oncology trial with an HR of 0.78.

→ **NEXT #467** Decode the statistics in my paper

475 Absolute effects: ARR, RRR, NNT and NNH

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / MBBS STUDENT

USE IT For every trial or cohort result you present.

PERSONA	You are an EBM tutor who converts relative results into absolute terms clinicians can use.
MY CONTEXT	Event counts or rates in each group, for benefits and harms: [PASTE NUMBERS FROM THE PAPER] Follow-up duration: [time]
TASK	1. Calculate the control event rate, the experimental event rate, ARR, RRR and NNT, with a CI for the NNT where possible. 2. Do the same for the main harm (ARI and NNH). 3. Interpret the benefit-harm balance in plain language. 4. Show how the NNT changes for a patient with a higher or lower baseline risk.
RULES	Show every formula and step, state the time frame of the NNT, and use only the paper's numbers, because NNT without a time frame is meaningless.
OUTPUT	A calculation table, then the plain-language interpretation.

PRO TIP "Treat 25 patients for one year to prevent one death" lands better than "a 20% relative risk reduction".

TRY IT WITH Mortality 18% vs 22% over 90 days; major bleeding 4% vs 2%.

→ **NEXT #487** The clinical bottom line

476 Can we trust these results?

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT For the core appraisal section of your journal club.

PERSONA	You are a methodologist who judges internal validity systematically.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The paper (methods and results): [PASTE OR UPLOAD] Design: [design]
TASK	1. Assess selection bias, performance bias, detection bias, attrition bias and reporting bias. 2. For each, give a judgement (low, some concern, high) and a quote from the paper. 3. Say whether any bias could plausibly explain the entire result. 4. Summarise the overall internal validity in 3 sentences.
RULES	Quote the paper for each judgement, and mark unreported items [NOT REPORTED], because judgements without evidence won't survive faculty questioning.
OUTPUT	A bias table (bias, judgement, quote), then the summary.

PRO TIP Attrition above 20%, or unequal between groups, is a common and underrated threat.

TRY IT WITH An open-label RCT with a subjective primary outcome and 18% loss to follow-up.

→ **NEXT #477** Does this apply to our patients?

477 Does this apply to our patients?

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT For the applicability section of your journal club.

PERSONA	You are a clinician-researcher who asks whether trial results transfer to real Indian practice.
MY CONTEXT	The paper's population, setting, intervention delivery and exclusions: [PASTE] Our patients and setting: [e.g. tertiary government hospital, high TB and malnutrition burden, limited ICU beds]
TASK	1. Compare the study population with our patients: age, severity, comorbidities, ethnicity and socio-economic factors. 2. Compare the setting and resources: staffing, monitoring, drug and device availability, and cost. 3. Identify exclusions that remove patients we commonly see. 4. Judge whether the effect would likely be larger, smaller or similar here. 5. Write the applicability slide.
RULES	Base the comparison on the paper's reported characteristics and my description, and mark assumptions as [ASSUMPTION], because applicability judgements are opinion and should be labelled as such.
OUTPUT	A comparison table (factor, study, our setting, impact), the judgement, then the slide.

PRO TIP The exclusion criteria often reveal who the results don't apply to. Read them carefully.

TRY IT WITH A European ICU trial applied to a busy Indian government ICU.

→ **NEXT #487** The clinical bottom line

478 Hunt for the hidden bias

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT When the paper looks clean and you want to find what others will miss.

PERSONA	You are a sceptical peer reviewer with a gift for spotting subtle bias.
MY CONTEXT	The full paper: [PASTE OR UPLOAD]
TASK	1. Look for subtle issues: run-in periods, enrichment, early stopping for benefit, composite outcomes driven by soft components, per-protocol-only analyses, changes to outcomes or sample size, imbalance in co-interventions, and industry involvement in analysis. 2. For each issue found, quote the paper and explain its likely effect. 3. Rank the issues by how much they could change the interpretation. 4. Suggest how to present the key issue in the journal club without being unfair.
RULES	Report only issues supported by the text, and say "none found" for categories that are clean, because inventing problems is as bad as missing them.
OUTPUT	An issue table (issue, quote, likely effect, rank), then the presentation suggestion.

PRO TIP Trials stopped early for benefit often overestimate the effect. Check the stopping rule.

TRY IT WITH An industry-funded trial stopped early after an interim analysis showed benefit.

→ **NEXT #476** Can we trust these results?

479 Confounding in an observational paper

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT When appraising a cohort or case-control study's adjustment strategy.

PERSONA	You are an epidemiologist who evaluates confounding control critically.
MY CONTEXT	Exposure, outcome and the variables adjusted for (from the methods and tables): [PASTE]
TASK	1. List the important confounders for this association from clinical reasoning. 2. Check which were measured and adjusted for, and which are missing. 3. Check for adjustment for mediators or colliders. 4. Judge whether residual confounding could explain the result, considering the effect size. 5. Suggest what a future study should measure.
RULES	Explain each confounder's link to both exposure and outcome, because a variable is only a confounder if it relates to both.
OUTPUT	A confounder table (variable, link to exposure, link to outcome, adjusted yes or no), then the judgement.

PRO TIP An E-value (if you know it) quickly shows how strong an unmeasured confounder would need to be.

TRY IT WITH A cohort linking proton-pump inhibitor use with dementia.

→ **NEXT #464** Appraise a cohort or case-control study

480 Are these the right outcomes?

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT When appraising any trial or cohort study.

PERSONA	You are a patient-centred outcomes researcher.
MY CONTEXT	The primary and secondary outcomes, with their definitions: [PASTE] The registered outcomes, if available: [PASTE]
TASK	1. Classify each outcome as patient-important or surrogate, and hard or soft. 2. For composite outcomes, check whether the components have similar importance and frequency, and which component drives the result. 3. Compare the reported outcomes with the registered ones and note any switching. 4. Judge whether the outcomes justify the authors' conclusion.
RULES	Base conclusions on the definitions in the paper, because outcome definitions often differ from their labels.
OUTPUT	An outcome table (outcome, type, importance, registered yes or no), then the composite analysis and judgement.

PRO TIP A composite driven by "hospitalisation" rather than "death" deserves a closer look.

TRY IT WITH A heart-failure trial with a composite of cardiovascular death and heart-failure hospitalisation.

→ **NEXT #497** Check the trial registry for outcome switching

481 Appraise a non-inferiority trial

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT When your paper claims a new treatment is "not worse" than the standard.

PERSONA	You are a trial statistician who specialises in non-inferiority and equivalence designs.
MY CONTEXT	The margin, its justification, the analysis populations (ITT and per-protocol) and the results with CIs: [PASTE]
TASK	1. Explain the non-inferiority logic and the margin in plain language. 2. Judge whether the margin is clinically acceptable and justified by prior evidence. 3. Check the results in both the ITT and per-protocol populations against the margin. 4. Assess whether the trial's conduct (adherence, cross-over) could bias towards non-inferiority. 5. Draw a simple CI-versus-margin diagram description for a slide.
RULES	Use the reported margin and CIs exactly, because the conclusion depends entirely on where the CI sits relative to the margin.
OUTPUT	The explanation, the margin judgement, the results check, then the diagram description.

PRO TIP Poor adherence makes treatments look similar, which favours a false non-inferiority result.

TRY IT WITH A trial showing oral antibiotics are non-inferior to IV antibiotics for bone infection.

→ **NEXT #463** Deep appraisal of a randomised controlled trial

482 Should we believe the subgroup finding?

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT When a paper highlights a subgroup effect.

PERSONA	You are a methodologist who applies established credibility criteria to subgroup claims.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The subgroup analyses (forest plot or table) and how the authors describe them: [PASTE]
TASK	1. Check whether the subgroup was pre-specified, with a hypothesised direction. 2. Check the test for interaction, not separate p values. 3. Check the number of subgroups tested and the plausibility of the effect. 4. Check consistency with other studies. 5. Give an overall credibility judgement.
RULES	Treat subgroup findings as hypothesis-generating unless the credibility criteria are met, because most subgroup "effects" do not replicate.
OUTPUT	A credibility checklist table, then the judgement.

PRO TIP "The effect was significant in men but not in women" is not evidence of a difference. Ask for the interaction test.

TRY IT WITH A trial claiming benefit only in patients under 65 years.

→ **NEXT #483** Detect spin in the discussion and conclusion

483 Detect spin in the discussion and conclusion

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT When the authors' conclusion seems more positive than their results.

PERSONA	You are a research-integrity researcher who studies spin in medical publications.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] The primary result: [paste] The abstract conclusion, the discussion and the conclusion: [PASTE]
TASK	1. Identify spin strategies: focusing on secondary outcomes, emphasising within-group changes, using "trend" language, claiming equivalence from a non-significant result, and extrapolating beyond the population. 2. Quote each example. 3. Rewrite the conclusion to match the primary result. 4. Suggest one slide showing the original and the rewritten conclusion side by side.
RULES	Quote the text exactly for each instance, because spin must be shown, not asserted.
OUTPUT	A spin table (strategy, quote), then the rewritten conclusion.

PRO TIP The rewritten, spin-free conclusion makes an excellent final journal-club slide.

TRY IT WITH A trial with a non-significant primary outcome whose conclusion says the drug "shows promise".

→ **NEXT #487** The clinical bottom line

484 Funding and conflicts: do they matter here?

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT When appraising who funded the study and who controlled it.

PERSONA	You are a research-ethics specialist who assesses the impact of funding and conflicts on research.
MY CONTEXT	The funding and conflict-of-interest statements and the role of the sponsor: [PASTE]
TASK	1. Summarise the funding and the sponsor's role in design, conduct, analysis and writing. 2. Summarise the authors' declared conflicts. 3. Identify aspects of the study where sponsor influence could matter (comparator choice, dose, outcomes, early stopping). 4. Give a balanced judgement: a concern to note, not a reason to dismiss good methods.
RULES	Base the assessment only on the declarations, and avoid implying misconduct, because industry funding alone does not invalidate a study.
OUTPUT	A summary table, the areas of potential influence, then the balanced judgement.

PRO TIP Check whether the comparator was given at an optimal dose. Weak comparators are a classic issue.

TRY IT WITH A pharmaceutical-sponsored trial comparing a new drug with a low dose of the standard drug.

→ **NEXT #478** Hunt for the hidden bias

485 Compare the paper with its editorial and letters

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT When the paper has an accompanying editorial or published correspondence.

PERSONA	You are an academic who synthesises expert commentary on major papers.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The editorial and any letters or author replies: [PASTE OR UPLOAD] My own appraisal points: [LIST]
TASK	1. Summarise the editorialist's main points of praise and concern. 2. Summarise the key criticisms in the letters and the authors' responses. 3. Compare them with my own appraisal: agreements and new points. 4. Write one slide summarising expert commentary.
RULES	Attribute every point to its source accurately, and quote where possible, because misattributing expert opinion is embarrassing in front of faculty.
OUTPUT	The summaries, the comparison, then the slide.

PRO TIP Letters to the editor often contain the sharpest critique of a paper, and they are free to read.

TRY IT WITH A landmark trial published with an editorial and 6 letters of correspondence.

→ **NEXT #461** Journal club presentation: appraisal to bottom line

486 How does this fit with previous evidence?

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT For the "context" part of your appraisal.

PERSONA	You are an evidence synthesiser who places new studies within the existing body of evidence.
MY CONTEXT	The paper's main finding: [paste] Prior key studies or reviews (uploaded or with details): [UPLOAD / LIST] Current guideline position: [paste with source]
TASK	1. Compare the new finding with the prior evidence: consistent, conflicting or new territory. 2. Explain any differences (population, design, intervention, outcome). 3. Judge whether this study should shift the weight of evidence. 4. Write one "where this fits" slide.
RULES	Use only the studies and guidelines I provided, marking others [VERIFY], because comparisons with misremembered studies undermine the appraisal.
OUTPUT	A comparison table, the judgement, then the slide.

PRO TIP A single trial rarely overturns a body of evidence. Say so if that's the case.

TRY IT WITH A new trial finding no benefit of a therapy that earlier small trials supported.

→ **NEXT #487** The clinical bottom line

487 The clinical bottom line

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT For the final slide of your journal club.

PERSONA	You are a senior clinician who turns appraisals into clear, honest practice decisions.
MY CONTEXT	My appraisal summary (validity, results, applicability): [PASTE] Current practice in our unit: [brief]
TASK	1. State whether the results are valid, important and applicable, in one line each. 2. State the bottom line in 2 sentences: change practice, consider in selected patients, or don't change practice. 3. Name the patients in whom it would or wouldn't apply. 4. Suggest what would need to happen before a local change (protocol, cost, training).
RULES	Match the strength of the recommendation to the strength of the evidence, because the bottom line is what the department will act on.
OUTPUT	The three one-liners, the bottom line, the patient groups, then the next steps.

PRO TIP End with a question to the head of department: "Should we change our protocol?". It starts the real discussion.

TRY IT WITH Appraisal of a trial of restrictive versus liberal transfusion in upper gastrointestinal bleeding.

→ **NEXT #488** Questions to drive the discussion

488 Questions to drive the discussion

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT To prepare the discussion part of your journal club.

PERSONA	You are a journal-club moderator who knows how to start productive debates.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The paper and my appraisal summary: [PASTE] Audience: [brief]
TASK	1. Write 6 discussion questions: 2 on methodology, 2 on interpretation, and 2 on local practice. 2. For each, write the key points a good answer would contain. 3. Suggest the order to ask them in, and when to invite senior faculty. 4. Suggest a closing question that brings the discussion back to our own patients.
RULES	Make the questions open-ended, not yes or no, because closed questions end discussions.
OUTPUT	The questions with answer points, then the facilitation order.

PRO TIP Ask the most junior person first. It keeps seniors from closing the discussion early.

TRY IT WITH A trial on early versus delayed laparoscopic cholecystectomy.

→ **NEXT #489** The faculty questions I'll face

489 The faculty questions I'll face

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT 2–3 days before your journal club.

PERSONA	You are a senior professor famous for difficult journal-club questions.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The paper: [PASTE OR UPLOAD] My slides or appraisal summary: [PASTE]
TASK	1. List 12 questions faculty are likely to ask, from basic design questions to deep methodology and practice implications. 2. For each, give a model answer grounded in the paper. 3. Mark the 3 hardest questions and suggest backup slides for them.
RULES	Ground every answer in the paper, and where the paper doesn't say, write "the paper does not report this" as the honest answer, because bluffing is obvious to faculty.
OUTPUT	A question-and-answer table, then the backup-slide suggestions.

PRO TIP "The paper doesn't report that, but I would expect..." is a perfectly good answer.

TRY IT WITH An RCT on tranexamic acid in gastrointestinal bleeding.

→ **NEXT #461** Journal club presentation: appraisal to bottom line

490 One-page appraisal handout

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT To circulate before or after the journal club.

PERSONA	You are a medical educator who designs compact evidence summaries.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] My appraisal: [PASTE] The paper's citation: [paste]
TASK	1. Create a one-page summary: citation, PICO, design, key results with absolute measures, key strengths, key limitations, applicability and the bottom line. 2. Keep it under 300 words, laid out in boxes. 3. Add 2 questions for the reader to consider. 4. Add a one-line verdict and the date of the journal club, for the department archive.
RULES	Keep every number identical to the paper, because handouts are kept and quoted later.
OUTPUT	The handout text with layout guidance.

PRO TIP A folder of these one-pagers becomes your department's evidence library over time.

TRY IT WITH Appraisal of a trial of balanced crystalloids versus saline in critically ill adults.

→ **NEXT #487** The clinical bottom line

491 Fill the CASP checklist for my paper

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / MBBS STUDENT

USE IT When your department requires a completed appraisal checklist.

PERSONA	You are an EBM tutor familiar with the CASP checklists for each study design.
MY CONTEXT	Study design: [RCT / cohort / case-control / diagnostic / systematic review / qualitative] The paper: [PASTE OR UPLOAD]
TASK	1. Identify the correct CASP checklist for the design. 2. Answer each checklist question with yes, no or can't tell, and a quote from the paper. 3. Summarise the overall appraisal in 3 sentences.
RULES	Base every answer on the paper's text and mark "can't tell" where it isn't reported, because the checklist is only valuable if it is honest.
OUTPUT	The checklist table (question, answer, quote), then the summary.

PRO TIP Check the current CASP checklist wording on the CASP website, as the questions are occasionally revised.

TRY IT WITH A prospective cohort on air pollution and childhood asthma exacerbations.

→ **NEXT #461** Journal club presentation: appraisal to bottom line

492 Certainty of evidence using GRADE

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT When you want to rate how confident we can be in the paper's main finding.

PERSONA	You are a GRADE-trained guideline methodologist.
MY CONTEXT	The paper's design and main outcome result: [PASTE] Its main limitations from my appraisal: [LIST]
TASK	1. Start from the design's baseline certainty. 2. Assess each downgrading domain: risk of bias, inconsistency, indirectness, imprecision and publication bias. 3. Assess the upgrading domains for observational studies (large effect, dose-response, plausible confounding). 4. Give the final certainty (high, moderate, low or very low) with reasoning. 5. Write a GRADE-style sentence ("...probably reduces...").
RULES	Apply GRADE to a single study cautiously, and state that GRADE is designed for bodies of evidence, because rating one study alone overstates precision.
OUTPUT	A GRADE table (domain, judgement, reason), then the final rating and the sentence.

PRO TIP GRADE-style wording, such as "probably" or "may", is an excellent way to phrase your bottom line.

TRY IT WITH A single large RCT with some risk of bias and a wide CI.

→ **NEXT #487** The clinical bottom line

493 Appraise a qualitative study

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT When your journal-club paper uses interviews or focus groups.

PERSONA	You are a qualitative health researcher familiar with COREQ and the CASP qualitative checklist.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The paper: [PASTE OR UPLOAD]
TASK	1. Assess the fit of a qualitative approach for the question. 2. Assess sampling, data collection, the researchers' reflexivity, analysis method, saturation and rigour (e.g. member checking, triangulation). 3. Assess whether the findings are supported by quotes. 4. Discuss transferability to our setting. 5. Summarise the value of the findings for practice.
RULES	Judge qualitative research by its own standards of rigour, not by sample size or statistics, because applying quantitative criteria misjudges it.
OUTPUT	Headed appraisal sections, then the summary.

PRO TIP Qualitative studies explain the "why" behind poor adherence and patient behaviour. They make excellent journal clubs.

TRY IT WITH Interviews with patients about reasons for stopping TB treatment.

→ **NEXT #461** Journal club presentation: appraisal to bottom line

494 Appraise an economic evaluation

STANDARD

2.2 JOURNAL CLUB · FACULTY / SENIOR RESIDENT

USE IT When your paper is a cost-effectiveness or cost-utility analysis.

PERSONA	You are a health economist who explains economic evaluations to clinicians.
MY CONTEXT	The paper: [PASTE OR UPLOAD] Our health system context: [e.g. Indian public sector, PMJAY]
TASK	1. Identify the type of evaluation, the perspective, the comparators and the time horizon. 2. Check the cost and outcome sources, discounting, and QALY or DALY calculations. 3. Explain the ICER and whether it is below the stated threshold. 4. Assess the sensitivity analyses and the uncertainty. 5. Judge transferability to Indian costs and willingness-to-pay thresholds.
RULES	Report the paper's own numbers and thresholds, and mark any Indian threshold as [VERIFY], because thresholds vary and are debated.
OUTPUT	Headed sections, then a transferability judgement.

PRO TIP An intervention can be cost-effective in one country and unaffordable in another. Emphasise that.

TRY IT WITH Cost-effectiveness of a new diagnostic test for TB in a high-burden setting.

→ **NEXT #477** Does this apply to our patients?

495 Appraise a prognostic or prediction-model study

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT When your paper develops or validates a risk score or prediction model.

PERSONA	You are a clinical prediction-model methodologist familiar with TRIPOD and PROBAST.
MY CONTEXT	The paper: [PASTE OR UPLOAD] Intended use of the model: [brief]
TASK	1. Assess the participants, predictors, outcome definition and timing. 2. Assess the sample size and events per variable, the handling of missing data and continuous predictors, and the variable-selection method. 3. Assess performance: discrimination (C-statistic), calibration and clinical utility. 4. Check for internal and external validation. 5. Judge readiness for clinical use.
RULES	Distinguish development from validation clearly, because a model tested only on its own development data will look better than it is.
OUTPUT	A PROBAST-style table by domain, then the readiness judgement.

PRO TIP Calibration matters as much as the C-statistic, and it is often not reported. Look for it.

TRY IT WITH A new score predicting in-hospital mortality in dengue.

→ **NEXT #461** Journal club presentation: appraisal to bottom line

496 Appraise a clinical guideline (AGREE II)

STANDARD

2.2 JOURNAL CLUB · FACULTY / SENIOR RESIDENT

USE IT When the journal-club document is a clinical practice guideline.

PERSONA	You are a guideline appraiser trained in AGREE II.
MY CONTEXT	The guideline (upload or paste the methods and recommendations): [UPLOAD]
TASK	1. Appraise the AGREE II domains: scope and purpose, stakeholder involvement, rigour of development, clarity of presentation, applicability and editorial independence. 2. Give a domain score or judgement with evidence from the document. 3. Highlight the recommendations with weak evidence but strong wording. 4. Give an overall recommendation for use.
RULES	Base scores on what the document reports, and mark missing methodological details as [NOT REPORTED] , because good guidelines document their methods transparently.
OUTPUT	An AGREE II domain table, the flagged recommendations, then the overall judgement.

PRO TIP Look for how conflicts of interest among panel members were managed. It's often revealing.

TRY IT WITH A national guideline on management of hypertension.

→ **NEXT #461** Journal club presentation: appraisal to bottom line

497 Check the trial registry for outcome switching

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT

USE IT For any RCT, before you present it.

PERSONA	You are a research-transparency analyst.
MY CONTEXT	Registration number and the registered primary and secondary outcomes, sample size and dates: [PASTE FROM THE REGISTRY] What the paper reports: [PASTE]
TASK	1. Compare the registered and reported primary outcomes: definition, measure and time point. 2. Compare the secondary outcomes, and note any added or dropped. 3. Compare the planned and actual sample sizes and dates (was it registered prospectively?). 4. Check whether the paper explains any changes. 5. Judge the impact on credibility.
RULES	Report only differences that are verifiable from the two sources, because an outcome switch is a serious accusation.
OUTPUT	A comparison table (item, registered, reported, explained yes or no), then the judgement.

PRO TIP Many trials change outcomes legitimately with an explanation. Note whether one is given.

TRY IT WITH A trial registered on ClinicalTrials.gov whose paper reports a different primary time point.

→ **NEXT #480** Are these the right outcomes?

498 Read a paper properly in 30 minutes

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / MBBS STUDENT

USE IT When you are short of time but must appraise a paper.

PERSONA	You are an efficient academic reader who teaches a structured reading method.
MY CONTEXT	The paper: [PASTE OR UPLOAD] My purpose: [journal club / clinical question / thesis]
TASK	1. Give a 30-minute reading plan: title and abstract (2 min), figures and tables (5 min), methods (10 min), results (8 min), discussion and limitations (5 min). 2. Give the 3 questions to answer at each stage. 3. Walk through the paper with me using this plan, answering those questions. 4. Summarise the key appraisal points found.
RULES	Answer the stage questions only from the paper, and note what is missing, because a fast read must still be accurate.
OUTPUT	The plan, the stage-by-stage answers, then the key points.

PRO TIP Look at the figures and tables before reading the text. They show you what the paper really found.

TRY IT WITH A 12-page cohort study on vitamin D and mortality.

→ **NEXT #461** Journal club presentation: appraisal to bottom line

499 Explain the paper to MBBS students

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT When you want to use a journal-club paper to teach undergraduates.

PERSONA	You are an EBM teacher for medical students.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] The paper and my appraisal: [PASTE] Student level: [e.g. final-year MBBS] Time: [minutes]
TASK	1. Explain the study's question and why it matters in simple terms. 2. Teach 2–3 EBM concepts using this paper (e.g. randomisation, NNT, bias). 3. Create 5 questions to check understanding. 4. Write a 10-minute teaching plan.
RULES	Keep the numbers accurate and the concepts simple but correct, because students will remember what you say as fact.
OUTPUT	The explanation, the concept list with examples, the questions, then the plan.

PRO TIP One real paper teaches EBM better than any lecture on EBM.

TRY IT WITH A trial of azithromycin for COPD exacerbations, for final-year MBBS students.

→ **NEXT #500** Quiz questions from the journal club

500 Quiz questions from the journal club

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT At the end of the journal club, or for department learning records.

PERSONA	You are an assessment writer who creates EBM-focused MCQs.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] The paper, my appraisal and the key teaching points: [PASTE]
TASK	1. Write 6 MCQs testing appraisal skills and results interpretation using this paper. 2. Include one question on NNT, one on bias and one on applicability. 3. Include one clinical-vignette question that asks how the result applies to a specific patient. 4. Give the answers with brief explanations, citing the relevant part of the paper.
RULES	Base every question on the paper's actual numbers and methods, because inaccurate questions teach the wrong lesson.
OUTPUT	The numbered MCQs, then the answer key.

PRO TIP Run the quiz through a live poll. It shows immediately what the group has understood.

TRY IT WITH A trial comparing two insulin regimens in hospitalised patients.

→ **NEXT #461** Journal club presentation: appraisal to bottom line

501 Turn my critique into a letter to the editor

STANDARD

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

USE IT When your appraisal found a significant issue that hasn't been raised.

PERSONA	You are an experienced correspondent who writes publishable letters to journals.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The paper's citation and publication date: [paste] My main critique point and the supporting evidence: [PASTE] The journal's letter limits and time window: [paste]
TASK	1. Check whether the issue has already been raised in published correspondence (tell me how to check). 2. Draft a concise, collegial letter within the limits. 3. Suggest a title.
RULES	Keep the tone constructive and cite only the paper and sources I provide, because letters that attack authors are rarely published.
OUTPUT	The checking steps, the title, then the letter with its word count.

PRO TIP Submit quickly. Most journals accept letters only within a few weeks of publication.

TRY IT WITH A trial whose exclusion criteria removed most patients with CKD, the majority in Indian practice.

→ **NEXT #485** Compare the paper with its editorial and letters

502 Moderating a journal club well

STANDARD

2.2 JOURNAL CLUB · FACULTY / SENIOR RESIDENT

USE IT When you are the faculty moderator or coordinator of a journal club.

PERSONA	You are a medical educator who has run a successful journal-club programme for years and trains new moderators.
MY CONTEXT	Format of our journal club (frequency, duration, presenters): [brief] The paper for this session and the presenter's level: [brief] Common problems in our sessions: [e.g. silence, seniors dominating, abstract-reading presentations]
TASK	1. Give a pre-session checklist: paper approval, the appraisal checklist, and the presenter's rehearsal. 2. Give a moderation script: opening, time-keeping, drawing out quieter participants, and managing dominant voices. 3. List 5 teaching points to weave in about appraisal methods. 4. Suggest how to close with a bottom line and an action item. 5. Suggest how to give the presenter constructive feedback afterwards.
RULES	Keep the focus on learning appraisal skills rather than catching the presenter out, because a psychologically safe session produces far better discussion.
OUTPUT	The checklist, the script, the teaching points, the closing, then the feedback structure.

PRO TIP Keep a log of each journal club's bottom line. Over a year it becomes the department's own evidence summary.

TRY IT WITH Monthly journal club for 15 residents where the discussion usually goes silent.

→ **NEXT #488** Questions to drive the discussion

QUICK PROMPTS · 2.2

503 This study's design in one line

QUICK

2.2 JOURNAL CLUB · MBBS STUDENT / PG RESIDENT

Act as an epidemiology tutor. From this methods section, [PASTE METHODS], state the study's true design in one line (e.g. "a multicentre, double-blind, placebo-controlled RCT"), and flag it if the authors' own label differs. Add one line on what this design can conclude.

TRY IT WITH A "prospective study" that analysed an existing registry.

504 Find the real primary outcome

QUICK

2.2 JOURNAL CLUB · PG RESIDENT

Act as a trial methodologist. From this paper, [PASTE METHODS AND RESULTS], identify the pre-specified primary outcome, its exact definition and time point, and whether the result for it was positive, negative or inconclusive. Answer in 4 lines, quoting the paper.

TRY IT WITH A trial reporting many outcomes in its abstract.

505 Quick ARR and NNT

QUICK

2.2 JOURNAL CLUB · MBBS STUDENT / PG RESIDENT

Act as an EBM tutor. With control event rate [CER] and experimental event rate [EER] over [TIME], calculate the ARR, RRR and NNT, showing the formulas and arithmetic step by step, and write one plain-language sentence I can say at journal club.

TRY IT WITH Mortality 24% in the control group vs 19% in the treatment group at 28 days.

506 What does this hazard ratio mean?

QUICK

2.2 JOURNAL CLUB · PG RESIDENT

Act as a biostatistician explaining to clinicians. Explain the hazard ratio [HR, 95% CI, p] for [OUTCOME] in this paper in plain language, including what the CI tells us, how an HR differs from a relative risk, and how to phrase it on a slide.

TRY IT WITH HR 0.82 (95% CI 0.70–0.96) for death from any cause.

507 Intention-to-treat or per-protocol?

QUICK

2.2 JOURNAL CLUB · PG RESIDENT

Act as a trial methodologist. Explain whether this trial's main analysis, [DESCRIBE ANALYSIS POPULATION], is intention-to-treat, modified ITT or per-protocol, why it matters for bias, and which analysis should drive the conclusion here. Keep it under 120 words. Flag anything you are unsure of so I can check it.

TRY IT WITH A trial that excluded patients who never received the study drug.

508 Was allocation really concealed?

QUICK

2.2 JOURNAL CLUB · PG RESIDENT

Act as a risk-of-bias assessor. From this description, [PASTE RANDOMISATION TEXT], judge whether allocation concealment was adequate, inadequate or unclear under RoB 2 thinking, quote the key phrase, and explain in 2 lines why it matters for the result. Flag anything you are unsure of so I can check it.

TRY IT WITH "Patients were randomised using sealed envelopes."

509 Who was blinded, and does it matter?

QUICK

2.2 JOURNAL CLUB · PG RESIDENT

Act as a trial appraiser. From this methods text, [PASTE TEXT], list who was blinded (participants, clinicians, outcome assessors, analysts), and judge whether the lack of blinding could have influenced the primary outcome [OUTCOME], given how subjective it is. Flag anything you are unsure of so I can check it.

TRY IT WITH An open-label trial with a pain-score primary outcome.

510 Is this loss to follow-up a problem?

QUICK

2.2 JOURNAL CLUB · PG RESIDENT

Act as a clinical epidemiologist. Losses to follow-up were [NUMBERS PER GROUP] out of [TOTALS]. Tell me whether this threatens validity, considering the size, the difference between groups and the event rate, and what sensitivity analysis the authors should have done.

TRY IT WITH 12% lost in the intervention arm vs 5% in the control arm.

511 The whole paper in one sentence

QUICK

2.2 JOURNAL CLUB · PG RESIDENT / MBBS STUDENT

Act as a senior academic. Summarise this paper, [PASTE ABSTRACT], in one sentence of under 35 words stating the design, population, intervention or exposure, and the main result with its key number, using associational language if the study is observational.

TRY IT WITH A cohort study of statin use and outcomes in sepsis.

512 Was this study adequately powered?

QUICK

2.2 JOURNAL CLUB · PG RESIDENT

Act as a biostatistician. Compare this trial's sample-size assumptions, [ASSUMED EVENT RATES AND EFFECT], with the observed results, [OBSERVED RATES]. Tell me whether the study was likely underpowered, and how that affects the interpretation of a non-significant result. Flag anything you are unsure of so I can check it.

TRY IT WITH Assumed control mortality 30%, observed 18%, with a non-significant result.

513 Is this outcome a surrogate?

QUICK

2.2 JOURNAL CLUB · PG RESIDENT

Act as an outcomes researcher. Tell me whether [OUTCOME] in this study is a surrogate or a patient-important outcome, whether it has been validated as a surrogate for [HARD OUTCOME], and what a clinician should conclude from a benefit on it. Keep it under 120 words.

TRY IT WITH HbA1c reduction as the primary outcome in a diabetes drug trial.

514 Would this change my practice?

QUICK

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

Act as a pragmatic senior clinician. Given this paper's result, [RESULT], quality, [APPRAISAL SUMMARY], and our setting, [SETTING], tell me in 4 lines whether this should change our practice now, in selected patients, or not yet, and why. Flag anything you are unsure of so I can check it.

TRY IT WITH A single-centre RCT showing benefit of a new sedation protocol.

515 Explain a methods term from my paper

QUICK

2.2 JOURNAL CLUB · MBBS STUDENT / PG RESIDENT

Act as an EBM tutor. Explain the term [TERM], as used in my paper [CONTEXT], in plain language in under 80 words, with an example from the paper, and say why it matters for judging the study's reliability. Flag anything you are unsure of so I can check it.

TRY IT WITH Term: "fragility index" in a trial of tranexamic acid.

516 Does the title overclaim?

QUICK

2.2 JOURNAL CLUB · PG RESIDENT

Act as a research-integrity reviewer. Compare this paper's title, [TITLE], with its design [DESIGN] and primary result [RESULT], tell me whether the title overclaims or implies causation inappropriately, and suggest an accurate alternative title. Flag anything you are unsure of so I can check it.

TRY IT WITH A cross-sectional study titled "Vitamin D deficiency causes severe COVID-19".

517 Relative versus absolute risk for the audience

QUICK

2.2 JOURNAL CLUB · MBBS STUDENT / PG RESIDENT

Act as a medical educator. Using this paper's numbers, [EVENT RATES], explain the difference between relative and absolute risk reduction to a mixed audience in 3 sentences, and show why the relative figure sounds more impressive. Flag anything you are unsure of so I can check it.

TRY IT WITH A drug that reduces stroke from 2% to 1% over 5 years.

518 Explain the key figure in three lines

QUICK

2.2 JOURNAL CLUB · PG RESIDENT

Act as a data-presentation coach. From this description of the paper's key figure, [DESCRIBE FIGURE], write 3 spoken lines explaining what the axes show, what the main pattern is, and what it means clinically, without overstating certainty. Flag anything you are unsure of so I can check it.

TRY IT WITH A figure showing cumulative incidence of infection in two groups over 90 days.

519 A sharp question to ask the presenter

QUICK

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

Act as an experienced journal-club participant. From this paper summary, [PASTE SUMMARY], suggest 3 sharp but fair questions to ask the presenter that test understanding of the methods and applicability, each under 25 words. Flag anything you are unsure of so I can check it.

TRY IT WITH Summary of a trial of early goal-directed therapy.

520 Summarise the paper for colleagues

QUICK

2.2 JOURNAL CLUB · PG RESIDENT / FACULTY

Act as a medical science communicator. Summarise this paper, [TITLE, DESIGN, KEY RESULT, APPRAISAL VERDICT], in a 4-line message for the department WhatsApp group: what was studied, what was found (with the absolute effect), the main limitation and our bottom line.

TRY IT WITH A trial of balanced crystalloids versus saline in the ICU.

2.3

Slide design & structure

Slides support the speaker; they are not the speech. One message per slide, visuals over paragraphs, and text readable from the back row.

40

prompts

4

Master

23

Standard

13

Quick

#521 Redesign my existing deck, slide by slide

#522 A personal slide design system for all my talks

#523 Build a deck with AI tools, safely

#524 Turn data and tables into clear slide visuals

#525 Rewrite slide titles as messages

#526 Turn a text-heavy slide into a visual

#527 Simplify a table for a slide

#528 Build a flowchart slide

#529 Adapt a guideline algorithm for a slide

#530 Two conditions side by side

#531 A timeline slide

#532 Clinical image slide with annotation

#533 Icons and simple graphics for concepts

#534 Colours that work for everyone

#535 Fonts and sizes that are readable

#536 When to use animation, and when not to

#537 Cut my deck to fit the time

#538 Write speaker notes for my slides

#539 Separate the slides from the handout

#540 A design brief for building slides in Canva

#541 A prompt for AI slide generators

#542 Choose the right chart for a slide

#543 Citing sources on slides

#544 Title and opening slide design

#545 Section divider slides that guide the audience

#546 Slides for online teaching and webinars

#547 Using images and figures legally

#548 Turn a slide title into a message

#549 Check a slide for clutter

#550 Paragraph to one clean slide

#551 A three-colour palette for my deck

#552 Font sizes for my room

#553 Icon ideas for my concepts

#554 Alt text for my slide images

#555 A caption for a figure on a slide

#556 Should these bullets be a diagram?

#557 Design my final slide

#558 QR code slide content

#559 Should this table be a chart?

#560 Does my slide pass the 6 × 6 rule?

Redesign my existing deck, slide by slide

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT

When your deck is text-heavy, cluttered or inconsistent, and the talk is in a few days.

WHY

Most medical decks are written like documents: full sentences, tiny fonts, six colours and tables pasted from papers. A slide-by-slide redesign into message titles, one idea per slide and simple visuals transforms the talk, without changing a single fact.

COPY THE PROMPT BELOW

PERSONA	You are a presentation designer who specialises in medical and scientific talks. You apply the assertion-evidence approach (a message title plus visual evidence), and you are strict about readability and consistency.
MY CONTEXT	My deck (paste the slide text, or upload the .pptx or PDF): [PASTE OR UPLOAD] Talk type, audience and duration: [e.g. departmental seminar, residents, 40 minutes] Venue and screen: [e.g. seminar room projector / hall / online] Template or branding rules: [institution template or "free"] Tool I will edit in: [PowerPoint / Google Slides / Keynote / Canva]
TASK	1. Diagnose the deck as a whole: text density, font sizes, colour consistency, visual hierarchy and slide count against the time. 2. For every slide, give: the problem, a message title (a full-sentence takeaway), the redesigned content (maximum 3 bullets, or a visual), and what moves to the speaker notes. 3. Identify slides to merge, split or delete. 4. Suggest a visual for each text-heavy slide: flowchart, table-to-chart, icon grid, timeline or annotated image. 5. Propose a consistent style: 2 fonts at most, a 3-colour palette plus neutrals, fixed title positions and a grid. 6. Give the edit steps in my tool (master slides, layout reuse, alignment tools).
RULES	Keep every fact, number and reference exactly as in my deck, because the redesign must change the presentation, not the content. Mark anything that looks factually doubtful as [CHECK] rather than correcting it. Keep body text at 24 pt or more and titles at 32 pt or more for a projector.
OUTPUT	The overall diagnosis, a slide-by-slide table (slide, problem, new title, new content, notes), the merge and delete list, the style guide, then the edit steps.
TWEAK	End with the 3 slides whose redesign will make the biggest difference, and before-and-after text for each.

How to get more out of it

- › Redesign the master slide first. Every slide improves at once.
- › Read only the slide titles in order. If they tell the story, the redesign has worked.
- › Keep the old deck. If time runs out, present the 10 most important redesigned slides.
- › → **NEXT #525** Rewrite slide titles as messages

A personal slide design system for all my talks

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT

When you present often and want every deck to look consistent and professional without starting from scratch.

WHY

Doctors present dozens of times a year: seminars, journal clubs, case presentations, CMEs. A simple, reusable design system (template, fonts, colours, layouts, chart style) saves hours, looks professional, and builds a recognisable teaching identity.

COPY THE PROMPT BELOW

PERSONA	You are a brand and presentation designer who builds simple design systems for clinicians and academics, focusing on readability, accessibility and speed of use.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Tool: [PowerPoint / Google Slides / Keynote / Canva] Institution branding requirements: [logo, colours, or "none"] Types of talks I give: [e.g. seminars, journal clubs, case presentations, CMEs] Style I like, or examples I admire: [brief or "suggest"] Screen types: [projector / large display / online]
TASK	1. Define the typography: one heading font and one body font (widely available), with sizes for titles, body, captions and references. 2. Define the colour palette: a background, text, one primary accent, one secondary accent and a highlight colour for key numbers, checked for colour-blind safety and projector contrast. 3. Define 8 reusable layouts: title, section divider, message plus single visual, two-column comparison, table, flowchart, image with annotation, and take-home. 4. Define chart and table styles: minimal gridlines, direct labels, consistent decimals. 5. Define rules for icons, images and references on slides. 6. Give the step-by-step set-up of the master template in my tool.
RULES	Recommend only widely available fonts and free resources, and mark institutional requirements as [CHECK INSTITUTION TEMPLATE], because unavailable fonts break when you present on another computer. Keep contrast high enough for poor projectors.
OUTPUT	A style-guide table (element, specification), the layout descriptions, the chart rules, then the set-up steps.
TWEAK	End with a one-page checklist to run on any deck before presenting.

How to get more out of it

- › Embed fonts or use system fonts. Hospital computers rarely have your fancy fonts.
- › Save the template as a theme file so every new deck starts from it.
- › Dark text on a light background is the safest choice for bright seminar rooms.
- › → NEXT #535 Fonts and sizes that are readable

Build a deck with AI tools, safely

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT

When you want to use AI (Claude, ChatGPT, Copilot in PowerPoint, Gamma, Canva) to create a first draft of a deck quickly.

WHY

AI slide tools produce beautiful decks in minutes, with invented statistics, outdated guidelines and generic content. Used properly, with your verified content as the input and a strict structure, they save hours without putting errors in front of an audience.

COPY THE PROMPT BELOW

PERSONA	You are a medical educator and AI-tools trainer who builds presentation workflows that are fast and accurate.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] What I most want to get out of this: [one line] Topic, audience and duration: [brief] AI tool I will use: [Claude / ChatGPT / Copilot in PowerPoint / Gamma / Canva Magic Design / other] My verified content (outline, notes, key sources, numbers): [PASTE] Design preferences or template: [brief]
TASK	1. Write the exact prompt to paste into my tool to generate the deck from my content: the structure, one message per slide, speaker notes, citations kept as given, and the design constraints. 2. Add instructions telling the tool to use only my content, to mark any gaps as [ADD CONTENT], and to create no statistics or references. 3. Give a step-by-step workflow: generate, review the text, fix the design, verify facts, add references, rehearse. 4. Give a verification checklist for AI-generated slides: numbers, drug doses, guideline names and years, image rights, and citations. 5. List the tool-specific limitations and workarounds for the tool I chose, marked [MAY DIFFER BY VERSION].
RULES	Every fact in the final deck must come from my verified content, because AI slide tools confidently produce plausible but wrong medical details. Show me the slide-by-slide text for approval before any file is built.
OUTPUT	The ready-to-paste prompt in a code block, the workflow, the verification checklist, then the tool notes.
TWEAK	End with the 5 most common errors AI tools introduce into medical slides, and how to catch each one.

How to get more out of it

- › Give the AI your outline and sources, not just a topic. Topic-only prompts produce generic decks.
- › Always check drug doses and guideline years on AI-made slides. They are the most frequent errors.
- › Declare AI assistance in your teaching portfolio where your institution expects it.
- › → **NEXT #521** Redesign my existing deck, slide by slide

Turn data and tables into clear slide visuals

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT

When your talk includes results, statistics or comparisons that currently sit in dense tables.

WHY

A 12-column table on a slide is unreadable from the third row of the audience. Choosing the right chart, stripping the clutter and highlighting the one number that matters turns data into a message people remember.

COPY THE PROMPT BELOW

PERSONA	You are a data-visualisation specialist for medical talks who makes charts readable from the back of a lecture hall.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] What I most want to get out of this: [one line] The data or tables (paste): [PASTE DATA / TABLES] The message each table should convey: [list, or "suggest"] Tool: [PowerPoint / Excel / Google Sheets / Canva] Audience and room size: [brief]
TASK	1. For each table, identify the single message it supports. 2. Recommend the best visual for that message (bar, dot plot, line, forest-style plot, simple highlighted table, or a single big number), with the reason. 3. Specify the design: sorted order, direct labels instead of legends, minimal gridlines, one highlight colour and a message title. 4. Say what to remove: extra decimals, unnecessary columns, legends that could be direct labels, and 3D effects. 5. Give step-by-step build instructions in my tool. 6. Write the message title and a one-line speaker note for each visual. 7. Flag any table that should stay a table because readers need the exact values. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep every number exactly as in my data, and keep the axes honest (bars start at zero), because distorted charts mislead the audience and damage your credibility.
OUTPUT	A table (original table, message, recommended visual, design notes), the build steps, then the titles and notes.
TWEAK	End with the one chart that best tells the main story of the talk.

How to get more out of it

- › If only one number matters, show only that number, in very large font, with context.
- › Sort bars by value, not alphabetically, unless the order has meaning.
- › Use colour to highlight, not to decorate. Grey everything except the key bar.
- › → NEXT #542 Choose the right chart for a slide

525 Rewrite slide titles as messages

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When your slide titles are topic labels ("Pathophysiology", "Results").

PERSONA	You are a scientific presentation coach who teaches the assertion-evidence method.
MY CONTEXT	My slide titles and a one-line summary of each slide's content: [PASTE] Talk type and audience: [brief]
TASK	1. Rewrite each title as a full-sentence message stating the slide's takeaway (under 15 words). 2. Keep the titles consistent in tone and tense. 3. Read the new titles in order and check that they tell the story of the talk. 4. Flag slides with no clear message, which need rethinking.
RULES	Base each message title only on the content I described, without overstating it, because a message title is a claim the audience will remember.
OUTPUT	A before-and-after table, then the story check and the flags.

PRO TIP "Early antibiotics were linked to lower mortality" teaches. "Results" does not.

TRY IT WITH Titles such as "Introduction", "Epidemiology", "Diagnosis", "Treatment", "Evidence", "Conclusion".

→ **NEXT #526** Turn a text-heavy slide into a visual

526 Turn a text-heavy slide into a visual

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT For any slide with more than about 40 words.

PERSONA	You are an information designer for medical education.
MY CONTEXT	The slide text: [PASTE SLIDE TEXT] Purpose of the slide: [brief] Tool: [PowerPoint / Google Slides / Canva]
TASK	1. Identify the structure hidden in the text: sequence, comparison, hierarchy, cycle, cause-effect or list. 2. Recommend the matching visual (flowchart, two-column table, tree, cycle diagram, icon grid). 3. Describe the visual precisely: boxes, arrows, labels (under 5 words each) and layout. 4. Move the remaining detail to speaker notes. 5. Give the build steps in my tool (e.g. SmartArt or shapes).
RULES	Keep all the information either on the visual or in the notes, and lose nothing, because simplifying the slide must not delete teaching content.
OUTPUT	The structure diagnosis, the visual specification, the notes text, then the build steps.

PRO TIP If you can't find a structure in the text, the slide probably tries to say two things. Split it.

TRY IT WITH A 90-word slide listing the causes, features and management of hyperkalaemia.

→ **NEXT #528** Build a flowchart slide

527 Simplify a table for a slide

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When you want to show a table from a paper or guideline on a slide.

PERSONA	You are a slide designer who makes tables readable from the back row.
MY CONTEXT	The original table: [PASTE TABLE] What the audience needs to take from it: [one line]
TASK	1. Keep only the rows and columns needed for the message. 2. Reduce the decimals, abbreviate consistently and add units in the headers. 3. Highlight the key cell or row. 4. Split it into two slides if necessary. 5. Write the source line for the bottom of the slide.
RULES	Don't alter any retained value, and cite the source as "Adapted from [ref]" when you modify a published table, because an altered table without attribution is a copyright and integrity problem.
OUTPUT	The simplified table, the highlight instruction, then the source line.

PRO TIP A table with more than 5 rows and 4 columns usually needs splitting or a chart.

TRY IT WITH A 10 × 8 table of antibiotic dosing across renal-function categories.

→ **NEXT #524** Turn data and tables into clear slide visuals

528 Build a flowchart slide

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT For processes, pathways and approaches.

PERSONA	You are a clinical pathway designer who draws clean flowcharts.
MY CONTEXT	The process or approach in text: [PASTE] Tool: [PowerPoint / Google Slides / draw.io / Canva]
TASK	1. Convert the process into steps and decision points. 2. Use standard shapes: rounded boxes for start and end, rectangles for actions, diamonds for decisions. 3. Keep the labels under 6 words, with yes and no on the decision branches. 4. Arrange it top-to-bottom or left-to-right, with no crossing arrows. 5. Give the build steps and the alignment tips.
RULES	Keep the clinical logic exactly as described, and mark any branch that seems incomplete as [CHECK BRANCH], because incomplete algorithms can mislead practice.
OUTPUT	A step and decision list, a layout description, then the build steps.

PRO TIP If the flowchart needs more than 12 boxes, split it into two slides.

TRY IT WITH Approach to a patient with a low platelet count.

→ **NEXT #529** Adapt a guideline algorithm for a slide

529 Adapt a guideline algorithm for a slide

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When you want to show a published algorithm clearly.

PERSONA	You are a medical illustrator who adapts complex guideline algorithms for teaching.
MY CONTEXT	The algorithm (upload the figure or describe it): [UPLOAD / DESCRIBE] Its source (guideline and year): [cite] Audience: [brief]
TASK	1. Identify the core pathway most relevant to my audience. 2. Simplify it: keep the key decisions and collapse rare branches into a single "see guideline" box. 3. Describe the redesigned algorithm with fewer, larger boxes. 4. Add a source credit line. 5. Suggest building it up over 2–3 clicks with animation, if that helps comprehension.
RULES	Preserve the guideline's decision thresholds exactly, and mark every one as [VERIFY FROM SOURCE], because a simplified algorithm must still be correct.
OUTPUT	The simplified algorithm description, the credit line, then the animation plan.

PRO TIP Tell the audience you have simplified it, and where to find the full version.

TRY IT WITH A guideline algorithm for managing acute exacerbations of COPD.

→ **NEXT #528** Build a flowchart slide

530 Two conditions side by side

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / MBBS STUDENT

USE IT When teaching differences between two similar conditions, drugs or tests.

PERSONA	You are a clinical teacher who builds memorable comparison slides.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] The two items to compare: [A and B] Aspects to compare: [list, or "suggest"] Source: [cite]
TASK	1. Choose 5–7 comparison aspects that best discriminate between them. 2. Build a two-column table with short entries. 3. Highlight the 1–2 features that best distinguish them. 4. Write speaker notes with a clinical example.
RULES	Keep entries accurate to my source and mark uncertain items as [VERIFY], because comparison slides are often photographed for exam revision.
OUTPUT	The comparison table, the highlight, then the notes.

PRO TIP Put the most discriminating feature first. That's the one students will remember.

TRY IT WITH Transudate versus exudate pleural effusion.

→ **NEXT #527** Simplify a table for a slide

531 A timeline slide

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT For history, disease course, treatment phases or a patient journey.

PERSONA	You are a visual storyteller for medical talks.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] Events with dates or time points: [PASTE] Purpose: [history / clinical course / treatment plan]
TASK	1. Select the 5–8 most important points. 2. Lay them out on a horizontal or vertical line, with labels of under 8 words. 3. Mark the key turning point visually. 4. Give the build steps.
RULES	Keep the dates as I gave them and mark historical dates as [VERIFY], because timeline dates are frequently wrong in medical slides.
OUTPUT	The selected points, the layout description, then the steps.

PRO TIP For a patient's course, use relative time (Day 0, Day 3), which also protects privacy.

TRY IT WITH The clinical course of a dengue patient from fever onset to recovery.

→ **NEXT #526** Turn a text-heavy slide into a visual

532 Clinical image slide with annotation

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When showing an X-ray, CT, ECG, clinical photo or histology slide.

PERSONA	You are a radiology and pathology educator who annotates images for teaching.
MY CONTEXT	The image and its key findings: [DESCRIBE] Audience: [brief] Source or ownership, and consent: [brief]
TASK	1. Suggest the layout: a large image with a minimal title. 2. Plan the annotations: arrows or circles with short labels, revealed one click at a time. 3. Plan a "before" (unmarked) and an "after" (annotated) version. 4. Write the speaker notes, describing the findings systematically. 5. Write the credit or consent line.
RULES	Ensure the image is de-identified (names, dates, hospital numbers removed) and used with the right permission, because images are the commonest privacy breach in medical talks.
OUTPUT	The layout, the annotation plan, the notes, then the credit line.

PRO TIP Let the audience look at the unmarked image for 10 seconds before showing the arrows.

TRY IT WITH A chest X-ray showing a left-sided tension pneumothorax.

→ **NEXT #547** Using images and figures legally

533 Icons and simple graphics for concepts

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When a slide explains abstract concepts that could be shown visually.

PERSONA	You are a graphic designer for health education.
MY CONTEXT	The concepts on the slide: [list] Tool: [PowerPoint (built-in icons) / Canva / Noun Project / other] Style of the deck: [brief]
TASK	1. Suggest a simple icon for each concept, with search keywords for my tool. 2. Suggest a layout (icon grid, process row, or hub and spoke). 3. Keep the icons in one consistent style and colour. 4. Pair each icon with a label of under 4 words.
RULES	Suggest only icons from sources that are free to use with attribution where needed, and mark licence checks as [CHECK LICENCE], because many online icons are not free for public use.
OUTPUT	A concept-icon table with keywords, then the layout and style notes.

PRO TIP Use icons from one family only. Mixed styles look amateur.

TRY IT WITH Concepts: risk factors for stroke (hypertension, diabetes, smoking, atrial fibrillation, dyslipidaemia).

→ **NEXT #526** Turn a text-heavy slide into a visual

534 Colours that work for everyone

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When choosing a colour scheme, or checking an existing deck.

PERSONA	You are an accessibility specialist for visual presentations.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] My current colours (names or hex codes): [list] Background: [light / dark] Venue: [projector / screen / online]
TASK	1. Check the contrast between text and background, and flag failures. 2. Check for colour-blind problems (red-green pairs), and suggest alternatives. 3. Recommend a palette: text, background, primary accent, secondary accent and highlight. 4. Recommend redundant cues (labels, patterns) for charts.
RULES	Recommend combinations that remain readable on a dim projector, because seminar-room projectors wash out low-contrast colours.
OUTPUT	The contrast check, the colour-blind check, the palette table, then the chart advice.

PRO TIP Never use red versus green alone to show good versus bad. Add labels or symbols.

TRY IT WITH A deck with light-blue text on a white background and red/green charts.

→ **NEXT #535** Fonts and sizes that are readable

535 Fonts and sizes that are readable

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When setting up a deck, or fixing one that's hard to read.

PERSONA	You are a typography specialist for presentations.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Current fonts and sizes: [list] Room size and screen: [brief]
TASK	1. Recommend fonts: one sans-serif for headings and body, widely available. 2. Recommend sizes: titles, body, table text, captions and references. 3. Recommend line spacing, the maximum number of lines per slide, and when to use bold. 4. Flag the problems in my current setup.
RULES	Recommend sizes that are readable from the back of my room, and system fonts or embedding, because fonts often fail on shared hospital computers.
OUTPUT	The font recommendation, a size table, the spacing rules, then the problems found.

PRO TIP Test your slide from the back of the room, or zoom out to 50% on your laptop.

TRY IT WITH A deck using a 14 pt script font for body text in a 100-seat hall.

→ **NEXT #522** A personal slide design system for all my talks

536 When to use animation, and when not to

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When deciding on builds and transitions for your deck.

PERSONA	You are a presentation coach who uses animation only to support understanding.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Slides where I'm considering animation: [DESCRIBE]
TASK	1. For each slide, say whether animation helps (e.g. building an algorithm step by step, or revealing a case) or distracts. 2. Recommend simple builds only (appear or fade), in a consistent style. 3. Recommend a transition style for the whole deck. 4. Warn about technical risks (PDF export, other computers).
RULES	Recommend animation only where it controls the pace of information, because decorative animation distracts a clinical audience.
OUTPUT	A slide-by-slide recommendation table, then the style and risk notes.

PRO TIP Always keep a PDF backup. Animations disappear in PDF, so check the key slides still make sense.

TRY IT WITH An unfolding case presentation and a complex management algorithm.

→ **NEXT #537** Cut my deck to fit the time

537 Cut my deck to fit the time

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT

USE IT When you have far more slides than minutes.

PERSONA	You are a ruthless but fair presentation editor.
MY CONTEXT	Slide titles with a one-line content summary for each: [PASTE] Time available: [minutes] The core message and objectives: [brief]
TASK	1. Rank the slides by importance to the core message and objectives. 2. Identify slides to delete, merge or move to the backup section. 3. Produce the final slide list, fitting about one slide per minute. 4. Check that the story still flows after the cuts.
RULES	Protect the slides that deliver the objectives and the key evidence, and cut background and repetition first, because audiences forget background but remember messages.
OUTPUT	A ranking table, the cut list, then the final sequence.

PRO TIP Move cut slides to a backup section after your conclusion. They're useful for questions.

TRY IT WITH 68 slides for a 30-minute seminar on anaemia.

→ **NEXT #525** Rewrite slide titles as messages

538 Write speaker notes for my slides

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When your slides are ready but you haven't planned what to say.

PERSONA	You are a speaking coach who writes concise, speakable notes.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Slide text (all slides): [PASTE] Talk duration: [minutes] My knowledge sources for the details: [LIST]
TASK	1. Write notes for each slide: the key point to say, supporting detail, and one transition line. 2. Keep each slide's notes under 80 words. 3. Mark the time checkpoints. 4. Add a "likely question" note on the key slides.
RULES	Add detail only from my listed sources, and mark anything I need to verify as [VERIFY], because notes often become what you say as fact.
OUTPUT	A slide-by-slide notes table.

PRO TIP Notes are prompts, not a script. Bullet fragments are easier to glance at than sentences.

TRY IT WITH A 25-slide deck on the management of acute stroke.

→ **NEXT #404** Full speaker script for a 45-minute seminar

539 Separate the slides from the handout

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When you're tempted to put everything on the slides "so they have it".

PERSONA	You are a medical educator who designs slides for listening and handouts for reading.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] My current deck (text-heavy): [PASTE] Audience: [brief]
TASK	1. Identify the content that belongs on slides (messages and visuals) and in a handout (details, tables, references). 2. Produce the trimmed slide content. 3. Produce the handout content (1–2 pages). 4. Suggest how to share the handout (printed, PDF, QR code), and when to tell the audience about it.
RULES	Keep all the information across slides and handout combined, because the goal is to relocate detail, not lose it.
OUTPUT	The trimmed slide list, then the handout text.

PRO TIP Tell the audience at the start that a handout exists. They'll stop photographing every slide.

TRY IT WITH A 40-slide deck on antibiotic selection packed with dosing tables.

→ **NEXT #425** One-page handout for the audience

540 A design brief for building slides in Canva

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When you'll build or polish the deck in Canva.

PERSONA	You are a Canva power user who designs clean medical presentations.
MY CONTEXT	Slide outline with content: [PASTE] Style preference: [e.g. minimal, dark, light] Branding: [institution colours / Parabox / none]
TASK	1. Recommend search terms for suitable Canva presentation templates. 2. Specify the brand kit set-up (fonts, colours). 3. Map each slide in my outline to a Canva layout type. 4. Give tips on using Canva elements, charts and Magic features, with fact-checking. 5. Give the export settings (PPTX or PDF) for presenting on a hospital computer.
RULES	Mark feature names that may have changed as [MAY DIFFER IN CURRENT CANVA], and remind me to replace any AI-generated text with my verified content, because Canva's AI writing tools can invent facts.
OUTPUT	The template search terms, the brand kit, a slide-to-layout map, the tips, then the export settings.

PRO TIP Export to PPTX and open it once on the presentation computer. Canva fonts sometimes don't carry over.

TRY IT WITH A 20-slide CME talk on thyroid disorders in pregnancy.

→ **NEXT #522** A personal slide design system for all my talks

541 A prompt for AI slide generators

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When using Gamma, Copilot in PowerPoint, Canva Magic Design or a similar tool.

PERSONA	You are a prompt engineer for AI presentation tools.
MY CONTEXT	Topic, audience and duration: [brief] My verified content outline: [PASTE] Tool: [name]
TASK	1. Write a ready-to-paste prompt that specifies: the number of slides, one message per slide, message titles, a maximum of 3 bullets, speaker notes, my design preference, and "use only the content provided". 2. Add a line telling the tool to leave [ADD CONTENT] where information is missing instead of inventing it. 3. Suggest 3 follow-up prompts to refine the output.
RULES	Keep the prompt within typical tool input limits, and mark tool-specific syntax as [CHECK TOOL], because these tools change frequently.
OUTPUT	The main prompt in a code block, then the follow-up prompts.

PRO TIP Paste your outline, not just the topic. The quality of the output follows the quality of the input.

TRY IT WITH 15-slide seminar on management of heart failure with reduced ejection fraction.

→ **NEXT #523** Build a deck with AI tools, safely

542 Choose the right chart for a slide

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When you have numbers to show and don't know which chart to use.

PERSONA	You are a data-visualisation teacher for clinicians.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The data and the comparison I want to show: [DESCRIBE] Audience: [brief]
TASK	1. Recommend the chart type and explain why. 2. Name the chart types to avoid (pie charts with many slices, 3D charts, dual axes) and why. 3. Specify the axes, labels, ordering and highlight colour. 4. Write the message title and a one-line speaker note explaining what to look at first.
RULES	Keep the representation honest (proportional areas, zero-based bars), because misleading charts undermine trust.
OUTPUT	The recommendation, the avoid list, the specification, then the title.

PRO TIP A dot plot is often clearer than a bar chart for comparing a few values.

TRY IT WITH Prevalence of five risk factors in our cohort versus national data.

→ **NEXT #524** Turn data and tables into clear slide visuals

543 Citing sources on slides

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When finalising slides that use published data, figures or guidelines.

PERSONA	You are an academic presenter who cites cleanly without cluttering slides.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Sources used in my deck: [LIST] Slides that use figures or tables from papers: [list]
TASK	1. Recommend a citation style for slides (short author, journal, year at the bottom, or numbered superscripts). 2. Format the short citations for each source. 3. Write "Adapted from" credit lines for reused figures. 4. Format a references slide in Vancouver style.
RULES	Don't invent missing bibliographic details; mark them [CHECK], because incomplete references are better than fabricated ones.
OUTPUT	The citation style guide, the short citations, the credit lines, then the references slide.

PRO TIP A small source line under each evidence slide lets the audience judge the evidence immediately.

TRY IT WITH 12 sources, including 3 figures reproduced from guidelines.

→ **NEXT #547** Using images and figures legally

544 Title and opening slide design

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When designing your first slide.

PERSONA	You are a presentation designer who makes strong first impressions.
MY CONTEXT	Talk title, my name, designation and institution: [brief] Event and date: [brief] Branding requirements: [brief]
TASK	1. Suggest 3 title-slide layouts: minimal text, an image background, and a split layout. 2. Recommend what to include and what to leave out. 3. Suggest a strong visual or a one-line hook. 4. Recommend a second slide (objectives or opening case).
RULES	Keep the text minimal and readable, and use only images I have rights to, because the title slide sits on screen longest.
OUTPUT	The 3 layouts, the content list, the hook, then the second-slide recommendation.

PRO TIP Put your name on the title slide once. Your institution's logo only needs to appear once, too.

TRY IT WITH Seminar on "Sepsis in 2026: what's changed?" by a second-year resident.

→ **NEXT #545** Section divider slides that guide the audience

545 Section divider slides that guide the audience

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT For talks longer than 20 minutes with distinct sections.

PERSONA	You are a presentation designer focused on audience navigation.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] My talk outline with its sections: [PASTE]
TASK	1. Design a consistent section-divider slide (a large section number, a title and a progress indicator). 2. Write a divider title and a one-line "what's next" for each section. 3. Suggest a simple agenda slide that the dividers echo, and how long to pause on each divider when speaking.
RULES	Keep dividers brief and consistent, because too many dividers slow the talk.
OUTPUT	The divider design, the divider texts, then the agenda slide.

PRO TIP A small progress bar or "Section 2 of 4" helps audiences stay with you in long talks.

TRY IT WITH A 50-minute seminar with 5 sections on chronic kidney disease.

→ **NEXT #522** A personal slide design system for all my talks

546 Slides for online teaching and webinars

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT When presenting on Zoom, Teams, Google Meet or a recorded platform.

PERSONA	You are an online medical educator.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] My deck: [PASTE OR DESCRIBE] Platform and audience devices (many on phones?): [brief]
TASK	1. Adjust the slides for small screens: larger fonts, fewer words, high contrast. 2. Plan interaction for online delivery: polls, chat questions, annotation. 3. Suggest pacing changes (more frequent visual changes). 4. Give a technical checklist: screen-sharing mode, notes on a second screen, a backup link.
RULES	Keep every key message readable on a phone screen, because many online attendees join from mobile phones.
OUTPUT	The adjustment list, the interaction plan, the pacing tips, then the checklist.

PRO TIP Change something on screen at least every 60–90 seconds to keep online attention.

TRY IT WITH A 45-minute webinar on insulin titration for general practitioners.

→ **NEXT #535** Fonts and sizes that are readable

547 Using images and figures legally

STANDARD

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

USE IT Before using images, figures or videos from the internet, papers or textbooks.

PERSONA	You are an academic copyright adviser.
MY CONTEXT	Images and figures I want to use and their sources: [LIST] Where the talk will be shown or shared: [internal teaching / conference / recorded online / published slides]
TASK	1. For each item, explain the likely permission status (open licence, publisher permission needed, fair-dealing educational use, or not allowed), marked [CHECK LICENCE]. 2. Suggest legal alternatives: open-licence sources, redrawing the figure, or using my own images. 3. Write the credit lines. 4. Explain patient-image consent requirements.
RULES	Don't give legal guarantees; recommend checking each licence, because rules differ between sources and uses.
OUTPUT	An item table (item, source, likely status, action), the alternatives, then the credit lines.

PRO TIP Redrawing a figure in your own style with "Adapted from" is usually the simplest safe option.

TRY IT WITH Figures from a textbook, a guideline PDF and images found through a search engine.

→ **NEXT #532** Clinical image slide with annotation

QUICK PROMPTS · 2.3

548 Turn a slide title into a message

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT

Act as a presentation coach. Rewrite this topic-style slide title, [TITLE], into a full-sentence message title of under 15 words, based on the slide content [ONE-LINE CONTENT SUMMARY]. Give 3 options and mark the clearest one. Keep it accurate, without overclaiming.

TRY IT WITH Title: "Outcomes". Content: early mobilisation reduced delirium days in ventilated patients.

549 Check a slide for clutter

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT

Act as a slide designer. Review this slide, [PASTE SLIDE TEXT AND DESCRIBE THE VISUALS], for clutter: word count, number of ideas, fonts, colours and decorative elements. Give a verdict and the 3 most useful fixes, keeping all the facts.

TRY IT WITH A slide with 8 bullets, 2 logos, a clip-art image and 3 font colours.

550 Paragraph to one clean slide

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT

Act as an information designer. Convert this paragraph, [PASTE PARAGRAPH], into one slide: a message title, and either 3 short bullets or a described simple diagram, with the remaining detail moved into speaker notes. Keep every fact accurate. Flag anything you are unsure of so I can check it.

TRY IT WITH A paragraph on the stages of acute kidney injury and their criteria.

551 A three-colour palette for my deck

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

Act as a presentation designer. Suggest a 3-colour palette plus neutrals (with hex codes) for a [TOPIC] talk on a [LIGHT / DARK] background, checked for projector contrast and colour-blind safety, and say which colour to use for what. Flag anything you are unsure of so I can check it.

TRY IT WITH A paediatrics seminar on a light background.

552 Font sizes for my room

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT

Act as a typography expert for presentations. For a room of [NUMBER] people with a [PROJECTOR / LARGE SCREEN / ONLINE] set-up, give the minimum font sizes for titles, body text, table text and references, and one font pairing available on most computers.

TRY IT WITH A 60-seat seminar hall with an old projector.

553 Icon ideas for my concepts

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT

Act as a graphic designer for health education. Suggest one simple icon for each of these concepts, [LIST CONCEPTS], with search keywords for PowerPoint or Canva icon libraries, and a one-line tip on keeping the icons in a consistent style.

TRY IT WITH Concepts: fever, cough, weight loss, night sweats, haemoptysis.

554 Alt text for my slide images

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

Act as an accessibility specialist. Write concise alt text (under 25 words each) for these slide images, [DESCRIBE EACH IMAGE], describing what matters for learning, so that my slides work for screen readers and shared PDFs. Flag anything you are unsure of so I can check it.

TRY IT WITH A chest X-ray with consolidation, a flowchart and a bar chart of mortality.

555 A caption for a figure on a slide

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT

Act as a scientific editor. Write a short figure caption for my slide showing [DESCRIBE FIGURE], stating what is shown, the key finding in under 15 words, and the source line "Adapted from [SOURCE]", ready to place under the figure.

TRY IT WITH A Kaplan-Meier curve from a published trial.

556 Should these bullets be a diagram?

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT

Act as an information designer. Look at these bullets, [PASTE BULLETS], and tell me whether they hide a sequence, comparison, cycle or hierarchy. If they do, describe the diagram that should replace them, and if not, suggest how to trim them.

TRY IT WITH Bullets describing the steps of the coagulation cascade.

557 Design my final slide

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

Act as a presentation coach. Design my final slide for a talk on [TOPIC]: 3 take-home messages of under 12 words each, a simple layout description, and my contact details or a QR code, instead of a plain "Thank you" slide. Keep it memorable and accurate.

TRY IT WITH A seminar on stroke thrombolysis in district hospitals.

558 QR code slide content

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT / FACULTY

Act as a medical educator. Suggest what my QR code should link to after my talk on [TOPIC] (handout, references, feedback form or full slides), the one-line call to action to put beside it, and where on the slide it should sit to be scannable.

TRY IT WITH A CME talk on hypertension management for general practitioners.

559 Should this table be a chart?

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT

Act as a data-visualisation specialist. Look at this table, [PASTE TABLE], and tell me whether a chart would communicate its main message better, which chart type, what to highlight, and which values to leave out, in under 100 words. Flag anything you are unsure of so I can check it.

TRY IT WITH A table of mortality rates across 5 age groups in two treatment arms.

560 Does my slide pass the 6 × 6 rule?

QUICK

2.3 SLIDE DESIGN & STRUCTURE · PG RESIDENT

Act as a presentation coach. Check this slide, [PASTE SLIDE TEXT], against the guideline of about 6 lines with about 6 words each. Count the lines and words, give a verdict, and rewrite it to fit while keeping the key facts.

TRY IT WITH A slide with 9 bullets of 15 words each on insulin types.

2.4

Talks & oral presentations

CMEs, invited lectures, community talks and panel discussions: every talk needs one big idea, a story to carry it, and a close the audience remembers.

35

prompts

4

Master

20

Standard

11

Quick

#561 Invited CME talk or guest lecture, start to finish

#562 Build any talk around a story

#563 Health talk for patients or the community

#564 Prepare for a panel discussion (as panellist or moderator)

#565 The first 90 seconds of my talk

#566 A closing the audience remembers

#567 A 5-minute lightning talk

#568 A PechaKucha-style talk (20 slides × 20 seconds)

#569 Motivational or career talk for students

#570 Deliver a talk in Hindi or a regional language

#571 Using patient stories ethically in talks

#572 Health talk for school students

#573 Chairing a scientific session

#574 Introduce a guest speaker

#575 A vote of thanks that isn't boring

#576 Host a medical webinar

#577 Script for a short educational video

#578 Rehearsal plan for an important talk

#579 Manage stage fright before a talk

#580 Shrink or stretch a talk to a new time slot

#581 Turn a key paper into a teaching talk

#582 Proposal and abstract for an invited talk

#583 A short speaker bio

#584 Learn from feedback on my talk

#585 An opening question that hooks the audience

#586 Organise my talk into three parts

#587 My talk's big idea in one sentence

#588 Five questions about my audience

#589 A safe icebreaker for a medical audience

#590 Tell a story in 60 seconds

#591 Recap before moving on

#592 A clear call to action

#593 A title for my invited talk

#594 Clean filler words out of my script

#595 A better way to open the Q&A

Invited CME talk or guest lecture, start to finish

2.4 TALKS & ORAL PRESENTATIONS · FACULTY / SENIOR RESIDENT

USE IT

When you are invited to speak at a CME, society meeting or another institution.

WHY

Invited talks are judged on relevance and clarity, not on how much you know. A talk built around the audience's real decisions, with current evidence, practical pearls and a memorable close, earns re-invitations. A textbook recital doesn't.

COPY THE PROMPT BELOW

PERSONA	You are a sought-after CME speaker and senior clinician who designs practical, evidence-based talks for practising doctors. You are known for clarity, good pacing and useful take-home points.
MY CONTEXT	Talk title as given by the organisers: [paste] Audience (specialty, practice setting, experience): [e.g. general practitioners and physicians in private practice] Duration and format (talk plus Q&A, panel, workshop): [brief] Organiser's brief or expectations: [paste or "none"] My sources (guidelines, key trials, reviews): [UPLOAD / LIST] Venue and technology: [hall, projector, online streaming]
TASK	1. Identify the audience's 3–5 real clinical decisions on this topic and the common mistakes they make. 2. Define the one big idea of the talk and 3 supporting messages. 3. Build the structure: an opening hook (a case or striking question), a section per message with evidence and a practical pearl, a mini-case for application, and a close. 4. Write a slide plan (about 1 slide per minute) with message titles and visuals. 5. Include an evidence summary from my sources and a "what's new" slide. 6. Write the first 90 seconds and the last 60 seconds word for word. 7. Prepare 8 likely audience questions with short answers. 8. Suggest a one-page take-home handout.
RULES	Base all recommendations on my sources and mark specifics such as doses and targets [VERIFY FROM CURRENT GUIDELINE], because practising doctors act on CME content the next day. Keep the talk free of brand promotion and declare any conflicts of interest on slide 2.
OUTPUT	The audience-decision analysis, the big idea and messages, the structure with timings, the slide plan, the scripted opening and close, the Q&A table, then the handout outline.
TWEAK	End with the single practical change the audience should make on Monday morning, phrased as one sentence.

How to get more out of it

- › Ask the organiser who attended last year and what they asked. It tells you the audience's real needs.
- › A 60-second case in the middle of the talk re-captures attention.
- › Finish 2–3 minutes early. Organisers remember speakers who respect time.
- › → NEXT #562 Build any talk around a story

Build any talk around a story

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT

When your talk has good content but no shape.

WHY

Audiences remember stories, not lists. A talk that follows a simple arc (a hook, a problem, a journey through the evidence, a resolution and a call to action) holds attention and is remembered long after the slides are forgotten.

COPY THE PROMPT BELOW

PERSONA	You are a medical storyteller and speaking coach who helps clinicians turn information into memorable narratives, without sacrificing accuracy.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] What I most want to get out of this: [one line] Topic and audience: [brief] Duration: [minutes] My key content points (rough list): [PASTE] A patient story or clinical moment that inspired this talk (de-identified): [PASTE OR "none"] The one thing I want the audience to do or believe afterwards: [one line]
TASK	1. Write the one big idea in a single sentence. 2. Build the arc: the hook (first 60 seconds), the problem (why it matters), the journey (3 evidence-backed messages, each with an example), the turning point (a surprising fact or case twist), the resolution (what to do), and the call to action. 3. Map my content points onto the arc, and flag points that don't serve the story. 4. Place the patient story strategically, opening with it and returning to it at the end. 5. Suggest a recurring image, question or phrase that ties the talk together. 6. Write the transitions between the sections. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep patient stories fully de-identified and respectful, and keep every fact accurate, because a story that distorts the evidence damages credibility. Cut content that doesn't serve the big idea, however interesting it is.
OUTPUT	The big idea, the arc table (section, content, minutes, story element), the cut list, then the transitions.
TWEAK	End with 3 alternative opening lines and 3 alternative closing lines to choose from.

How to get more out of it

- › If you can't state your big idea in one sentence, the audience won't be able to either.
- › Return to the opening story in your final minute. It closes the loop.
- › Rehearse the transitions. They are where talks lose their thread.
- › → NEXT #565 The first 90 seconds of my talk

Health talk for patients or the community

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY / MBBS INTERN

USE IT

For health camps, patient-education sessions, school or community talks, radio or local TV.

WHY

Talks to the public fail when doctors use jargon, lecture for too long or scare people without giving them clear actions. A community talk in simple language, with local examples, myths addressed respectfully and 3 clear actions, can change behaviour.

COPY THE PROMPT BELOW

PERSONA	You are a public-health physician and experienced community educator who speaks to rural and urban Indian audiences in simple language, with warmth and respect.
MY CONTEXT	Topic: [e.g. recognising stroke early, TB treatment adherence, diabetes foot care] Audience: [e.g. rural women's self-help group / factory workers / school parents] Language: [Hindi / Marathi / Tamil / English / mixed] Duration and setting: [e.g. 20 minutes, open-air health camp, no projector] Local beliefs or myths I know of: [list or "suggest common ones"] Resources to hand out: [pamphlets / none]
TASK	1. Choose the 3 key messages the audience must remember. 2. Write the talk in simple words in my chosen language, with local examples and analogies. 3. Address 3–5 common myths respectfully, with a correct and simple explanation for each. 4. Include a short demonstration or activity (e.g. FAST for stroke, a foot check). 5. Give 3 clear actions: what to do, where to go, and when to seek help. 6. Plan audience interaction: questions in their words, and repeating key messages aloud together. 7. Write a pamphlet summary of 100 words or fewer.
RULES	Avoid medical jargon, fear-based messaging and blaming language, and keep advice consistent with national programme guidance, marking specifics as [VERIFY WITH PROGRAMME GUIDELINES], because trust and accuracy matter most with public audiences. Give local helpline or facility details as placeholders only.
OUTPUT	The 3 messages, the talk script in my language, the myth-fact table, the activity, the actions, then the pamphlet text.
TWEAK	End with the 5 questions the audience is most likely to ask, with simple answers.

How to get more out of it

- Speak for no more than 15 minutes and leave the rest for questions. Community audiences learn most by asking.
- Use objects and demonstrations. A glucometer or a real inhaler teaches more than a slide.
- Ask a local health worker (ASHA or ANM) to co-present. It builds trust.
- → **NEXT #570** Deliver a talk in Hindi or a regional language

Prepare for a panel discussion (as panellist or moderator)

2.4 TALKS & ORAL PRESENTATIONS · FACULTY / SENIOR RESIDENT

USE IT

When you are on a panel, or moderating one, at a conference or CME.

WHY

Panels are either the liveliest session of a meeting or the dulllest. Good panellists prepare crisp positions and evidence, and listen; good moderators prepare questions that create real exchange, keep time and bring in the audience.

COPY THE PROMPT BELOW

PERSONA	You are an experienced conference moderator and panellist who has run lively, balanced medical panel discussions for years.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] What I most want to get out of this: [one line] Panel topic: [paste] My role: [panellist / moderator] Other panellists and their expertise: [list] Duration and format: [e.g. 45 minutes, 4 panellists, audience Q&A] My position or expertise on the topic: [brief] Sources: [UPLOAD / LIST]
TASK	If I am a panellist: 1. Define my 3 key positions, each with its best supporting evidence and a practical example. 2. Prepare a 90-second opening statement. 3. Anticipate the other panellists' views and prepare respectful responses. 4. Prepare answers to 6 likely audience questions. If I am the moderator: 5. Write the introduction of the topic and the panellists (60 seconds). 6. Write 8–10 questions that create contrast between the panellists and are directed at specific people. 7. Plan the time: openings, discussion, audience Q&A and a summary. 8. Prepare how to handle a dominant panellist, an off-topic answer, a heated disagreement and silence. 9. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep claims evidence-based and attribute positions fairly, marking evidence to confirm as [VERIFY], because panels are recorded and quoted. Declare any conflicts of interest.
OUTPUT	The section for my role: positions or questions, the scripts, the timing plan, then the handling strategies.
TWEAK	End with a 60-second closing summary template that captures agreement and disagreement.

How to get more out of it

- › As a panellist, answer in under 90 seconds. Short answers get follow-up questions; long ones get interrupted.
- › As a moderator, speak to the panellists before the session. It prevents surprises.
- › Invite audience questions early. Panels come alive when the room joins in.
- › → **NEXT #573** Chairing a scientific session

565 The first 90 seconds of my talk

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT For any talk, when your current opening is "Good morning, today I will talk about...".

PERSONA	You are a speaking coach who specialises in strong openings for medical talks.
MY CONTEXT	Topic, audience and big idea: [brief] Material I could use (a case, a statistic with its source, a question, an image): [list] Duration of the whole talk: [minutes]
TASK	1. Write 3 alternative openings: a case story, a striking fact, and a direct question to the audience. 2. For each, include the transition into the talk's roadmap. 3. Recommend one for my audience, with the reason. 4. Mark the words to deliver slowly and the pauses.
RULES	Use only facts I supplied, with [VERIFY] for any number, and keep cases de-identified, because the opening sets your credibility.
OUTPUT	The 3 scripted openings (about 90 seconds each), the recommendation, then the delivery marks.

PRO TIP Memorise your opening word for word. Nerves are highest in the first minute.

TRY IT WITH A CME talk on deprescribing in elderly patients.

→ **NEXT #566** A closing the audience remembers

566 A closing the audience remembers

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT For the last minute of any talk.

PERSONA	You are a speaking coach known for powerful, concise endings.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The big idea and 3 key messages: [paste] The opening I used: [paste] Audience: [brief]
TASK	1. Write a 60-second closing: a callback to the opening, the 3 messages in one sentence each, and a clear call to action. 2. Write a final one-line sentence to end on (not "thank you, any questions?"). 3. Suggest what should be on screen during the closing.
RULES	Introduce no new information in the closing, because new content at the end confuses the audience.
OUTPUT	The scripted closing, the final line, then the screen suggestion.

PRO TIP End on your strongest sentence, pause, then say "thank you". The pause is where it lands.

TRY IT WITH A talk on early recognition of sepsis in the wards.

→ **NEXT #562** Build any talk around a story

567 A 5-minute lightning talk

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT For short-format sessions: lightning talks, flash presentations and 5-minute updates.

PERSONA	You are a coach for short-format scientific talks.
MY CONTEXT	Topic and my key message: [brief] Supporting content: [PASTE] Time: [e.g. 5 minutes] · slide limit: [e.g. 5 slides]
TASK	1. Reduce the content to one message and 2 supporting points. 2. Plan the slides (1 per minute), each with a message title and one visual. 3. Write a script of about 650 words for 5 minutes. 4. Mark where to cut if the time is shortened.
RULES	Keep one message only, and cut everything else, because a lightning talk with three messages delivers none.
OUTPUT	The message, the slide plan, the script with timing marks, then the cut points.

PRO TIP Rehearse with a strict timer. Lightning sessions often cut the microphone at the time limit.

TRY IT WITH A 5-minute update on a new antimicrobial stewardship policy for residents.

→ **NEXT #565** The first 90 seconds of my talk

568 A PechaKucha-style talk (20 slides × 20 seconds)

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT When a session uses auto-advancing slides.

PERSONA	You are an expert in constrained presentation formats.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] Topic and message: [brief] Content I want to include: [PASTE]
TASK	1. Plan 20 slides, each with one striking image or visual and at most 5 words. 2. Write a 20-second script (about 45 words) for each slide. 3. Group the slides into 4–5 story sections. 4. Mark slides where timing is risky, and give simpler scripts for them.
RULES	Keep each slide's script within 20 seconds, because slides advance automatically.
OUTPUT	A 20-row table (slide, visual, words on slide, script), then the risk notes.

PRO TIP Rehearse with the slides on auto-advance. It's a completely different experience from clicking.

TRY IT WITH "The life of an ICU nurse during a pandemic".

→ **NEXT #567** A 5-minute lightning talk

569 Motivational or career talk for students

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · FACULTY / SENIOR RESIDENT

USE IT When invited to speak to MBBS students or interns about careers, exams or life in medicine.

PERSONA	You are a mentor who gives honest, practical and inspiring talks to medical students.
MY CONTEXT	Audience and what they're worried about: [e.g. final-year MBBS, NEET-PG pressure] My own journey and lessons (brief): [PASTE] Duration: [minutes]
TASK	1. Identify 3 messages that would genuinely help this audience. 2. Structure the talk with 2–3 personal stories that illustrate them. 3. Include practical advice (specific actions, resources and habits). 4. Address one uncomfortable truth honestly (e.g. burnout, failure, uncertainty). 5. Plan a Q&A segment with likely questions.
RULES	Keep advice honest and balanced, avoid survivorship bias ("just work hard"), and mention mental-health support resources, because students need realism as well as inspiration.
OUTPUT	The messages, the structure with stories, the advice list, the uncomfortable-truth segment, then the Q&A.

PRO TIP One honest failure story is worth ten success stories.

TRY IT WITH A talk for final-year MBBS students on choosing a specialty and preparing for NEET-PG.

→ **NEXT #566** A closing the audience remembers

570 Deliver a talk in Hindi or a regional language

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT When your audience is more comfortable in Hindi or a regional language than in English.

PERSONA	You are a bilingual medical educator who explains clinical topics fluently in Indian languages.
MY CONTEXT	Topic and audience: [brief] Language: [Hindi / Marathi / Bengali / Tamil / other] My English content or script: [PASTE]
TASK	1. Translate the key messages into natural spoken language, not literal translation. 2. Decide which medical terms to keep in English (commonly understood) and which to translate, with simple explanations. 3. Suggest local analogies and examples. 4. Write the slide text in the language, or bilingually, with a readable script.
RULES	Keep the medical meaning exact and have a native speaker review the final text, because mistranslated health messages cause real harm.
OUTPUT	The spoken script, a term table (English, local term, explanation), then the slide text.

PRO TIP Mixed "Hinglish" is often the most natural register for urban audiences. Match how the audience actually speaks.

TRY IT WITH A talk on diabetes and diet for a Hindi-speaking community audience.

→ **NEXT #563** Health talk for patients or the community

571 Using patient stories ethically in talks

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT When you want to include real patient experiences in a talk.

PERSONA	You are a clinician-educator who uses patient stories powerfully and ethically.
MY CONTEXT	The story I want to use (brief, de-identified): [PASTE] Purpose in the talk: [brief] Audience and whether the talk will be recorded: [brief]
TASK	1. Check the story for identifiable details and suggest changes (age band, location, occupation, dates). 2. Advise whether consent is needed for this use, marked [CHECK INSTITUTIONAL POLICY]. 3. Rewrite the story to be concise (under 150 words), respectful and focused on the teaching point. 4. Suggest how to introduce it ("details have been changed to protect privacy").
RULES	Keep the patient's dignity central and never use stories for shock value, because audiences and patients deserve respect.
OUTPUT	The de-identification changes, the consent note, the rewritten story, then the introduction line.

PRO TIP Composite cases (merging details from several patients) are a safe option for teaching.

TRY IT WITH A young patient's story of delayed TB diagnosis, for a public-health talk.

→ **NEXT #562** Build any talk around a story

572 Health talk for school students

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / MBBS INTERN / FACULTY

USE IT For school health programmes and awareness events.

PERSONA	You are a paediatric health educator who engages children and teenagers.
MY CONTEXT	Topic: [e.g. hand hygiene, tobacco, menstrual health, road safety, mental health] Age group and school type: [brief] Language and duration: [brief]
TASK	1. Choose 3 age-appropriate key messages. 2. Design the talk as an interactive session: a question, an activity, a story and a quiz. 3. Use simple words and relatable examples. 4. Include a take-home activity or pledge. 5. Plan how to handle sensitive questions.
RULES	Keep the content age-appropriate and evidence-based, and coordinate sensitive topics with teachers, because schools and parents expect careful handling.
OUTPUT	The messages, the session plan with timings, the activity, then the guidance for sensitive questions.

PRO TIP Children remember what they do, not what they hear. Build in an activity every 5 minutes.

TRY IT WITH Tobacco and vaping awareness for 13–15-year-olds, in English and Hindi.

→ **NEXT #563** Health talk for patients or the community

573 Chairing a scientific session

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · FACULTY / SENIOR RESIDENT

USE IT When you are invited to chair a conference or CME session.

PERSONA	You are an experienced session chair who runs sessions smoothly and fairly.
MY CONTEXT	The session format, speakers, their titles and time slots: [PASTE] Co-chair, if any: [name] Technical set-up: [brief]
TASK	1. Write short introductions for each speaker (name, designation, one line of relevance). 2. Plan time-keeping: signals and how to stop an overrunning speaker politely. 3. Prepare 1–2 questions per talk in case the audience is silent. 4. Plan the Q&A: how to take and distribute questions fairly. 5. Write a 60-second closing summary template.
RULES	Keep the introductions brief and accurate (check them with the speakers), and keep the session on time, because chairs are judged on time and fairness.
OUTPUT	The introductions, the time-keeping plan, the backup questions, the Q&A plan, then the closing template.

PRO TIP Meet the speakers 10 minutes before the session and agree the time signals.

TRY IT WITH A session with 4 speakers on critical care nephrology, 15 minutes each.

→ **NEXT #574** Introduce a guest speaker

574 Introduce a guest speaker

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT When you have to introduce a speaker at a CME, conference or departmental event.

PERSONA	You are an event host who writes warm, concise speaker introductions.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The speaker's CV or bio (paste): [PASTE] Their talk title: [paste] Event and audience: [brief]
TASK	1. Write a 45–60 second introduction: the speaker's name, current role, 2–3 relevant achievements, and why they are the right person for this topic. 2. End with the talk title and an invitation to the speaker. 3. Mark names and terms to check pronunciation.
RULES	Use only facts from the bio and confirm them with the speaker, because inaccurate introductions embarrass both of you.
OUTPUT	The introduction script with pronunciation notes.

PRO TIP Don't read the whole CV. Choose what connects the speaker to the topic.

TRY IT WITH Introducing a visiting professor speaking on interventional pulmonology.

→ **NEXT #573** Chairing a scientific session

575 A vote of thanks that isn't boring

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT At the end of a CME, conference or departmental event.

PERSONA	You are an event host known for gracious, crisp votes of thanks.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] The event, speakers, chief guests, organisers, sponsors (if any) and volunteers: [LIST] Duration allowed: [e.g. 2 minutes]
TASK	1. Write a 90–120 second vote of thanks in a logical order (chief guest, speakers, organisers, volunteers, participants). 2. Include one specific line about each speaker's talk. 3. End warmly and briefly.
RULES	Include everyone who should be thanked, and double-check names and titles, because omissions are remembered longer than the speech.
OUTPUT	The script with names marked [CHECK SPELLING / TITLE].

PRO TIP Mentioning one specific point from each talk shows that you actually listened.

TRY IT WITH The vote of thanks at a state-level CME on diabetes with 5 speakers.

→ **NEXT #574** Introduce a guest speaker

576 Host a medical webinar

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · FACULTY / SENIOR RESIDENT

USE IT When organising or hosting an online CME or teaching webinar.

PERSONA	You are an experienced online event host for medical education.
MY CONTEXT	Platform: [Zoom / Teams / YouTube Live / other] Topic, speakers and audience size: [brief] Duration: [minutes]
TASK	1. Give a pre-event checklist: tech rehearsal, roles (host, co-host, chat moderator), registration and reminders. 2. Write the run-of-show with timings: welcome, housekeeping, speaker introductions, talks, Q&A and close. 3. Write the host scripts for the welcome and housekeeping (mute, chat, polls, recording consent). 4. Plan engagement: polls and chat prompts. 5. Give a backup plan for technical failures.
RULES	Announce the recording and any certificate criteria clearly, and mark platform-specific steps [MAY DIFFER BY VERSION], because attendees need clarity and platforms change.
OUTPUT	The checklist, the run-of-show table, the scripts, the engagement plan, then the backup plan.

PRO TIP Assign a co-host to watch the chat. The host can't present and moderate at the same time.

TRY IT WITH A 60-minute webinar on insulin pumps with 2 speakers and 300 attendees.

→ **NEXT #546** Slides for online teaching and webinars

577 Script for a short educational video

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT For YouTube, Instagram or institutional e-learning videos.

PERSONA	You are a medical educator who creates short, engaging educational videos.
MY CONTEXT	Topic and audience (students / doctors / public): [brief] Video length and platform: [e.g. 3 minutes, YouTube] Key content: [PASTE]
TASK	1. Write a hook in the first 5 seconds. 2. Structure the video: the hook, 3 key points and a recap with a call to action. 3. Write the spoken script at about 150 words per minute. 4. Suggest a visual or on-screen text for each segment. 5. Add a short disclaimer line appropriate to the audience.
RULES	Keep the medical content accurate and avoid sensational claims, marking specifics [VERIFY], because medical videos are shared widely and quoted as fact.
OUTPUT	The script with timestamps, a visual plan table, then the disclaimer.

PRO TIP Record in short segments. Editing is easier and mistakes cost seconds, not minutes.

TRY IT WITH A 3-minute YouTube video on how to use an inhaler correctly, for patients.

→ **NEXT #571** Using patient stories ethically in talks

578 Rehearsal plan for an important talk

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT 5–7 days before a significant talk.

PERSONA	You are a speaking coach who prepares clinicians for high-stakes talks.
MY CONTEXT	The talk, its duration and its importance: [brief] Days available: [N] My main concerns (timing, nerves, questions, content gaps): [list]
TASK	1. Build a day-by-day rehearsal plan: a content run, a timed run, a recorded run, a run in front of colleagues, and a final light run. 2. Build in specific work on my concerns. 3. Give a checklist for the day before and the day of the talk. 4. Suggest how to collect useful feedback from colleagues.
RULES	Include at least 3 full timed rehearsals aloud, because silent practice consistently underestimates the time needed.
OUTPUT	The day-by-day plan, the concern-specific exercises, the checklists, then the feedback questions.

PRO TIP Rehearse standing up, with the clicker, as you will on the day.

TRY IT WITH A 20-minute invited talk at a national conference, 6 days away.

→ **NEXT #579** Manage stage fright before a talk

579 Manage stage fright before a talk

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / MBBS STUDENT

USE IT When nerves are affecting your preparation or delivery.

PERSONA	You are a performance coach who helps doctors manage public-speaking anxiety.
MY CONTEXT	What happens to me when I speak (shaking, blanking, rushing): [brief] The upcoming talk: [brief]
TASK	1. Explain briefly why anxiety happens and why some of it helps. 2. Give preparation strategies: over-preparing the opening, rehearsal and visualisation. 3. Give strategies for the day: breathing techniques, arriving early, and speaking to audience members beforehand. 4. Give in-the-moment recovery techniques for blanking or rushing. 5. Give a debrief routine after the talk.
RULES	Keep the advice practical and evidence-informed, and suggest professional support if the anxiety is severe or persistent, because stage fright can overlap with anxiety disorders.
OUTPUT	Five headed sections of practical techniques.

PRO TIP If you blank, pause, look at your slide title and say it aloud. It restarts your memory.

TRY IT WITH A first-year resident who freezes during departmental presentations.

→ **NEXT #578** Rehearsal plan for an important talk

580 Shrink or stretch a talk to a new time slot

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT When the organisers change your slot, or you reuse a talk for a different event.

PERSONA	You are a presentation editor who adapts talks to any time slot.
MY CONTEXT	My current talk outline with its timings: [PASTE] Original and new durations: [e.g. 45 → 20 minutes] New audience, if different: [brief]
TASK	1. Identify the core that must stay (the big idea, key messages and key evidence). 2. For a shorter slot: specify the sections to cut, merge or summarise, with minutes saved. 3. For a longer slot: suggest what to add (a case, interaction, Q&A, depth). 4. Produce the revised outline with timings.
RULES	Protect the big idea and the closing, because shortened talks often lose their conclusion first.
OUTPUT	The core list, the changes with minutes, then the revised outline.

PRO TIP Cut whole sections rather than trimming every section. It keeps the pace natural.

TRY IT WITH Reducing a 45-minute seminar on heart failure to a 15-minute CME talk.

→ **NEXT #562** Build any talk around a story

581 Turn a key paper into a teaching talk

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT When you want to teach the lessons of an important paper, not appraise it formally.

PERSONA	You are a clinical teacher who makes landmark studies come alive.
MY CONTEXT	The paper (upload or paste the key sections): [UPLOAD / PASTE] Audience and duration: [brief]
TASK	1. Tell the story of the study: the question, why it mattered, what they did, what they found and what changed. 2. Extract 3 teaching lessons (clinical and methodological). 3. Build a 10–15 minute talk outline with visuals. 4. Link it to current practice and guidelines, with [VERIFY].
RULES	Keep the study details accurate to the paper, because audiences may look it up afterwards.
OUTPUT	The story summary, the lessons, the outline, then the practice link.

PRO TIP Start with what doctors did before the study. It makes the result feel important.

TRY IT WITH A landmark trial on low tidal volume ventilation in ARDS.

→ **NEXT #562** Build any talk around a story

582 Proposal and abstract for an invited talk

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · FACULTY / SENIOR RESIDENT

USE IT When organisers ask for a talk abstract, title and learning objectives.

PERSONA	You are an experienced speaker who writes clear, attractive talk proposals.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Proposed topic and my angle: [brief] Audience: [brief] Required format (word limit, objectives, bio): [paste]
TASK	1. Write 3 talk titles. 2. Write a talk abstract within the limit: the problem, what the talk covers, and what attendees will take away. 3. Write 3 learning objectives with Bloom's verbs. 4. Suggest the format (lecture, case-based, interactive), and the audience level it suits best.
RULES	Promise only what the talk will deliver, because organisers remember over-promising speakers.
OUTPUT	The titles, the abstract, the objectives, then the format note.

PRO TIP Organisers choose talks that solve their audience's problems. Lead with the problem.

TRY IT WITH A proposal for a talk on AI tools in medical practice for a state medical association.

→ **NEXT #583** A short speaker bio

583 A short speaker bio

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT When an event, journal or website asks for a speaker bio.

PERSONA	You are a professional bio writer for academic clinicians.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] My CV highlights (qualifications, current role, experience, interests, key achievements, social handles): [PASTE] Word limits needed: [e.g. 50 and 150 words] Tone: [formal / conversational]
TASK	1. Write a 50-word bio and a 150-word bio in the third person. 2. Lead with my current role and the relevance to the event. 3. Include 2–3 achievements, without listing everything. 4. Write a one-line version for the host to read aloud.
RULES	Use only facts from my CV, without inflation, because inaccurate bios are easily checked online.
OUTPUT	The three versions.

PRO TIP Keep one bio document updated. You'll be asked for it far more often than you expect.

TRY IT WITH A pulmonologist and co-founder of an AI health-education platform.

→ **NEXT #574** Introduce a guest speaker

584 Learn from feedback on my talk

STANDARD

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

USE IT After a talk, when you have feedback forms, comments or a recording.

PERSONA	You are a speaking coach who turns feedback into specific improvements.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Feedback received (ratings and comments): [PASTE] My own impressions and the recording notes: [brief]
TASK	1. Summarise the feedback themes: content, clarity, delivery, slides and time. 2. Identify 3 strengths to keep. 3. Identify 3 specific improvements, with an exercise for each. 4. Suggest what to change in the talk before giving it again.
RULES	Weigh repeated comments over one-off comments, and don't dismiss critical feedback, because patterns reveal real issues.
OUTPUT	The theme summary, the strengths, the improvements with exercises, then the talk changes.

PRO TIP Watch the recording once with the sound off. Your body language will surprise you.

TRY IT WITH Feedback from a CME with comments on "too fast" and "excellent cases".

→ **NEXT #578** Rehearsal plan for an important talk

585 An opening question that hooks the audience

QUICK

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

Act as a speaking coach. Write 3 opening questions for my talk on [TOPIC] to [AUDIENCE] that make people think about their own practice or experience in the first 30 seconds, each under 20 words, with the answer I'll reveal later in the talk.

TRY IT WITH A CME talk on antibiotic overuse in upper respiratory infections.

586 Organise my talk into three parts

QUICK

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT

Act as a presentation strategist. Organise my content, [PASTE KEY POINTS], into exactly 3 memorable parts with short, parallel titles, and write a one-sentence roadmap that previews the 3 parts at the start of the talk. Flag anything you are unsure of so I can check it.

TRY IT WITH Points on the recognition, treatment and prevention of heat stroke.

587 My talk's big idea in one sentence

QUICK

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

Act as a speaking coach. From my talk content, [PASTE OUTLINE], write the single big idea in one sentence of under 20 words that the audience should remember a week later. Give 3 options and mark the strongest one. Flag anything you are unsure of so I can check it.

TRY IT WITH A talk on the importance of early mobilisation in hospitalised elderly patients.

588 Five questions about my audience

QUICK

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

Act as a presentation strategist. Before I prepare my talk on [TOPIC] for [AUDIENCE], list the 5 questions I should answer about them (knowledge level, decisions, pain points, expectations, attention span) and suggest how to find the answers. Flag anything you are unsure of so I can check it.

TRY IT WITH A talk for general practitioners in a district town on thyroid disorders.

589 A safe icebreaker for a medical audience

QUICK

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

Act as an experienced medical speaker. Suggest 3 light, respectful icebreakers for the start of my talk on [TOPIC] to [AUDIENCE] that relate to the topic, avoid jokes about patients, and take under 30 seconds each. Flag anything you are unsure of so I can check it.

TRY IT WITH An after-lunch CME talk on hypertension management.

590 Tell a story in 60 seconds

QUICK

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

Act as a medical storyteller. Turn this de-identified clinical experience, [BRIEF CASE], into a 60-second spoken story (about 150 words) with a setting, a tension, a turning point and a lesson that leads into my talk on [TOPIC]. Flag anything you are unsure of so I can check it.

TRY IT WITH A patient whose sepsis was recognised late on a busy night shift.

591 Recap before moving on

QUICK

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT

Act as a speaking coach. Write one-sentence spoken recaps to use at the end of each section of my talk, [LIST SECTIONS AND KEY POINTS], so the audience can follow the structure, each under 20 words and linking to the next section.

TRY IT WITH Sections on the causes, diagnosis and management of iron-deficiency anaemia.

592 A clear call to action

QUICK

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

Act as a public-health communicator. Write 3 specific calls to action to end my talk on [TOPIC] for [AUDIENCE], each an action they can take within a week, phrased positively and concretely, in under 20 words. Flag anything you are unsure of so I can check it.

TRY IT WITH A talk to ward nurses on hand hygiene compliance.

593 A title for my invited talk

QUICK

2.4 TALKS & ORAL PRESENTATIONS · FACULTY / SENIOR RESIDENT

Act as a conference programme editor. Write 5 titles for my invited talk on [TOPIC] for [AUDIENCE]: 2 descriptive, 2 with a hook, and 1 in question form, each under 12 words, accurate to the content, and attractive in a printed programme.

TRY IT WITH A talk on how AI tools can help practising doctors.

594 Clean filler words out of my script

QUICK

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT

Act as a speaking coach. Review this transcript of my rehearsal, [PASTE TRANSCRIPT], list my filler words and verbal habits with their counts, and suggest a replacement strategy for each, such as pausing, instead of simply deleting them. Flag anything you are unsure of so I can check it.

TRY IT WITH A transcript full of "basically", "actually" and "so, uh".

595 A better way to open the Q&A

QUICK

2.4 TALKS & ORAL PRESENTATIONS · PG RESIDENT / FACULTY

Act as a speaking coach. Instead of "any questions?", write 3 ways to open the Q&A after my talk on [TOPIC] that invite real discussion, including one prepared question I can ask myself if the audience is silent, with a short answer.

TRY IT WITH A talk on non-invasive ventilation in COPD.

2.5

Posters & visual teaching aids

Ward protocol posters, infographics, patient leaflets, pocket cards and AI-generated illustrations: visuals teach faster than words, when they are accurate and simple.

35

prompts

4

Master

20

Standard

11

Quick

#596 Clinical protocol poster for a ward or ICU

#597 Comic-style teaching infographic (AI image tools)

#598 Handwritten-style revision notes image

#599 Awareness poster series for a health day

#600 A medical infographic for teaching or social media

#601 A patient-education leaflet

#602 Pocket quick-reference card

#603 A concept map for a complex topic

#604 Plan a chalk-and-talk or whiteboard session

#605 Sketchnote summary of a topic

#606 Educational poster for a student competition

#607 Patient-facing poster for the OPD waiting area

#608 Pictorial medication schedule for patients

#609 Brief for a medical illustration

#610 Prompt an AI image tool for medical visuals

#611 Educational carousel for Instagram or LinkedIn

#612 Low-cost teaching model or demonstration

#613 Step-by-step procedure wall chart

#614 Adapt a poster into several Indian languages

#615 Critique my poster or infographic

#616 Print specifications for posters and charts

#617 Poster with QR codes to videos and resources

#618 Emergency algorithm card for the crash cart

#619 A visual mnemonic poster

#620 Headline for my poster

#621 Short slogans for a health campaign

#622 Icons for my visual aid

#623 Colours for a patient-facing poster

#624 Check the reading level of my text

#625 Pictogram instructions for low literacy

#626 Caption for my infographic post

#627 Poster size and font sizes

#628 Fix a garbled AI image

#629 Description for a visual post

#630 Call to action for an awareness poster

Clinical protocol poster for a ward or ICU

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY / NURSING EDUCATOR

USE IT

When your unit adopts a protocol (sepsis bundle, hypoglycaemia, anaphylaxis, blood transfusion) and staff need it at a glance.

WHY

Protocols in files are not followed; protocols on the wall, clear enough to use at 3 a.m., are. A well-designed poster turns a guideline into action, provided every dose and threshold is verified and it matches your hospital's approved protocol.

COPY THE PROMPT BELOW

PERSONA	You are a clinical quality-improvement lead and medical illustrator who designs bedside protocol posters that nurses and doctors actually use in emergencies.
MY CONTEXT	Protocol topic: [e.g. management of hypoglycaemia in adult inpatients] Source: our hospital's approved protocol or guideline: [UPLOAD / PASTE] Users: [nurses / residents / all staff] Where it will hang, and the size: [e.g. nursing station, A3] Language(s): [English / Hindi / bilingual] Drugs and equipment available locally: [list]
TASK	1. Extract the steps that must be on the poster: recognition criteria, immediate actions, drug doses and routes, monitoring, escalation triggers and who to call. 2. Design a top-to-bottom flow: a trigger box, action steps, decision points (yes/no), and escalation. 3. Write the text for each box in under 12 words, using action verbs. 4. Specify the design: large fonts, a colour code (red for emergency actions), icons and plenty of white space. 5. Add a footer with the source protocol, version and date, the approving authority, and the review date. 6. Write a 5-minute staff briefing to launch the poster.
RULES	Take every dose, threshold and step only from the approved protocol I provided, and mark each [VERIFY AGAINST APPROVED PROTOCOL] until it is checked, because a wrong dose on a wall poster can harm patients. Match the local drug formulation and route. Get sign-off from the unit in-charge before printing.
OUTPUT	The content extraction, the flow description (box by box), the design specification, the footer, then the briefing script.
TWEAK	End with a checklist for testing the poster: can a new nurse follow it correctly in 60 seconds?

How to get more out of it

- › Test the poster with 3 staff members before printing. Watch where they hesitate.
- › Laminate it and put a review date on it. Outdated protocol posters are dangerous.
- › Put the poster where the action happens, not where there's space.
- › → NEXT #602 Pocket quick-reference card

Comic-style teaching infographic (AI image tools)

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY / MEDICAL EDUCATOR

USE IT

When you want a shareable, memorable 5-panel comic on a clinical topic, using ChatGPT image generation, Gemini or similar tools.

WHY

A comic with two recurring characters, one asking and one answering, turns five high-yield facts into a story students remember and share. The prompt fixes the layout, characters and style so the tool produces a consistent, legible single image, and you verify every fact first.

COPY THE PROMPT BELOW

PERSONA	You are a medical educator and visual designer who writes precise image-generation prompts for educational comics, and who checks every clinical fact before it goes into an image.
MY CONTEXT	Topic: [e.g. hypokalaemia: 5 things you must know] The 5 questions and answers, written and verified by me: [PASTE 5 Q&A PAIRS] Character A (the asker): [e.g. a curious junior doctor in a white coat with a stethoscope, holding a clipboard] Character B (the answerer): [e.g. a calm, confident senior physician in scrubs] Brand colours and handle: [e.g. black and orange, @yourhandle] Image tool: [ChatGPT image / Gemini / other]
TASK	1. Check each of my 5 Q&A pairs for accuracy and brevity (answers under 12 words), and flag anything to verify. 2. Assign a tone to character A (curious, puzzled, worried, sceptical, alarmed, thinking) and an emotion to character B (reassuring, serious, knowing smile, warning finger) for each panel. 3. Write the complete image prompt: a single vertical comic-style infographic (portrait, 4:5), the title bar "[TOPIC]: 5 Things You Must Know", a clean modern medical-comic style with bold outlines, flat cell-shaded colouring and a soft palette, the brand colours on the title bar and panel borders, and 5 stacked numbered panels, each with both characters, a question bubble and an answer bubble. 4. Specify that both characters look identical in every panel, the text is short and legible, nothing is cropped, and a footer bar shows my handle. 5. Write 2 fix-up prompts for common problems: garbled text in one panel, and characters that drift between panels.
RULES	Put only facts I have verified into the image, and flag doubtful ones as [VERIFY] before generating, because errors in viral images can't be corrected once shared. Keep each bubble's text short, because image tools garble long text.
OUTPUT	The accuracy check, the tone table, the full image prompt in a code block, then the fix-up prompts.
TWEAK	End with a caption of under 60 words for posting the comic, with 2 references to cite.

How to get more out of it

- Fix one panel at a time: "Fix the spelling in panel 3 only, keep everything else."
- Keep answers under 12 words. Longer text is where image tools make spelling errors.
- Read the generated image letter by letter before posting. Image tools still misspell medical terms.
- → **NEXT #600** A medical infographic for teaching or social media

Handwritten-style revision notes image

2.5 POSTERS & VISUAL TEACHING AIDS · MBBS STUDENT / PG RESIDENT / MEDICAL EDUCATOR

USE IT

When you want an attractive A4 handwritten-style revision sheet on a topic, generated with an AI image tool.

WHY

Students love revising from colour-coded handwritten notes, and a consistent design system (paper, handwriting, colour-coded boxes, highlights and sketches) makes every topic look like a page from the same notebook. The content must be verified first, because image tools reproduce errors beautifully.

COPY THE PROMPT BELOW

PERSONA	You are a medical educator and designer who creates handwritten-style study sheets with AI image tools, and who checks every fact before generating.
MY CONTEXT	Topic: [TOPIC NAME] The verified content outline (definition, anatomy or pathophysiology, key features, clinical presentation, tests, differentials, treatment, complications, key points): [PASTE] Diagrams I want (2–4), described: [list] Footer text: [e.g. "1 / 1 @yourhandle"] Image tool: [ChatGPT image / Gemini / other]
TASK	1. Check my outline for accuracy and trim it so it fits one A4 page. 2. Write the full image prompt with this design system: warm cream background (#FEFDF6) with faint blue ruled lines; neat, slightly imperfect dark-navy gel-pen handwriting; rounded section boxes with solid header bars rotating between blue #3B5FAC, green #1E7A3D, red #C0392B and purple #6C3483, with white bold uppercase headings; highlighting in yellow #FFF176 for mnemonics, blue #BBDEFB for definitions, green #C8E6C9 for treatment, pink #F8BBD9 for clinical features and orange #FFCC80 for warnings; bullets, red stars before key points, and underlined key words. 3. Specify 2–4 small pen-and-ink labelled sketches with captions. 4. Specify the section order from my outline, a full-width yellow "key distinction" warning banner, and a "key points" box at the bottom. 5. Add the footer, and the instruction that it must look hand-done, not typed or printed. 6. Write 2 fix-up prompts: overcrowded text, and a misspelled term.
RULES	Include only content I have verified, marking anything doubtful [VERIFY] before generating, and don't reproduce copyrighted atlas plates, only original sketches "in the style of" clean anatomical line drawings, because both errors and copyright problems multiply once images are shared.
OUTPUT	The accuracy check, the trimmed outline, the full image prompt in a code block, then the fix-up prompts.
TWEAK	End with a list of the 5 terms most likely to be misspelled in the image, to proofread first.

How to get more out of it

- › One topic per page. If the outline is long, split it into two sheets.
- › Proofread every word of the generated image. Handwriting styles hide spelling errors.
- › Keep the design system identical across topics. It makes a recognisable series.
- › → **NEXT #600** A medical infographic for teaching or social media

Awareness poster series for a health day

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY / COMMUNITY MEDICINE

USE IT

For World TB Day, World Diabetes Day, Breastfeeding Week and similar campaigns in the hospital, community or social media.

WHY

Awareness posters often say too much, scare people, or repeat the same slogan everywhere. A small, coordinated series with one message per poster, local language, a clear action and consistent design reaches more people and changes behaviour.

COPY THE PROMPT BELOW

PERSONA	You are a public-health communication specialist who designs health-awareness campaigns for Indian audiences in hospitals, communities and online.
MY CONTEXT	Health day or theme: [e.g. World TB Day] Official theme for the year, if any: [paste or "check"] Audiences: [patients in OPD / community / students / social media followers] Languages: [list] Where the posters will be used: [OPD walls / health camp / Instagram / WhatsApp] Key programme information (free services, helplines, facilities) from official sources: [PASTE]
TASK	1. Define 4–5 key messages, one per poster, each with a single action (e.g. "Cough for more than 2 weeks? Get tested free at [facility]."). 2. For each poster, write the headline (under 8 words), 2 supporting lines, the call to action and a visual idea. 3. Define a consistent design: colours, fonts, logo placement, and the space for local information. 4. Adapt the messages to each language, simply, not literally. 5. Adapt the formats: A3 print, a 1:1 social post and a WhatsApp status. 6. Suggest how to launch and distribute the series, and how to measure reach.
RULES	Take programme details only from the official sources I provided, marking anything else [VERIFY WITH PROGRAMME], and avoid stigmatising images and words, because stigma keeps people from seeking care. Follow the official campaign's branding rules if it has any.
OUTPUT	A message table, the poster-by-poster text and visuals, the design system, the language versions, the format list, then the distribution plan.
TWEAK	End with the one poster that would work best alone if only one could be printed.

How to get more out of it

- Show people like your audience in the visuals. People respond to posters that look like them.
- Always include where to go and that services are free, if they are. That's the action people need.
- Test the headline with 3 people from the audience. If they can't repeat it back, simplify it.
- → **NEXT #601** A patient-education leaflet

600 A medical infographic for teaching or social media

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT When you want to summarise a topic visually for students, colleagues or the public.

PERSONA	You are a medical infographic designer who balances accuracy and visual simplicity.
MY CONTEXT	Topic and audience: [brief] Verified key facts or data to include: [PASTE] Format: [A4 portrait / 1:1 Instagram / 4:5 / slide] Tool: [Canva / PowerPoint / AI image tool]
TASK	1. Choose a structure: a process, a comparison, a list with icons, a statistics highlight, or a timeline. 2. Write a headline, 5–7 content blocks (under 15 words each) and a source line. 3. Suggest icons and a colour scheme. 4. Suggest the reading flow and hierarchy.
RULES	Use only the facts I provided, with sources cited on the infographic, and mark numbers [VERIFY], because infographics are shared without context.
OUTPUT	The structure, the text blocks, the design notes, then the source line.

PRO TIP One infographic, one message. If it needs a second message, make a second infographic.

TRY IT WITH "5 signs of sepsis every nurse should know", 4:5 for Instagram.

→ **NEXT #611** Educational carousel for Instagram or LinkedIn

601 A patient-education leaflet

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY / NURSE EDUCATOR

USE IT For patients to take home after a diagnosis, procedure or discharge.

PERSONA	You are a patient-education specialist who writes clear leaflets at a low reading level.
MY CONTEXT	Topic: [e.g. living with heart failure / preparing for colonoscopy] Patient group and language: [brief] Our hospital's advice and contact details (placeholders are fine): [PASTE]
TASK	1. Structure the leaflet: what it is, why it matters, what you should do, warning signs to come back for, and who to contact. 2. Write it at about a sixth-to-eighth-grade reading level, in short sentences. 3. Suggest simple pictures or icons. 4. Make a version in [LANGUAGE] if requested. 5. Keep it to one or two sides of A4.
RULES	Keep all advice consistent with our hospital's protocols, marking specifics [VERIFY WITH TREATING TEAM], because patients follow leaflets literally.
OUTPUT	The leaflet text with headings, the picture suggestions, then the translated version if requested.

PRO TIP Test the leaflet on a patient's relative. If they can't tell you the warning signs afterwards, rewrite them.

TRY IT WITH A discharge leaflet on inhaler use and COPD warning signs, in Hindi and English.

→ **NEXT #608** Pictorial medication schedule for patients

602 Pocket quick-reference card

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / MBBS INTERN

USE IT For a pocket card or phone-wallpaper reference on something you need often at the bedside.

PERSONA	You are a clinical educator who designs dense but readable quick-reference cards.
MY CONTEXT	Topic: [e.g. ABG interpretation steps / common emergency drug doses / GCS] Verified content from source: [PASTE WITH SOURCE] Size: [ID card / half A5 / phone screen]
TASK	1. Select the essential information that fits. 2. Organise it into a clear layout (sections, tables, a mnemonic). 3. Suggest a front and back design. 4. Add the source and version date in small print.
RULES	Mark every dose and threshold [VERIFY FROM SOURCE] until checked, and include "check the local protocol" wording, because pocket cards get used for years after guidelines change.
OUTPUT	The card content by side, the layout, then the source line.

PRO TIP Put the date on the card, and replace it when the guideline updates.

TRY IT WITH An emergency drug-dose card for adult cardiac arrest.

→ **NEXT #596** Clinical protocol poster for a ward or ICU

603 A concept map for a complex topic

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · MBBS STUDENT / PG RESIDENT / FACULTY

USE IT For teaching or revising topics with many connected ideas.

PERSONA	You are a medical educator who uses concept maps to show how knowledge connects.
MY CONTEXT	Topic: [TOPIC] Level: [brief] Key concepts I want included: [list or "suggest"]
TASK	1. Identify the central concept and 12–20 linked concepts. 2. Define the links with labelled arrows ("causes", "leads to", "treated by", "detected by"). 3. Organise them into clusters (e.g. mechanisms, features, tests, treatment). 4. Describe the layout for drawing by hand or in a tool (e.g. draw.io, Miro). 5. Suggest 3 questions to test understanding using the map.
RULES	Keep every link clinically accurate and mark any uncertain relationship [VERIFY], because a wrong arrow teaches a wrong mechanism.
OUTPUT	A concept list, a link table (from, label, to), the layout description, then the questions.

PRO TIP Ask students to build the map themselves first, then compare it with yours. That's where the learning happens.

TRY IT WITH Topic: heart failure (causes, mechanisms, features, investigations and drug classes).

→ **NEXT #604** Plan a chalk-and-talk or whiteboard session

604 Plan a chalk-and-talk or whiteboard session

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT For bedside teaching, tutorials or rooms without projectors.

PERSONA	You are a master chalk-and-talk teacher.
MY CONTEXT	Topic and duration: [brief] Audience and group size: [brief] Board size: [small whiteboard / large blackboard]
TASK	1. Plan the board layout in zones: title, a central diagram, key points, and a "parking lot" for questions. 2. Plan the build order: what to draw first, and when to add each element as you talk. 3. Specify the diagrams to draw, with simple step-by-step drawing instructions. 4. Suggest colour use (if markers are available). 5. Plan a final summary that the finished board shows.
RULES	Keep drawings simple enough to draw in under 2 minutes each, because long drawing loses the audience.
OUTPUT	The board-zone layout, the build sequence with timings, the drawing instructions, then the summary.

PRO TIP Practise the diagram twice beforehand. Confident drawing looks expert; hesitant drawing looks unprepared.

TRY IT WITH A 20-minute tutorial on the brachial plexus for MBBS students.

→ **NEXT #603** A concept map for a complex topic

605 Sketchnote summary of a topic

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · MBBS STUDENT / PG RESIDENT

USE IT For visual notes during or after a lecture, or as a teaching handout.

PERSONA	You are a visual-thinking coach who teaches sketchnoting to medical students.
MY CONTEXT	Topic or lecture content: [PASTE] Audience: [self / students]
TASK	1. Identify the central idea and 5–7 supporting ideas. 2. Suggest a simple icon or doodle for each idea. 3. Suggest a layout (radial, linear path or grid) with arrows and short labels. 4. Suggest one memorable example to draw. 5. If an AI image tool will be used, write a prompt with a text fallback.
RULES	Keep the labels accurate and short, and check the key facts, because sketchnotes are revised from repeatedly.
OUTPUT	The ideas, the icons, the layout, then the image prompt.

PRO TIP Simple shapes and arrows are enough. Sketchnoting isn't about drawing talent.

TRY IT WITH A sketchnote of the renin-angiotensin-aldosterone system.

→ **NEXT #603** A concept map for a complex topic

606 Educational poster for a student competition

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · MBBS STUDENT / PG RESIDENT

USE IT For poster competitions at college festivals, health days or academic events.

PERSONA	You are a judge of educational poster competitions.
MY CONTEXT	Theme and rules (size, format, judging criteria): [PASTE] My topic idea: [brief] Audience: [brief]
TASK	1. Sharpen the topic into one clear message that fits the theme. 2. Plan the poster: headline, 3–5 content blocks, visuals and a call to action. 3. Map the plan against the judging criteria (content, creativity, clarity, relevance). 4. Suggest how to present the poster to the judges in 90 seconds.
RULES	Keep all medical content accurate and cited, and follow the size and format rules exactly, because rule violations disqualify entries.
OUTPUT	The message, the poster plan, the criteria map, then the pitch.

PRO TIP Judges walk past dozens of posters. A striking headline and one strong visual decide who stops.

TRY IT WITH A theme on antimicrobial resistance for a college health festival.

→ **NEXT #600** A medical infographic for teaching or social media

607 Patient-facing poster for the OPD waiting area

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT For waiting areas, where patients have a few minutes and a real need.

PERSONA	You are a patient-communication designer for Indian hospitals.
MY CONTEXT	Message topic: [e.g. how to use an inhaler / diabetic foot care / when to come back] Patient languages and literacy: [brief] Poster size and location: [brief]
TASK	1. Choose one message with one action. 2. Write a headline (under 7 words) and 3–4 picture-supported steps. 3. Suggest pictograms for low-literacy readers. 4. Produce versions in the required languages. 5. Suggest placement (eye level, near the relevant clinic).
RULES	Keep advice consistent with our clinicians' practice, marking specifics [VERIFY WITH DEPARTMENT], and avoid frightening images, because patients should feel informed, not scared.
OUTPUT	The headline, the steps with pictograms, the language versions, then the placement.

PRO TIP Pictures of hands doing each step work better than any text for inhaler technique.

TRY IT WITH "How to use your inhaler" for a chest-clinic waiting area, in Hindi and English.

→ **NEXT #601** A patient-education leaflet

608 Pictorial medication schedule for patients

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / NURSE EDUCATOR / PHARMACIST

USE IT For patients who struggle with complex medication schedules, especially elderly or low-literacy patients.

PERSONA	You are a clinical pharmacist experienced in designing pictorial medication aids.
MY CONTEXT	The patient's medication list with timings (de-identified): [PASTE] Patient literacy and language: [brief]
TASK	1. Design a daily chart with columns for morning, afternoon, evening and night, and pictures of meal times (before or after food). 2. Represent each medicine with its colour and shape (to be filled in from the actual tablets) and its purpose in simple words. 3. Add key warnings (e.g. do not stop suddenly). 4. Add a tick box for each dose.
RULES	Copy the medication details exactly from the prescription and have the pharmacist or doctor check the chart before giving it to the patient, because a wrong chart causes medication errors.
OUTPUT	The chart layout with the content filled in, then the verification step.

PRO TIP Use actual photos of the patient's tablets if possible. Generic pill icons confuse people.

TRY IT WITH An elderly patient on 7 medicines, including insulin, with limited literacy.

→ **NEXT #601** A patient-education leaflet

609 Brief for a medical illustration

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT When you need an accurate diagram for a talk, poster or paper and will draw it or commission it.

PERSONA	You are a medical illustrator who writes precise illustration briefs.
MY CONTEXT	What the illustration must show: [brief] Where it will be used and the size: [brief] Style: [line drawing / colour / schematic] Reference sources (textbook, atlas, published figure): [LIST]
TASK	1. Describe the illustration's content precisely: structures, labels, orientation and view. 2. Specify the labels (terminology) and leader-line placement. 3. Specify the style, colours and level of detail. 4. List accuracy checkpoints for the finished illustration.
RULES	Base anatomical and pathological details on the named reference sources, and don't copy copyrighted figures directly, because accuracy and copyright both matter.
OUTPUT	The content description, the label list, the style specification, then the checklist.

PRO TIP Ask for a draft sketch before the final version. Anatomical errors are easier to fix early.

TRY IT WITH A labelled diagram of the bronchopulmonary segments for a seminar.

→ **NEXT #610** Prompt an AI image tool for medical visuals

610 Prompt an AI image tool for medical visuals

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT When generating illustrations for slides or posters with ChatGPT, Gemini or other image tools.

PERSONA	You are a prompt engineer for AI image generation in medical education.
MY CONTEXT	What I need (concept, diagram or scene): [brief] Where it will be used: [slide / poster / social post] Tool: [name]
TASK	1. Write a detailed image prompt: subject, style (flat vector, line drawing, 3D render), composition, colours, text to include (minimal) and aspect ratio. 2. Add instructions to avoid anatomical inaccuracies and to keep text minimal. 3. Suggest a two-step approach: generate a text-free image, then add labels in PowerPoint or Canva. 4. Write a checklist for reviewing the generated image.
RULES	Don't rely on AI images for precise anatomy or data, and label them "illustrative", because AI images frequently contain anatomical and text errors.
OUTPUT	The image prompt in a code block, the approach, then the review checklist.

PRO TIP Add labels yourself afterwards. AI-generated text inside images is the most error-prone part.

TRY IT WITH A friendly illustration of a doctor explaining an inhaler to an elderly patient, for a slide.

→ **NEXT #597** Comic-style teaching infographic (AI image tools)

611 Educational carousel for Instagram or LinkedIn

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT When sharing medical education on social media in a swipeable format.

PERSONA	You are a medical educator and social-media content strategist.
MY CONTEXT	Topic and audience: [e.g. residents / students / the public] Verified key points: [PASTE] Number of slides: [e.g. 7–10] Brand style: [brief]
TASK	1. Write a hook for slide 1 (under 10 words). 2. Write the slide-by-slide text: one idea per slide, under 30 words each. 3. Write the final slide: a summary and a call to action (save, share, follow). 4. Suggest visuals for each slide. 5. Write a caption with sources.
RULES	Use only verified content and cite the sources in the caption, and avoid sensational claims, because medical misinformation spreads quickly on social media.
OUTPUT	The slide-by-slide text table, the visual notes, then the caption.

PRO TIP A strong first slide decides everything. If it doesn't stop the scroll, the rest won't be seen.

TRY IT WITH "7 ECG patterns every intern must recognise", 9 slides.

→ **NEXT #600** A medical infographic for teaching or social media

612 Low-cost teaching model or demonstration

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT When a physical model would teach better than a slide.

PERSONA	You are a creative medical educator who builds low-cost teaching models.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The concept to demonstrate: [brief] Audience: [brief] Materials and budget available: [brief]
TASK	1. Suggest 2–3 model or demonstration ideas using cheap, available materials. 2. Give build instructions for the best one. 3. Write a demonstration script linking the model to the clinical concept. 4. Note the model's limitations (where the analogy breaks down).
RULES	Ensure the model is safe to handle and doesn't introduce misconceptions, and state its limitations explicitly, because models are remembered vividly, including their errors.
OUTPUT	The ideas, the build instructions, the script, then the limitations.

PRO TIP A balloon in a jar teaches lung compliance better than any animation.

TRY IT WITH Demonstrating pneumothorax and chest-drain function for nursing students.

→ **NEXT #604** Plan a chalk-and-talk or whiteboard session

613 Step-by-step procedure wall chart

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / NURSE EDUCATOR

USE IT For procedures done often on a ward: central-line care, catheterisation, nebulisation, NG-tube insertion.

PERSONA	You are a clinical skills educator who designs procedure charts.
MY CONTEXT	Procedure: [name] Our hospital's approved procedure steps: [PASTE / UPLOAD] Users: [brief]
TASK	1. Break the procedure into 8–12 numbered steps with action verbs. 2. Mark the critical safety steps (hand hygiene, verification, sterility) visually. 3. Suggest a picture or icon for each step. 4. Add a checklist box format for audits. 5. Add the source, version and review date.
RULES	Follow our approved procedure exactly and mark any step to confirm as [VERIFY WITH SOP], because procedure charts become the de facto standard on the ward.
OUTPUT	The numbered steps, the safety highlights, the picture list, then the footer.

PRO TIP Put the chart where the procedure is prepared, such as the dressing trolley or the clean utility room.

TRY IT WITH Steps for central-line dressing change in the ICU.

→ **NEXT #596** Clinical protocol poster for a ward or ICU

614 Adapt a poster into several Indian languages

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT When a poster or leaflet must reach patients who speak different languages.

PERSONA	You are a health-communication translator experienced in Indian languages.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The English poster text: [PASTE] Target languages: [list] Audience literacy: [brief]
TASK	1. Adapt (not literally translate) the text into each language, keeping it simple. 2. Keep familiar medical words (e.g. "sugar", "BP") where patients use them. 3. Check the text length fits the layout, and suggest cuts if not. 4. Recommend a review by a native speaker from the target community.
RULES	Keep the meaning exactly consistent across languages, because inconsistent translations confuse patients and staff.
OUTPUT	A text table by language, the layout notes, then the review recommendation.

PRO TIP Back-translate the key warning line into English to check it survived translation intact.

TRY IT WITH A "warning signs of dengue" poster in Hindi, Bengali and Odia.

→ **NEXT #607** Patient-facing poster for the OPD waiting area

615 Critique my poster or infographic

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT Before printing or posting any visual aid.

PERSONA	You are a senior medical educator and design critic.
MY CONTEXT	My poster or infographic text and a description (or upload the image): [PASTE / UPLOAD] Purpose and audience: [brief]
TASK	1. Check accuracy: flag any claim, number or dose to verify. 2. Check clarity: the single message, reading order and text amount. 3. Check the design: contrast, font size, hierarchy and clutter. 4. Check accessibility: language, literacy and colour-blind safety. 5. Give the top 5 fixes.
RULES	Flag doubtful facts as [VERIFY] rather than correcting them from memory, because I must confirm them from my sources.
OUTPUT	A review table by criterion, then the top 5 fixes.

PRO TIP Look at your poster from across the room for 5 seconds. What you remember is its real message.

TRY IT WITH An A3 poster on hand hygiene with 180 words and 6 colours.

→ **NEXT #600** A medical infographic for teaching or social media

616 Print specifications for posters and charts

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT Before sending posters, charts or leaflets to print.

PERSONA	You are a print-production specialist.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Item, size and quantity: [brief] Where it will be used (wall, bedside, outdoors, handout): [brief] File source: [Canva / PowerPoint / other]
TASK	1. Recommend the file format (PDF), resolution (300 dpi), colour mode and bleed. 2. Recommend the paper or material (lamination, vinyl, sunboard) for the use and durability needed. 3. Recommend minimum font sizes for the viewing distance. 4. Give a pre-print checklist (spelling, logos, contact details, version date).
RULES	Recommend wipeable, laminated materials for clinical areas, because paper posters in wards become infection-control problems.
OUTPUT	The specification table, then the checklist.

PRO TIP Print one proof copy and look at it on the actual wall before printing the full batch.

TRY IT WITH 20 laminated A3 sepsis-protocol posters for medical wards.

→ **NEXT #596** Clinical protocol poster for a ward or ICU

617 Poster with QR codes to videos and resources

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

USE IT When a poster should lead people to videos, forms or detailed resources.

PERSONA	You are a digital-health educator who connects print and online resources.
MY CONTEXT	Poster topic: [brief] Resources to link (videos, PDFs, forms): [list] Audience and their phones or data access: [brief]
TASK	1. Choose what the QR code should link to, and why. 2. Write the call to action beside the QR code. 3. Suggest QR code size and placement for scanning distance. 4. Suggest a stable link strategy (a link you control and can update). 5. Suggest how to track scans.
RULES	Link only to trustworthy, stable resources and test every code before printing, because broken or hijacked links undermine trust.
OUTPUT	The link choice, the call to action, the size and placement, the link strategy, then the tracking.

PRO TIP Use a URL you can redirect later. Printed posters outlive most links.

TRY IT WITH A poster in the physiotherapy department linking to home-exercise videos.

→ **NEXT #607** Patient-facing poster for the OPD waiting area

618 Emergency algorithm card for the crash cart

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / NURSE EDUCATOR

USE IT For emergencies where seconds matter: anaphylaxis, cardiac arrest, status epilepticus, hyperkalaemia, massive haemorrhage.

PERSONA	You are a resuscitation-training instructor who designs algorithm cards for crash carts and emergency rooms.
MY CONTEXT	Emergency: [name] Source: our hospital protocol or the current resuscitation guideline: [UPLOAD / PASTE] Drugs and equipment on our crash cart: [list] Card size: [A5 / A4, laminated]
TASK	1. Extract the time-critical steps in order, with drug, dose, route and repeat interval. 2. Design a one-page flow that can be read in under 10 seconds per step. 3. Colour-code the actions (e.g. red for immediate, amber for next, green for monitor). 4. Add a "call for help" box with placeholders for local numbers. 5. Add the source, version date and review date.
RULES	Take every dose only from the protocol I provided and mark each [VERIFY AGAINST PROTOCOL], because an error on an emergency card is repeated in every emergency until someone notices.
OUTPUT	The step list, the card layout description, the colour code, then the footer.

PRO TIP Run a mock drill with the card. Where people hesitate, the card needs simplifying.

TRY IT WITH An anaphylaxis card for the emergency department, adults.

→ **NEXT #602** Pocket quick-reference card

619 A visual mnemonic poster

STANDARD

2.5 POSTERS & VISUAL TEACHING AIDS · MBBS STUDENT / PG RESIDENT / FACULTY

USE IT When a list must be memorised and a picture could anchor it.

PERSONA	You are a medical educator skilled in visual memory techniques.
MY CONTEXT	The list to remember: [LIST] Audience: [brief] Format: [poster / slide / flashcard]
TASK	1. Create a mnemonic (acronym, phrase or story) where each element maps accurately to an item. 2. Design a visual scene or image that links the mnemonic to the items. 3. Write a caption explaining each element. 4. Suggest an image prompt or drawing instructions.
RULES	Check that every mnemonic element maps to the correct item, and keep it respectful, avoiding offensive humour, because memorable errors are the worst kind.
OUTPUT	The mnemonic, the scene description, the caption, then the image prompt.

PRO TIP Absurd, vivid images are easiest to remember, as long as they stay professional.

TRY IT WITH Causes of clubbing, as a visual poster for final-year MBBS students.

→ **NEXT #597** Comic-style teaching infographic (AI image tools)

620 Headline for my poster

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

Act as a health-communication copywriter. Write 5 headlines of under 8 words for a poster on [TOPIC] for [AUDIENCE], each stating the benefit or action rather than the topic name, accurate and free of fear-based language. Mark your favourite. Flag anything you are unsure of so I can check it.

TRY IT WITH A poster encouraging TB testing for anyone with a cough of more than 2 weeks.

621 Short slogans for a health campaign

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

Act as a public-health campaign writer. Write 6 slogans of under 7 words for [HEALTH DAY OR THEME], in English and [LANGUAGE], that are memorable, positive, non-stigmatising and linked to one clear action. Flag anything you are unsure of so I can check it.

TRY IT WITH World Diabetes Day, English and Hindi.

622 Icons for my visual aid

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT

Act as a graphic designer for health materials. For these items on my poster, [LIST ITEMS], suggest one simple, culturally appropriate icon for each, with search keywords for Canva or PowerPoint, and keep them in a single consistent style. Flag anything you are unsure of so I can check it.

TRY IT WITH Items: wash hands, wear mask, cover cough, see a doctor, take full treatment.

623 Colours for a patient-facing poster

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT

Act as a health-communication designer. Suggest a colour scheme with hex codes for a patient-facing poster on [TOPIC] that is calm, readable, high-contrast, colour-blind safe and not alarming, and say which colour to use for the call to action. Flag anything you are unsure of so I can check it.

TRY IT WITH A poster on diabetes foot care for an OPD waiting area.

624 Check the reading level of my text

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / NURSE EDUCATOR

Act as a plain-language editor. Check this patient text, [PASTE TEXT], for reading level, long sentences and medical jargon, and rewrite it at about a sixth-grade level, keeping every instruction accurate. List the words you replaced. Flag anything you are unsure of so I can check it.

TRY IT WITH Discharge instructions after a heart attack, written by a resident.

625 Pictogram instructions for low literacy

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / NURSE EDUCATOR

Act as a patient-education designer. Turn these instructions, [PASTE STEPS], into 4–6 pictogram panels, describing each picture and giving a caption of 5 words or fewer, so that patients with limited literacy can follow them. Flag anything you are unsure of so I can check it.

TRY IT WITH How to use an insulin pen.

626 Caption for my infographic post

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

Act as a medical social-media writer. Write a caption of under 80 words for my infographic on [TOPIC] for [AUDIENCE], with a hook line, a one-line summary, 2 sources, a call to save or share, and a short "educational, not medical advice" note.

TRY IT WITH An infographic on the warning signs of stroke (BE-FAST).

627 Poster size and font sizes

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT

Act as a print designer. For a [POSTER SIZE] poster viewed from [DISTANCE] metres in [LOCATION], recommend the minimum font sizes for the headline, body text and footnotes, the maximum word count, and the margins, in a short table. Flag anything you are unsure of so I can check it.

TRY IT WITH An A3 poster viewed from 2 metres in a ward corridor.

628 Fix a garbled AI image

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

Act as an AI image-prompt expert. My generated image has these problems: [DESCRIBE PROBLEMS, e.g. misspelled words, a drifting character, cropped text]. Write targeted fix-up prompts that change only the faulty parts and keep everything else identical. Flag anything you are unsure of so I can check it.

TRY IT WITH Panel 3 says "hypokalmia" and character B's face has changed.

629 Description for a visual post

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT

Act as an accessibility writer. Write alt text of under 40 words for my medical infographic or carousel image, [DESCRIBE IMAGE], so that screen-reader users get the key message, and suggest where to add it on Instagram or LinkedIn. Flag anything you are unsure of so I can check it.

TRY IT WITH A carousel slide showing the 5 steps of an ABG interpretation.

630 Call to action for an awareness poster

QUICK

2.5 POSTERS & VISUAL TEACHING AIDS · PG RESIDENT / FACULTY

Act as a public-health communicator. Write 4 calls to action for a poster on [TOPIC] for [AUDIENCE] that tell people exactly what to do, where and when, with placeholders for local facility names and helplines, in under 15 words each.

TRY IT WITH A poster on early cancer screening for women in a district town.

2.6

Case presentations & grand rounds

A good case presentation tells a clinical story in the order the evidence arrived, reasons aloud, and ends with what everyone in the room should learn.

40

prompts

5

Master

23

Standard

12

Quick

#631 Departmental case presentation, start to finish

#632 Grand rounds presentation

#633 Morbidity and mortality (M&M) presentation

#634 Short "interesting case" presentation with teaching

#635 Presenting at a multidisciplinary team meeting or tumour board

#636 A three-line case summary

#637 Organise a messy history for presentation

#638 Present examination findings clearly

#639 Justify a differential diagnosis for a case

#640 Present and justify the investigations

#641 Present the management plan and its reasoning

#642 ISBAR handover for a patient

#643 Ward-round presentation (SOAP)

#644 Systems-based ICU rounds presentation

#645 Present a case to a senior on the phone

#646 Case presentation built around images

#647 Teaching points from a case

#648 A brief literature review for my case

#649 A 10–12 slide case-presentation deck

#650 Case discussion on a management controversy

#651 Present a case with an ethical dilemma

#652 Present a rare diagnosis without losing the audience

#653 Learning from a delayed or missed diagnosis

#654 The questions the head of department will ask

#655 Root-cause analysis for an M&M case

#656 Present a follow-up update on a previous case

#657 Turn a case into a teaching session for MBBS students

#658 Link a case to a recent paper

#659 Opening one-liner for my case

#660 Summarise my case in three lines

#661 Which negatives matter?

#662 Why is this case worth presenting?

#663 Build the problem list

#664 Describe the vitals trend

#665 Present the lab results concisely

#666 Hospital course as a timeline

#667 The final diagnosis statement

#668 One learning point from my case

#669 SBAR script for a phone call

#670 A title for my case presentation

Departmental case presentation, start to finish

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / MBBS INTERN

USE IT

When you present a case at a departmental case discussion or clinical meeting.

WHY

Case presentations are where residents show clinical reasoning. A structured presentation (a crisp history, focused examination, a summary, a justified differential, a rational work-up and a clear plan) shows your thinking and invites useful discussion. A disorganised one invites interruption.

COPY THE PROMPT BELOW

PERSONA	You are a senior physician and clinical teacher who has trained generations of residents in case presentation. You value clinical reasoning over exhaustive lists, and you expect every statement to be justified.
MY CONTEXT	Specialty and my level: [e.g. MD Medicine, year 1] De-identified case notes (history, examination, investigations with results and dates as Day 0, Day 2..., treatment and course): [PASTE NOTES] Time allowed: [e.g. 15 minutes + 15 minutes discussion] Format: [bedside / slides / verbal] The discussion point my unit wants: [diagnostic dilemma / management decision / teaching]
TASK	1. Write the opening one-liner (age band, sex, key background, presenting problem and duration). 2. Structure the history: presenting complaint, history of present illness in chronological order with pertinent positives and negatives, and relevant past, drug, family, social and personal history. 3. Present the examination findings: general, vitals and systemic, relevant findings first, with pertinent negatives. 4. Write a 3–4 line summary that frames the problem. 5. Give a provisional diagnosis and a ranked differential, each with supporting and opposing features. 6. Present investigations in a logical order, with results interpreted and trends shown. 7. Present the management and course, with the reasoning behind key decisions. 8. End with the discussion points or questions for the audience, and 3 learning points.
RULES	Use only facts from my notes, keep everything de-identified, and write [NOT DOCUMENTED] where a key detail is missing rather than inventing it, because the audience will ask about it. Reason step by step when justifying the differential. This is for learning, not a substitute for the treating team's judgement.
OUTPUT	The full presentation in sections, suitable to speak or to put on 10–12 slides, then the discussion points and learning points.
TWEAK	End with the 5 questions the chairperson is most likely to ask, and short, evidence-aware answers marked [VERIFY] where guideline-dependent.

How to get more out of it

- › Present in the order the information became available. It lets the audience reason along with you.
- › Pertinent negatives are as important as positives. They show you considered the differential.
- › Rehearse the summary until you can say it without notes. It's the part everyone remembers.
- › → NEXT #636 A three-line case summary

Grand rounds presentation

2.6 CASE PRESENTATIONS & GRAND ROUNDS · SENIOR RESIDENT / FACULTY

USE IT

When you present at hospital or departmental grand rounds.

WHY

Grand rounds combine a case with a broader teaching review for a mixed, often senior audience. The best ones use the case as a doorway into current evidence, practical controversies and system lessons, and they keep time.

COPY THE PROMPT BELOW

PERSONA	You are an academic physician who has presented and chaired grand rounds for many years. You design sessions that are clinically grounded, evidence-rich and relevant to the whole hospital.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] What I most want to get out of this: [one line] The case (de-identified summary): [PASTE] The broader topic the case illustrates: [e.g. drug-induced liver injury] Audience (departments, seniority): [brief] Duration: [e.g. 45 minutes] Sources for the review part (guidelines, key papers): [UPLOAD / LIST] Guest discussants, if any: [brief]
TASK	1. Plan the structure: case presentation (10 minutes), case discussion or discussant (10 minutes), topic review (15 minutes), take-home points and system lessons (5 minutes), and Q&A. 2. Condense the case into a presentation with a clear clinical question. 3. Build the topic review from my sources: epidemiology, mechanisms relevant to the case, diagnosis, management and current evidence, with slide titles. 4. Identify system or quality lessons for the hospital, if any. 5. Write briefing questions for the discussant. 6. Write the take-home points. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Base the review on my sources and mark guideline or trial specifics [VERIFY], because grand-rounds audiences include experts in the topic. Keep the case de-identified and blame-free.
OUTPUT	The session plan with timings, the case slides, the review slide outline, the system lessons, the discussant questions, then the take-home points.
TWEAK	End with the question the most senior person in the room is most likely to ask, and how to handle it.

How to get more out of it

- › Invite a discussant from another department. It broadens the discussion and the audience.
- › Keep the review focused on what the case raised. Grand rounds aren't a textbook chapter.
- › Circulate the take-home points afterwards. They extend the reach beyond the room.
- › → **NEXT #649** A 10–12 slide case-presentation deck

Morbidity and mortality (M&M) presentation

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / FACULTY

USE IT

When presenting an adverse outcome, complication or death at an M&M or death-audit meeting.

WHY

M&M meetings exist to learn and prevent recurrence, not to blame. A structured presentation (the facts, a timeline, the analysis of contributing factors using a systems lens, the evidence, and concrete actions) makes the meeting productive and psychologically safe.

COPY THE PROMPT BELOW

PERSONA	You are a patient-safety physician who runs M&M meetings using a systems approach and a just culture. You are factual, balanced and focused on learning.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The case, de-identified, with a timeline of events: [PASTE] The outcome (complication or death): [brief] Local protocols relevant to the case: [list] Institutional M&M format, if any: [paste]
TASK	1. Present the case factually as a timeline, without judgemental language. 2. Identify the key decision points and what information was available at each one. 3. Analyse the contributing factors with a structured framework: patient factors, task and technology, individual, team, environment and organisation. 4. Classify the event (e.g. known complication, possible error, system failure, unavoidable) as a discussion point, not a verdict. 5. Summarise the relevant evidence or guideline standards, marked [VERIFY]. 6. Propose 3–5 specific, actionable system improvements with owners and timelines (as suggestions). 7. Write discussion questions for the meeting, including one on how a similar event could be detected earlier. 8. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Use neutral, non-blaming language and no names or identifiers, because psychological safety is what makes M&M work. Use only facts from my timeline, and mark unknowns [NOT KNOWN].
OUTPUT	The timeline, the decision-point table, the contributing-factor analysis, the classification, the evidence summary, the improvement proposals, then the discussion questions.
TWEAK	End with one-line "learning messages" suitable for circulating to the department.

How to get more out of it

- › Ask "what made sense to the team at that time?" rather than "who made the mistake?".
- › Track your action items. An M&M without follow-up changes nothing.
- › Keep the presentation confidential according to your institution's rules for quality-improvement meetings.
- › → **NEXT #655** Root-cause analysis for an M&M case

Short "interesting case" presentation with teaching

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

USE IT

For 10-minute "interesting case" or "case of the week" slots.

WHY

Short case slots succeed when they are tightly structured: the hook, the key findings, the reasoning to diagnosis, and one or two strong teaching points. Presenters who try to include everything run over and teach nothing.

COPY THE PROMPT BELOW

PERSONA	You are a clinical teacher who coaches residents to present interesting cases in 10 minutes with maximum teaching value.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] What I most want to get out of this: [one line] The case (de-identified notes): [PASTE] What makes it interesting: [rare diagnosis / atypical presentation / diagnostic twist / management lesson] Time: [e.g. 10 minutes] Audience: [brief] The key image or result that cracked the case: [describe] What the treating team would do differently next time: [brief]
TASK	1. Write a hook for the first 30 seconds. 2. Select only the history, examination and investigation findings that drive the reasoning. 3. Present the case as an unfolding puzzle: reveal the information in stages and pause for audience guesses. 4. Reveal the diagnosis and exactly how it was confirmed (the test, finding or response to treatment). 5. Deliver 2 teaching points with brief evidence, marked [VERIFY]. 6. Add a 30-second "what we would do differently" reflection. 7. Write 3 audience questions to use at the pauses, with the expected answers. 8. Plan 6–8 slides with timings that fit the slot exactly. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep it fully de-identified and include only facts from my notes, because interesting cases are often recognisable to colleagues.
OUTPUT	The hook, the selected findings, the staged reveal plan, the diagnosis reveal, the teaching points, then the slide plan.
TWEAK	End with one "take-home pearl" for the audience to remember.

How to get more out of it

- The pause before revealing the diagnosis is where the audience learns. Don't rush it.
- Show one key image or result, large, at the turning point.
- Finish with what you would do differently next time. It's the most useful sentence in the talk.
- → NEXT #647 Teaching points from a case

Presenting at a multidisciplinary team meeting or tumour board

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / SENIOR RESIDENT

USE IT

When presenting a patient at an MDT, tumour board, or joint medical-surgical meeting.

WHY

MDTs make major decisions in minutes. A crisp, standardised presentation with the right information for each specialty (radiology, pathology, oncology, surgery) leads to better decisions and saves everyone's time.

COPY THE PROMPT BELOW

PERSONA	You are an MDT coordinator and senior clinician who has run tumour boards and multidisciplinary meetings for years.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Meeting type: [tumour board / ILD MDT / complex-case MDT] Patient summary (de-identified): age band, sex, performance status, comorbidities: [PASTE] Relevant history, imaging, pathology and prior treatment: [PASTE] The specific question for the MDT: [e.g. surgery versus chemoradiotherapy?] Patient's preferences, if known: [brief]
TASK	1. Write a 60-second standardised presentation: the one-liner, the key history, performance status, and the question. 2. Organise the data by specialty: imaging findings for radiology, pathology findings and markers, and staging if applicable. 3. List the options to be discussed, with the relevant considerations for each. 4. Prepare a template for documenting the MDT outcome. 5. Flag missing information needed for a decision, and who should obtain it before the meeting. 6. Note any patient-specific factors (frailty, social support, finances) the MDT should weigh. 6. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	State staging and markers exactly as in the reports, marking any staging version as [VERIFY EDITION], and include the patient's preferences where known, because MDT decisions must be accurate and patient-centred.
OUTPUT	The spoken presentation, the specialty data sections, the options table, the missing-information list, then the documentation template.
TWEAK	End with the 3 questions the MDT is most likely to ask the presenter.

How to get more out of it

- › State the question for the MDT first, not last. It focuses everyone's attention.
- › Have the images and pathology slides ready to display. MDTs stall while people search for them.
- › Document the decision and the rationale before leaving the room.
- › → NEXT #636 A three-line case summary

636 A three-line case summary

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / MBBS INTERN

USE IT For the summary statement after history and examination, or at the start of rounds.

PERSONA	You are a clinical teacher who values concise, reasoning-rich summaries.
MY CONTEXT	Audience or purpose: [brief] Case details (de-identified): [PASTE] Specialty: [brief]
TASK	1. Write a 3-line summary: the one-liner (age band, sex, key background), the key clinical syndrome using semantic qualifiers (acute or chronic, unilateral or bilateral, progressive), and the key positive and negative findings. 2. Write an alternative 1-line version for handovers. 3. Explain which words carry the diagnostic reasoning.
RULES	Use only facts from the case, and don't insert a diagnosis into the summary unless it is confirmed, because summaries shape everyone's thinking.
OUTPUT	The 3-line summary, the 1-line version, then the reasoning note.

PRO TIP Semantic qualifiers ("acute, painful, unilateral") turn a summary into a diagnostic tool.

TRY IT WITH Notes on a 45-year-old man with 3 weeks of fever, weight loss and a right-sided pleural effusion.

→ **NEXT #639** Justify a differential diagnosis for a case

637 Organise a messy history for presentation

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS STUDENT / PG RESIDENT

USE IT When your clerking notes are complete but disorganised.

PERSONA	You are a clinical examiner who knows what a well-presented history sounds like.
MY CONTEXT	My clerking notes (de-identified): [PASTE] Specialty and presenting problem: [brief]
TASK	1. Reorganise the notes into the standard format: presenting complaint, HPI (chronological), past, drug, allergy, family, personal and social history, and review of systems. 2. Move the pertinent positives and negatives into the HPI. 3. Flag the gaps in the history that the audience will ask about. 4. Write the spoken version (about 2 minutes).
RULES	Use only information in my notes, and list the gaps rather than filling them in, because invented history is a serious error.
OUTPUT	The organised history, the gap list, then the spoken version.

PRO TIP For each symptom, cover onset, duration, progression and associated features. Examiners listen for them.

TRY IT WITH Notes on an elderly woman with breathlessness, leg swelling and reduced urine output.

→ **NEXT #638** Present examination findings clearly

638 Present examination findings clearly

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS STUDENT / PG RESIDENT

USE IT When presenting examination findings on rounds or in a case discussion.

PERSONA	You are a clinical examiner who values focused, organised findings.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] Examination findings (de-identified): [PASTE] Presenting problem: [brief]
TASK	1. Order the findings: general appearance, vitals, general examination, then the relevant system first and other systems briefly. 2. Highlight the findings that support or oppose the leading diagnoses. 3. Include pertinent negatives. 4. Write the spoken version (about 90 seconds).
RULES	Report the findings as documented, and don't add findings that weren't examined, because examiners test what you actually elicited.
OUTPUT	The organised findings, the highlight list, then the spoken version.

PRO TIP Say the relevant negatives aloud: "no clubbing, no lymphadenopathy". They show a targeted examination.

TRY IT WITH Findings in a patient with suspected infective endocarditis.

→ **NEXT #636** A three-line case summary

639 Justify a differential diagnosis for a case

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / MBBS STUDENT

USE IT For the differential-diagnosis part of a case presentation.

PERSONA	You are a diagnostician who teaches structured differential reasoning.
MY CONTEXT	Case summary with key findings: [PASTE] My provisional diagnosis and differentials: [list]
TASK	1. Rank the differentials by likelihood, with points for and against each from the case. 2. Identify the can't-miss diagnoses and whether they have been excluded. 3. Suggest the discriminating investigation for the top 2. 4. Write the spoken justification (about 2 minutes).
RULES	Reason step by step and use only the case facts, marking guideline-based criteria [VERIFY], because the audience will challenge each point. This supports learning, not the treating team's decisions.
OUTPUT	A differential table (diagnosis, for, against, next test), the can't-miss check, then the spoken version.

PRO TIP A short, well-justified differential beats a long, unranked list.

TRY IT WITH A young woman with fever, arthralgia, rash and pancytopenia.

→ **NEXT #640** Present and justify the investigations

640 Present and justify the investigations

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

USE IT When presenting what tests were done and why.

PERSONA	You are a senior clinician who expects every test to answer a question.
MY CONTEXT	Investigations done with results and dates: [PASTE] Differential diagnosis: [list]
TASK	1. Group the tests: confirming the diagnosis, excluding differentials, assessing severity, and baseline for treatment. 2. For each important test, state the question it answered. 3. Present the abnormal results with trends in a compact table. 4. Identify any test that wasn't needed, or one that is still pending.
RULES	Report the values with units and reference ranges as in my data, because the audience interprets the numbers.
OUTPUT	The grouped test table, the trend table, then the needed and pending notes.

PRO TIP Present trends, not single values: "creatinine 1.2 → 2.8 → 1.6 over 5 days".

TRY IT WITH Investigations in a patient with suspected tuberculous meningitis.

→ **NEXT #641** Present the management plan and its reasoning

641 Present the management plan and its reasoning

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

USE IT For the management part of a case presentation or rounds.

PERSONA	You are a senior clinician who teaches management reasoning.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Diagnosis, current status and management given: [PASTE] Guidelines or protocols followed: [list]
TASK	1. Present the management by problem (a problem-based list). 2. For each key decision, state the reasoning and the evidence or guideline behind it, marked [VERIFY]. 3. State the monitoring plan and the escalation criteria. 4. State the discharge or follow-up plan.
RULES	Describe the management as documented and don't recommend changes to real patient care, because this is a teaching presentation.
OUTPUT	The problem-based plan, the reasoning notes, the monitoring and escalation, then the follow-up.

PRO TIP A problem list (e.g. "1. Septic shock 2. AKI 3. Hyperglycaemia") keeps management presentations organised.

TRY IT WITH Management of a patient with diabetic ketoacidosis and pneumonia.

→ **NEXT #649** A 10–12 slide case-presentation deck

642 ISBAR handover for a patient

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / MBBS INTERN / NURSE

USE IT For shift handovers and inter-team transfers.

PERSONA	You are a patient-safety educator who teaches structured handover.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] Patient details (de-identified), current status and pending issues: [PASTE] Receiving team: [brief]
TASK	1. Write the handover in ISBAR format: Identify, Situation, Background, Assessment, Recommendation. 2. Highlight pending results, tasks and "watch for" items. 3. Include escalation criteria (if X, then do Y). 4. Keep it speakable in 60–90 seconds, and add a one-line code-status or goals-of-care note if relevant.
RULES	Include only verified information, and state uncertainty explicitly, because handover errors are a major cause of patient harm.
OUTPUT	The ISBAR handover, then the pending-task checklist.

PRO TIP Ask the receiving doctor to read back the key tasks. Read-back catches errors.

TRY IT WITH Night handover of a post-operative patient with falling urine output.

→ **NEXT #645** Present a case to a senior on the phone

643 Ward-round presentation (SOAP)

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS INTERN / PG RESIDENT

USE IT For presenting your patients on daily ward rounds.

PERSONA	You are a consultant who likes concise, organised ward-round presentations.
MY CONTEXT	Patient's data for today (de-identified): overnight events, symptoms, vitals, examination, new results and current treatment: [PASTE]
TASK	1. Write the presentation in SOAP format: Subjective, Objective (vitals, examination, results with trends), Assessment by problem, and Plan by problem. 2. Keep it to about 60 seconds for a stable patient and 2 minutes for a complex one. 3. Put the questions for the consultant at the end.
RULES	Present the data exactly and flag any abnormal result that needs a decision, because rounds are where decisions happen.
OUTPUT	The SOAP presentation, then the questions for the consultant.

PRO TIP Start with overnight events and anything urgent. Consultants want to know what's changed first.

TRY IT WITH Day 3 of admission for community-acquired pneumonia with new-onset atrial fibrillation.

→ **NEXT #644** Systems-based ICU rounds presentation

644 Systems-based ICU rounds presentation

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT (CRITICAL CARE, ANAESTHESIA, MEDICINE)

USE IT For presenting critically ill patients on ICU rounds.

PERSONA	You are an intensivist who expects systematic, concise ICU presentations.
MY CONTEXT	Patient's ICU data (de-identified), system by system: [PASTE] Day of ICU stay and the admitting diagnosis: [brief]
TASK	1. Present in a systems format: neurological (sedation), respiratory (ventilator settings, ABG), cardiovascular (haemodynamics, vasopressors), renal (fluid balance), GI and nutrition, haematology, infection (cultures, antibiotic day), lines and tubes, and prophylaxis. 2. Close with a problem list and today's goals. 3. Include a daily checklist (e.g. sedation hold, ventilator weaning, lines, DVT and ulcer prophylaxis, feeding).
RULES	Report numbers and settings exactly as recorded, because ICU decisions depend on precise values.
OUTPUT	The systems presentation, the problem list with goals, then the checklist.

PRO TIP Always state the antibiotic day and the culture results. It's the most frequently asked ICU rounds question.

TRY IT WITH Day 4 of septic shock from a urinary source, ventilated, on noradrenaline.

→ **NEXT #643** Ward-round presentation (SOAP)

645 Present a case to a senior on the phone

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS INTERN / PG RESIDENT

USE IT When calling a senior or another department about a patient, especially at night.

PERSONA	You are a senior resident who receives dozens of phone referrals and knows what a good one sounds like.
MY CONTEXT	Patient details (de-identified), the concern and the relevant data: [PASTE] Who I'm calling and what I need: [advice / review / transfer]
TASK	1. Write the call in SBAR: Situation (who I am, the patient, the concern), Background, Assessment (vitals and key findings, and my impression), Recommendation or Request (exactly what I need). 2. Keep it under 60 seconds. 3. List the data to have ready before dialling (vitals, recent results, current drugs, allergies).
RULES	State my specific request clearly, and include the most worrying finding early, because vague calls lead to vague responses.
OUTPUT	The SBAR script, then the "have ready" checklist.

PRO TIP Start with the request if it's urgent: "I need you to see a patient with a falling GCS now."

TRY IT WITH Calling a surgical resident about a ward patient with suspected acute abdomen.

→ **NEXT #642** ISBAR handover for a patient

646 Case presentation built around images

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

USE IT When your case hinges on imaging, ECGs, fundoscopy, skin findings or histology.

PERSONA	You are a clinician-educator who uses images to teach reasoning.
MY CONTEXT	The case summary and the key images (described or uploaded, de-identified): [PASTE / UPLOAD] Audience: [brief]
TASK	1. Place each image at the point in the story where it changed thinking. 2. For each, write "what do you see?" prompts and the systematic description. 3. Plan annotated versions with arrows and labels. 4. Link the image findings to the differential.
RULES	De-identify all images (names, dates, hospital numbers) and describe the findings conservatively, marking uncertain ones [CONFIRM WITH REPORT], because images carry hidden identifiers and over-reading misleads.
OUTPUT	An image placement table, the prompts and descriptions, the annotation plan, then the reasoning links.

PRO TIP Show the image first without comment. Let the audience look before you speak.

TRY IT WITH A case of miliary tuberculosis with serial chest X-rays and a CT.

→ **NEXT #649** A 10–12 slide case-presentation deck

647 Teaching points from a case

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / FACULTY

USE IT To close any case presentation.

PERSONA	You are a clinical educator who distills cases into memorable lessons.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Case summary and outcome: [PASTE] What went well, and what was difficult: [brief]
TASK	1. Write 3–5 teaching points, each one sentence, specific and actionable. 2. Classify them: diagnostic, management, systems or communication. 3. Link each to evidence or a guideline where relevant, marked [VERIFY]. 4. Suggest one question to test the audience on each point.
RULES	Make every point follow from this case, not general textbook facts, because case-specific lessons stick.
OUTPUT	The teaching points with classification, the evidence links, then the test questions.

PRO TIP The best teaching point is often "what I would do differently".

TRY IT WITH A case of hyponatraemia that worsened with normal saline (SIADH).

→ **NEXT #649** A 10–12 slide case-presentation deck

648 A brief literature review for my case

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

USE IT When your case discussion needs 2–3 slides on what the literature says.

PERSONA	You are an academic clinician who summarises literature quickly and accurately.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The case's key question (diagnosis, management or rarity): [brief] Sources I found (uploaded or with details): [UPLOAD / LIST]
TASK	1. Summarise what the sources say about the key question. 2. Compare the case with published cases or series, if available. 3. Summarise the current guideline position, marked [VERIFY]. 4. Write 2–3 slides with speaker notes.
RULES	Use only the sources I provided, and mark anything else [FIND SOURCE], because invented citations in case presentations are easily exposed.
OUTPUT	The summary, the comparison, the guideline note, then the slides.

PRO TIP A search limited to case reports in PubMed often finds similar cases quickly.

TRY IT WITH A case of isoniazid-induced pellagra.

→ **NEXT #647** Teaching points from a case

649 A 10–12 slide case-presentation deck

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

USE IT When your case presentation needs slides.

PERSONA	You are a presentation coach for clinical meetings.
MY CONTEXT	My structured case presentation: [PASTE] Time: [minutes] Images and tables available: [list]
TASK	1. Plan 10–12 slides: title and one-liner; history; examination; summary; differential; investigations (with a trend table); imaging; diagnosis; management and course; discussion; teaching points. 2. Write a message title and brief content for each slide. 3. Suggest where the reveals and pauses should be. 4. Check that the timing fits.
RULES	Keep the slides de-identified, with no names, dates or hospital numbers, including in image metadata, because slides get shared.
OUTPUT	The slide table (slide, title, content, visual, time), then the reveal plan.

PRO TIP Use a lab-trend table rather than pasting lab reports. It's readable and shows the trajectory.

TRY IT WITH A case of severe falciparum malaria with AKI.

→ **NEXT #631** Departmental case presentation, start to finish

650 Case discussion on a management controversy

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / FACULTY

USE IT When a case raises a genuine management dilemma worth debating.

PERSONA	You are a senior clinician who moderates balanced case-based debates.
MY CONTEXT	The case and the decision point: [PASTE] The options considered, and what was chosen: [brief] Sources for each option: [UPLOAD / LIST]
TASK	1. Frame the dilemma as a clear question. 2. Present the evidence for each option, with its strengths and weaknesses, marked [VERIFY]. 3. Consider patient factors, preferences and resources. 4. Explain what was done and why, and the outcome. 5. Write the discussion questions for the audience.
RULES	Present both sides fairly and don't judge the treating team with hindsight, because outcome bias distorts learning.
OUTPUT	The dilemma, the evidence table, the patient factors, the decision explanation, then the questions.

PRO TIP Ask the audience to vote before and after the discussion. Changed votes show learning.

TRY IT WITH Anticoagulation in a patient with a recent GI bleed and new pulmonary embolism.

→ **NEXT #648** A brief literature review for my case

651 Present a case with an ethical dilemma

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / FACULTY

USE IT For cases involving consent, capacity, end-of-life care, resource allocation or confidentiality.

PERSONA	You are a clinical ethicist who facilitates case discussions using principled frameworks.
MY CONTEXT	The case (de-identified) and the ethical question: [PASTE] Relevant Indian law or guidelines I'm aware of: [list]
TASK	1. State the ethical question clearly. 2. Analyse it using a framework (e.g. the four principles, or the four-box method: medical indications, patient preferences, quality of life, contextual features). 3. Identify the stakeholders and their perspectives. 4. Outline the options with their ethical and legal considerations, marked [VERIFY LEGAL POSITION]. 5. Write the discussion questions.
RULES	Present the perspectives respectfully and don't give legal advice, and mark legal points for verification, because ethics cases involve real legal and emotional stakes.
OUTPUT	The question, the framework analysis, the stakeholders, the options table, then the questions.

PRO TIP Invite a nurse, social worker or ethicist to the discussion. Ethics cases benefit from several perspectives.

TRY IT WITH A family requesting withdrawal of ventilation for an elderly patient without a documented advance directive.

→ **NEXT #647** Teaching points from a case

652 Present a rare diagnosis without losing the audience

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

USE IT When your case is a rare disease that most of the audience hasn't seen.

PERSONA	You are a clinical teacher who makes rare cases relevant to everyday practice.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The case (de-identified) and the rare diagnosis: [PASTE] Audience: [brief]
TASK	1. Frame the case around the common presentation that hid the rare diagnosis. 2. Explain the diagnostic clues that pointed to it. 3. Give a brief overview of the condition (5 bullets), marked [VERIFY]. 4. Draw lessons that apply to common practice (e.g. when to think beyond the common).
RULES	Keep the overview accurate and brief, because rare-disease details are easily misremembered.
OUTPUT	The framing, the clues, the overview, then the lessons.

PRO TIP "When the common diagnosis doesn't fit, look again" is the universal lesson of rare cases.

TRY IT WITH Adult-onset Still's disease presenting as fever of unknown origin.

→ **NEXT #634** Short "interesting case" presentation with teaching

653 Learning from a delayed or missed diagnosis

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / FACULTY

USE IT For teaching sessions on diagnostic error, using a de-identified case.

PERSONA	You are a diagnostic-reasoning expert who teaches cognitive bias without blame.
MY CONTEXT	The case timeline and the eventual diagnosis: [PASTE] The points where the diagnosis could have been made: [brief]
TASK	1. Map the diagnostic journey on a timeline. 2. Identify the cognitive biases likely at play (anchoring, premature closure, availability, framing) at each point. 3. Identify the system factors (workload, handover, test delays). 4. Suggest de-biasing strategies and system fixes. 5. Write the teaching points.
RULES	Frame the analysis around thinking processes and systems, not individuals, and keep it de-identified, because blame shuts down learning.
OUTPUT	The timeline, the bias table, the system factors, the strategies, then the teaching points.

PRO TIP A "diagnostic time-out" (asking "what else could this be?") is a simple, teachable de-biasing tool.

TRY IT WITH A pulmonary embolism initially treated as pneumonia for 3 days.

→ **NEXT #633** Morbidity and mortality (M&M) presentation

654 The questions the head of department will ask

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

USE IT The day before presenting a case to your unit head or at a departmental meeting.

PERSONA	You are a senior professor known for incisive case-discussion questions.
MY CONTEXT	My case presentation: [PASTE] My specialty and year: [brief]
TASK	1. List 12 likely questions: on the history, examination technique, differential, investigations, management choices, recent evidence, and "what would you do if...". 2. Give a model answer for each, grounded in the case, with [VERIFY] for guideline facts. 3. Mark the 3 hardest questions and suggest how to prepare for them.
RULES	Base questions on the actual case and my level, and admit uncertainty in the model answers where appropriate, because bluffing is easily detected.
OUTPUT	A question-and-answer table, then the preparation plan for the hardest three.

PRO TIP "I don't know, but I would find out by..." is a respectable answer. Guessing isn't.

TRY IT WITH A presentation of a young patient with a first episode of status epilepticus.

→ **NEXT #631** Departmental case presentation, start to finish

655 Root-cause analysis for an M&M case

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / FACULTY

USE IT When preparing the analysis part of an M&M or quality-improvement meeting.

PERSONA	You are a patient-safety specialist trained in root-cause analysis.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The event timeline (de-identified): [PASTE] Initial thoughts on contributing factors: [brief]
TASK	1. Build a fishbone (Ishikawa) diagram description with categories: people, processes, equipment, environment, communication and policies. 2. Apply "5 whys" to the 2 most important contributing factors. 3. Distinguish root causes from contributing factors. 4. Propose SMART corrective actions.
RULES	Focus on systems and processes, not individual blame, and base the analysis on the facts in the timeline, because RCA aims at prevention, not punishment.
OUTPUT	The fishbone description, the 5-whys chains, the root-cause list, then the actions table.

PRO TIP If your "root cause" is "human error", ask "why" at least twice more.

TRY IT WITH A delayed administration of antibiotics to a septic patient in the emergency department.

→ **NEXT #633** Morbidity and mortality (M&M) presentation

656 Present a follow-up update on a previous case

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

USE IT When the department wants to hear how a previously presented patient did.

PERSONA	You are a clinical teacher who closes the loop on cases.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] The original presentation summary: [PASTE] The follow-up course and outcome: [PASTE]
TASK	1. Remind the audience of the case in 2 lines. 2. Summarise the course since then: treatment response, complications and the current status. 3. Say whether the outcome confirms or changes the original diagnosis or plan. 4. Draw updated lessons.
RULES	Keep it de-identified and factual, and acknowledge it plainly if the original reasoning was wrong, because honest follow-up is powerful teaching.
OUTPUT	The reminder, the course summary, the diagnostic check, then the updated lessons.

PRO TIP Follow-up presentations teach humility and the natural history of disease. Do more of them.

TRY IT WITH A 6-month follow-up of a patient with autoimmune encephalitis.

→ **NEXT #647** Teaching points from a case

657 Turn a case into a teaching session for MBBS students

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / FACULTY

USE IT When teaching undergraduates using a real ward case.

PERSONA	You are a clinical tutor for MBBS students.
MY CONTEXT	The case (de-identified): [PASTE] Students' year and what they have learned so far: [brief] Time: [minutes]
TASK	1. Choose 3 learning objectives appropriate for the students' level. 2. Plan a Socratic session: questions at each stage of the case. 3. Include one clinical skill to practise (e.g. eliciting a sign) if possible. 4. Link the case to their exam topics (e.g. NEET-PG and university exams). 5. Write 5 check questions.
RULES	Keep the questions at the students' level and correct misconceptions gently, and obtain the patient's consent for bedside teaching, because respect for patients is itself a lesson.
OUTPUT	The objectives, the Socratic question plan, the skill station, the exam links, then the check questions.

PRO TIP Ask students to commit to an answer before you reveal it. Commitment drives learning.

TRY IT WITH A patient with mitral stenosis for third-year MBBS students.

→ **NEXT #647** Teaching points from a case

658 Link a case to a recent paper

STANDARD

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT / FACULTY

USE IT When a case presentation would be stronger with one recent, relevant study.

PERSONA	You are an academic clinician who connects bedside cases to current evidence.
MY CONTEXT	The case summary and its key clinical question: [PASTE] The recent paper I found (upload or paste the abstract): [UPLOAD / PASTE] Audience: [brief]
TASK	1. Summarise the paper in 3 lines (design, population, key result). 2. Explain how it applies, or doesn't apply, to this patient. 3. Note the paper's main limitation for this context. 4. Write one slide connecting the case and the paper.
RULES	Use only the paper I provided, and don't overstate how directly it applies to one patient, because case-to-trial reasoning is easy to overstretch.
OUTPUT	The summary, the applicability note, the limitation, then the slide text.

PRO TIP One well-chosen recent paper makes a case discussion feel current and evidence-based.

TRY IT WITH A patient with severe community-acquired pneumonia and a recent trial of corticosteroids.

→ **NEXT #648** A brief literature review for my case

QUICK PROMPTS · 2.6

659 Opening one-liner for my case

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS INTERN / PG RESIDENT

Act as a clinical teacher. Write the opening one-liner for my case presentation from these details, [PASTE KEY DETAILS], stating the age band, sex, the key relevant background and the presenting problem with its duration, in under 30 words and without giving away the diagnosis.

TRY IT WITH A 58-year-old diabetic woman with 5 days of fever and right upper-quadrant pain.

660 Summarise my case in three lines

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS INTERN / PG RESIDENT

Act as a senior physician. Summarise this case, [PASTE CASE NOTES], in 3 lines using semantic qualifiers (acute or chronic, unilateral or bilateral, progressive) and the key positives and negatives, without naming an unconfirmed diagnosis. Flag anything you are unsure of so I can check it.

TRY IT WITH Notes on a teenager with fever, sore throat, rash and joint pains.

661 Which negatives matter?

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS STUDENT / PG RESIDENT

Act as a clinical examiner. For a patient presenting with [PRESENTATION] and a leading diagnosis of [DIAGNOSIS], list the 6–8 pertinent negatives I should mention in history and examination, explaining in one line which differential each one argues against. Flag anything you are unsure of so I can check it.

TRY IT WITH Presentation: acute chest pain. Leading diagnosis: aortic dissection.

662 Why is this case worth presenting?

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

Act as a case-meeting coordinator. From this case summary, [PASTE SUMMARY], tell me in 3 sentences what makes the case worth presenting (a rare diagnosis, an atypical presentation, a diagnostic twist or a management lesson) and what the audience should take away.

TRY IT WITH A patient with hypokalaemic paralysis who turned out to have thyrotoxicosis.

663 Build the problem list

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS INTERN / PG RESIDENT

Act as a physician on rounds. From this case, [PASTE NOTES], create a prioritised, numbered problem list (active problems first, then chronic and background issues), with one line on the current status of each, and without adding problems not supported by the notes.

TRY IT WITH An elderly patient with pneumonia, AKI, hyponatraemia, diabetes and delirium.

664 Describe the vitals trend

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS INTERN / PG RESIDENT

Act as a clinical teacher. Describe the trend in these vital signs, [PASTE VITALS WITH TIMES], in 2–3 spoken sentences for a case presentation, highlighting clinically important changes and any that should trigger escalation. Flag anything you are unsure of so I can check it.

TRY IT WITH HR 88 → 104 → 118, BP 124/80 → 106/68 → 92/58, and RR 18 → 24 over 12 hours.

665 Present the lab results concisely

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS INTERN / PG RESIDENT

Act as a senior resident. Convert these lab results, [PASTE RESULTS WITH DATES], into a compact trend table with units, highlight the abnormal values, and write one spoken sentence summarising what the labs show overall. Flag anything you are unsure of so I can check it.

TRY IT WITH Five days of creatinine, potassium, sodium, haemoglobin and platelet counts.

666 Hospital course as a timeline

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

Act as a clinical documentation expert. Turn this hospital course, [PASTE NOTES], into a day-by-day timeline of key events, investigations, treatment changes and responses, using relative days (Day 0, Day 1...) and removing identifiers, for one slide. Flag anything you are unsure of so I can check it.

TRY IT WITH A 10-day admission for dengue with warning signs and plasma leakage.

667 The final diagnosis statement

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

Act as a senior physician. Write the complete final-diagnosis statement for this case, [PASTE DETAILS], in standard format: the primary diagnosis with its aetiology and severity, then its complications, then the comorbidities, as used in discharge summaries and case presentations.

TRY IT WITH A patient with community-acquired pneumonia complicated by septic shock and AKI, with diabetes.

668 One learning point from my case

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

Act as a clinical educator. From this case, [PASTE BRIEF CASE], write the single most valuable learning point in one sentence of under 25 words, specific and actionable, and one question to test the audience on it. Flag anything you are unsure of so I can check it.

TRY IT WITH A patient whose hypoglycaemia was caused by a sulphonylurea in renal failure.

669 SBAR script for a phone call

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · MBBS INTERN / PG RESIDENT / NURSE

Act as a patient-safety trainer. Write an SBAR phone script of under 60 seconds for calling [WHO] about this patient, [PASTE DETAILS], putting the most worrying finding early and ending with a specific request. Flag anything you are unsure of so I can check it.

TRY IT WITH Calling the ICU resident about a ward patient with a falling GCS and rising respiratory rate.

670 A title for my case presentation

QUICK

2.6 CASE PRESENTATIONS & GRAND ROUNDS · PG RESIDENT

Act as a case-meeting coordinator. Write 4 titles for my case presentation, [ONE-LINE SUMMARY], that intrigue without giving away the diagnosis, such as "Fever that wouldn't settle", each under 10 words, plus one descriptive subtitle. Flag anything you are unsure of so I can check it.

TRY IT WITH A case of infective endocarditis presenting as a stroke in a young adult.

2.7

Delivery, Q&A & teaching

Content earns attention; delivery keeps it. Handle questions with honesty, teach with methods that work, and give feedback that helps people grow.

30

prompts

4

Master

17

Standard

9

Quick

#671 Plan a small-group teaching session

#672 Structured bedside teaching session

#673 Handle any question after my talk

#674 Improve my voice, pace and presence

#675 The one-minute preceptor, applied

#676 Teach learners to present with SNAPPS

#677 Design a flipped-classroom session

#678 Write a simulation scenario

#679 Teach a clinical skill step by step

#680 Give feedback that helps

#681 Orientation teaching for new interns

#682 Engage a large lecture of 100+ students

#683 Handle difficult small-group dynamics

#684 Keep an online class engaged

#685 Check learning during teaching

#686 Build a teaching portfolio

#687 Respond to a hostile or aggressive question

#688 Answer "I don't know" with confidence

#689 Exercises for voice and pace

#690 Body language and stage presence

#691 Peer observation of teaching

#692 Rephrase an audience question

#693 Handle an off-topic question

#694 Use pauses to sound confident

#695 Fix wandering eye contact

#696 Remind myself to slow down

#697 Opening activity for a teaching session

#698 An exit-ticket question

#699 One sentence of specific feedback

#700 Praise that reinforces the right behaviour

Plan a small-group teaching session

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT

When you teach interns, students or juniors in a tutorial, skills lab or classroom.

WHY

Most clinical teaching is a mini-lecture with no plan. A structured session (clear objectives, activation of prior knowledge, active learning, practice with feedback and a quick check of learning) teaches far more in the same time, and it gives you evidence for your teaching portfolio.

COPY THE PROMPT BELOW

PERSONA	You are a medical educator trained in instructional design who designs short, active small-group sessions for clinical learners, and you are known for making every minute count.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Topic: [TOPIC] Learners: [e.g. 8 final-year MBBS students / interns / first-year residents] What they already know: [brief] Time and setting: [e.g. 45 minutes, seminar room with a whiteboard] Resources available: [projector, mannequin, cases, handouts]
TASK	1. Write 3 learning objectives with Bloom's verbs. 2. Plan a warm-up that activates prior knowledge (a question, a case or a quick quiz). 3. Plan the core content as active learning: case-based discussion, think-pair-share, or worked examples, not a lecture. 4. Plan a practice activity with feedback (applying the concept to a new case or skill). 5. Plan a closing check of learning (an exit question or mini-quiz) and a summary. 6. Build a minute-by-minute lesson plan with the teacher's actions and the learners' actions. 7. Suggest adaptations for a stronger and a weaker group. 8. Suggest how to collect feedback on the session.
RULES	Keep teacher talk to under a third of the session time, because learners retain far more from doing than from listening. Keep all clinical content accurate and mark guideline specifics [VERIFY].
OUTPUT	The objectives, a lesson-plan table (time, activity, teacher, learners, materials), the adaptations, then the feedback method.
TWEAK	End with the single question that best tests whether the session achieved its main objective.

How to get more out of it

- › Start with a question, not a slide. It shows you what the group already knows.
- › Plan less content than you think. Active learning takes more time, and teaches more.
- › Keep your lesson plans. Over a year they become a reusable teaching library.
- › → **NEXT #675** The one-minute preceptor, applied

Structured bedside teaching session

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT

When teaching at the bedside on rounds or in a dedicated teaching session.

WHY

The bedside is the richest place to teach clinical skills and reasoning, and the easiest place to embarrass a patient or bore a learner. A plan covering patient consent, learner roles, focused skills and a debrief away from the bed makes bedside teaching safe, respectful and effective.

COPY THE PROMPT BELOW

PERSONA	You are a senior clinical teacher known for respectful, high-yield bedside teaching, who uses frameworks such as SNAPPS and the one-minute preceptor.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The patient's condition and key signs (de-identified): [brief] Learners and their level: [brief] Time: [e.g. 20 minutes] Skill or concept to teach: [e.g. examining for a pleural effusion]
TASK	1. Plan the pre-briefing away from the bed: objectives, roles and etiquette. 2. Plan patient consent and involvement, and how to talk about findings in front of the patient. 3. Plan the bedside part: a focused history or examination by one learner, others observing with a checklist, and the teacher demonstrating key signs. 4. Plan questions that develop reasoning without grilling learners in front of the patient. 5. Plan the debrief away from the bed: findings, reasoning, differential and teaching points. 6. Plan how to thank and reassure the patient. 7. Plan a follow-up learning task. 8. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Obtain the patient's consent, protect their dignity and privacy, and discuss sensitive matters away from the bed, because patients' trust in the teaching hospital depends on it. Keep the learners' mistakes for the debrief, not the bedside.
OUTPUT	A session plan with phases and timings, the question list, the observation checklist, then the follow-up task.
TWEAK	End with 3 phrases to use at the bedside that teach without frightening the patient.

How to get more out of it

- › Ask the patient's permission in front of the learners. It models respect.
- › Let one learner examine while others watch for specific things. Everyone stays engaged.
- › Five minutes of focused bedside teaching beats thirty minutes of standing around.
- › → **NEXT #676** Teach learners to present with SNAPPS

Handle any question after my talk

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT

Before any talk, presentation or viva-style discussion where questions will come.

WHY

The Q&A is where credibility is won or lost. Speakers who listen fully, clarify, answer briefly, admit uncertainty honestly and handle hostile or off-topic questions calmly are remembered as experts. Preparation makes that possible.

COPY THE PROMPT BELOW

PERSONA	You are a communication coach who trains doctors to handle audience questions with confidence and honesty.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] What I most want to get out of this: [one line] My talk topic and key content: [PASTE SUMMARY] Audience and the likely tough questioners: [brief] Known weaknesses or controversies in my content: [list]
TASK	1. Teach a response framework: listen fully, pause, rephrase or clarify, answer briefly (under 60 seconds), and check that the question was answered. 2. Prepare answers to 10 likely questions, including 3 difficult ones. 3. Script responses for special situations: a question I can't answer, a hostile or aggressive question, an off-topic question, a long statement disguised as a question, and a question that exposes a real error in my talk. 4. Plan how to manage time when many people want to ask. 5. Plan how to end the Q&A gracefully, including a final sentence that restates my key message. 6. Suggest 2 backup slides that would answer the most likely difficult questions. 6. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Model honesty: admitting "I don't know" and offering to follow up, and acknowledging real errors, because audiences forgive uncertainty but not bluffing. Ground the answers in my content, marking facts to verify as [VERIFY].
OUTPUT	The framework, the Q&A table, the special-situation scripts, the time-management plan, then the closing line.
TWEAK	End with the 3 questions I most fear, and a calm 30-second answer to each.

How to get more out of it

- › Repeat or rephrase the question before answering. It helps the audience and buys you thinking time.
- › Answer the question asked, not the one you prepared.
- › A short answer followed by "does that answer your question?" is better than a long one.
- › → **NEXT #687** Respond to a hostile or aggressive question

Improve my voice, pace and presence

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT

When feedback says you speak too fast, too softly, or read your slides, or when you want to present with more authority.

WHY

Delivery determines whether good content lands. Pace, pauses, vocal variety, eye contact and movement are skills that improve quickly with specific practice and feedback, and they transfer to rounds, vivas and patient communication.

COPY THE PROMPT BELOW

PERSONA	You are a speech and presentation coach who works with clinicians and uses recordings and targeted exercises.
MY CONTEXT	My specialty and year: [e.g. MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] My delivery issues as I see them, and feedback I've received: [list] A transcript or description of a recent talk (optional): [PASTE] Upcoming talks: [brief] Time I can spend practising per week: [e.g. 2 hours]
TASK	1. Diagnose the likely causes of my issues (nerves, over-preparation of content, under-rehearsal, habits). 2. Give specific exercises for pace (target words per minute, marked pauses), voice (projection, variety), filler words, eye contact and gestures. 3. Give a practice plan over 2–3 weeks, with measurable targets. 4. Explain how to self-assess with recordings (what to watch for). 5. Give a 5-minute warm-up routine before a talk. 6. Give tips for specific settings: a large hall, a small room, online. 7. Suggest how to get useful feedback from one trusted colleague after each talk. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep the exercises practical and achievable within my time, and focus on 2–3 issues at a time, because trying to fix everything at once fixes nothing.
OUTPUT	The diagnosis, an exercises table (issue, exercise, target), the practice plan, the self-assessment guide, the warm-up, then the setting tips.
TWEAK	End with one change I can make in my very next talk that will have the biggest effect.

How to get more out of it

- › Record yourself on your phone. It's uncomfortable, and it's the fastest way to improve.
- › Mark pauses in your notes with a "/" symbol. Planned pauses slow you down naturally.
- › Speak to one person at a time for a sentence or two. It fixes wandering eye contact.
- › → NEXT #584 Learn from feedback on my talk

675 The one-minute preceptor, applied

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT When a learner presents a case to you in a busy OPD or ward.

PERSONA	You are a clinical teacher who uses the one-minute preceptor model efficiently.
MY CONTEXT	A case a learner might present to me (de-identified): [PASTE] The learner's level: [brief]
TASK	1. Walk through the 5 microskills with scripts for this case: get a commitment, probe for supporting evidence, teach a general rule, reinforce what was done well, and correct mistakes. 2. Write the likely learner responses and my follow-ups. 3. Keep the whole interaction to about 2 minutes.
RULES	Keep the teaching rule accurate and generalisable, and give the correction privately and constructively, because the learner's confidence matters in busy clinics.
OUTPUT	A 5-step script table (microskill, my words, learner's likely answer, follow-up).

PRO TIP "What do you think is going on?" is the most powerful teaching question in a busy clinic.

TRY IT WITH An intern presenting a patient with a first episode of seizure in the OPD.

→ **NEXT #676** Teach learners to present with SNAPPS

676 Teach learners to present with SNAPPS

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT When you want learners to drive their own case presentations and reasoning.

PERSONA	You are a clinical educator who uses the learner-centred SNAPPS model.
MY CONTEXT	The learner's level and a sample case (de-identified): [brief]
TASK	1. Explain the SNAPPS steps briefly: Summarise, Narrow the differential, Analyse it, Probe the preceptor, Plan management, Select a learning issue. 2. Model a SNAPPS presentation of the case. 3. Write the preceptor's responses at each step. 4. Give a pocket card for learners to use.
RULES	Keep the clinical content of the example accurate, and emphasise that probing the preceptor is encouraged, because SNAPPS works only when learners feel safe to ask.
OUTPUT	The explanation, the modelled presentation, the preceptor responses, then the pocket card.

PRO TIP The "select a learning issue" step turns every case into self-directed learning.

TRY IT WITH A second-year MBBS student presenting a child with fever and a rash.

→ **NEXT #675** The one-minute preceptor, applied

677 Design a flipped-classroom session

STANDARD

2.7 DELIVERY, Q&A & TEACHING · FACULTY / SENIOR RESIDENT

USE IT When learners can prepare in advance and you want class time for application.

PERSONA	You are a medical educator experienced in flipped and blended learning.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Topic and learners: [brief] Class time: [minutes] Resources for pre-work (videos, readings, notes): [list or "suggest"]
TASK	1. Design the pre-work: 20–30 minutes, with a short check quiz. 2. Design the class: a quick quiz review, application activities (cases, problem-solving), and a wrap-up. 3. Plan for learners who didn't do the pre-work. 4. Plan the assessment and feedback.
RULES	Keep the pre-work short and clearly linked to the class activity, because long pre-work doesn't get done.
OUTPUT	The pre-work plan, the class plan with timings, the contingency, then the assessment.

PRO TIP Start the class with a question that only makes sense if people did the pre-work.

TRY IT WITH A flipped session on ABG interpretation for interns.

→ **NEXT #671** Plan a small-group teaching session

678 Write a simulation scenario

STANDARD

2.7 DELIVERY, Q&A & TEACHING · FACULTY / SENIOR RESIDENT

USE IT For skills-lab or in-situ simulation training.

PERSONA	You are a simulation educator who writes realistic, safe scenarios with structured debriefs.
MY CONTEXT	Learning objectives: [list] Learners and their team composition: [brief] Equipment or mannequin available: [brief] Duration: [e.g. 10-minute scenario + 20-minute debrief]
TASK	1. Write the scenario: a patient brief, the initial state (vitals, findings) and the expected actions. 2. Build the progression: how vitals change with the right and wrong actions. 3. Specify the confederate roles and cues. 4. Write the debrief plan (e.g. reactions, analysis, summary) with key questions. 5. Add a checklist of critical actions.
RULES	Base the expected actions on current guidelines marked [VERIFY], and maintain psychological safety during the debrief, because simulation should build confidence, not fear.
OUTPUT	The scenario sheet, the progression table, the roles, the debrief guide, then the checklist.

PRO TIP The debrief is where the learning happens. Plan twice as much time for it as for the scenario.

TRY IT WITH Anaphylaxis in a ward patient after an IV antibiotic, for interns and nurses.

→ **NEXT #680** Give feedback that helps

679 Teach a clinical skill step by step

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT When teaching a procedure or examination skill to juniors.

PERSONA	You are a clinical skills tutor who uses Peyton's four-step approach.
MY CONTEXT	The skill: [e.g. arterial blood gas sampling, lumbar puncture, CPR] Learners and their level: [brief] Equipment: [brief]
TASK	1. Break the skill into steps with the key safety points. 2. Plan Peyton's 4 steps: demonstrate at normal speed, deconstruct (demonstrate with explanation), comprehension (the learner talks you through it while you perform), and performance (the learner performs). 3. Write the common errors and how to correct them. 4. Create a competency checklist for assessment.
RULES	Follow the standard technique and local protocols, marking any variation [CHECK LOCAL SOP], because skills are learned as they are first taught.
OUTPUT	The step list, the 4-step plan, the error list, then the checklist.

PRO TIP Step 3, where the learner explains while you do, is the step most teachers skip, and it's the most powerful.

TRY IT WITH Teaching arterial blood gas sampling to interns on a mannequin arm.

→ **NEXT #680** Give feedback that helps

680 Give feedback that helps

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT After observing a learner's presentation, procedure or patient interaction.

PERSONA	You are a medical educator trained in constructive feedback models (Pendleton, ask-tell-ask, the feedback sandwich done well).
MY CONTEXT	What I observed (behaviours, not personality): [brief] The learner's level and goals: [brief] Setting (private or public): [brief]
TASK	1. Choose the feedback model best suited to the situation. 2. Write a feedback script: start by asking for self-assessment, reinforce specific strengths, name 1–2 specific improvements with examples, and agree an action plan. 3. Suggest phrasing that is specific and behavioural. 4. Plan a follow-up.
RULES	Keep the feedback specific, behavioural and private, focus on 1–2 improvements, and avoid vague praise or criticism, because vague or public feedback doesn't change behaviour.
OUTPUT	The chosen model, the script, the phrasing examples, then the follow-up plan.

PRO TIP "Your summary was clear because you used semantic qualifiers" teaches. "Good job" doesn't.

TRY IT WITH Feedback to an intern whose case presentation was disorganised but whose examination was thorough.

→ **NEXT #675** The one-minute preceptor, applied

681 Orientation teaching for new interns

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT When a new batch of interns joins your department.

PERSONA	You are a department educator who designs practical orientation sessions.
MY CONTEXT	Department and the interns' roles: [brief] Time available: [e.g. 2 hours] Common problems new interns face here: [list]
TASK	1. Plan the session: introductions, roles and expectations, key protocols, documentation standards, escalation pathways and common pitfalls. 2. Include hands-on elements (e.g. writing a note, an SBAR call). 3. Create a one-page survival guide. 4. Plan a check-in after one week.
RULES	Keep protocols consistent with the department's approved versions, marking them [VERIFY WITH DEPARTMENT], and emphasise when to call for help, because new interns are most at risk of not escalating.
OUTPUT	The session plan, the hands-on activities, the survival guide, then the check-in plan.

PRO TIP "When in doubt, call" should be said more than once. New interns worry about disturbing seniors.

TRY IT WITH Orientation for 20 interns joining the medicine department.

→ **NEXT #671** Plan a small-group teaching session

682 Engage a large lecture of 100+ students

STANDARD

2.7 DELIVERY, Q&A & TEACHING · FACULTY / SENIOR RESIDENT

USE IT When you must lecture to a large MBBS batch.

PERSONA	You are an award-winning lecturer who keeps large classes engaged.
MY CONTEXT	Topic and duration: [brief] Batch size and year: [brief] Technology available: [projector, microphone, poll app, none]
TASK	1. Chunk the lecture into 10–15 minute segments, with an activity between segments. 2. Suggest activities that work with large groups: polls, think-pair-share, a show of hands, one-minute papers. 3. Plan a clinical hook for each segment. 4. Plan the end-of-lecture summary and a check of learning.
RULES	Keep each segment focused on one objective, because attention drops sharply after about 15 minutes of continuous lecturing.
OUTPUT	A segment plan with activities and timings, then the summary and check.

PRO TIP A 2-minute "turn to your neighbour and explain..." resets attention in any size of room.

TRY IT WITH A 60-minute lecture on the pharmacology of antihypertensives for 150 second-year students.

→ **NEXT #671** Plan a small-group teaching session

683 Handle difficult small-group dynamics

STANDARD

2.7 DELIVERY, Q&A & TEACHING · FACULTY / SENIOR RESIDENT

USE IT When a tutorial group has a dominant talker, a silent group or disengaged learners.

PERSONA	You are an experienced small-group facilitator.
MY CONTEXT	The group problem: [e.g. one learner dominates; others are silent; phones out] Group size and level: [brief]
TASK	1. Diagnose the likely causes. 2. Suggest facilitation techniques: structured turn-taking, directed questions, pair work, roles and ground rules. 3. Script how to redirect a dominant learner respectfully. 4. Script how to draw in quiet learners without putting them on the spot. 5. Suggest how to set ground rules at the next session.
RULES	Keep the strategies respectful of every learner, because humiliation damages the whole group's learning.
OUTPUT	The diagnosis, the techniques, the scripts, then the ground rules.

PRO TIP "Let's hear from someone who hasn't spoken yet" works better than naming the quiet person.

TRY IT WITH A group of 10 residents where one senior answers every question.

→ **NEXT #671** Plan a small-group teaching session

684 Keep an online class engaged

STANDARD

2.7 DELIVERY, Q&A & TEACHING · FACULTY / SENIOR RESIDENT

USE IT When teaching over Zoom, Teams or Google Meet.

PERSONA	You are an online medical educator who keeps virtual learners active.
MY CONTEXT	Topic, duration and number of learners: [brief] Platform features available: [polls, breakout rooms, chat, whiteboard]
TASK	1. Plan interaction every 5–10 minutes: polls, chat questions, breakout cases and annotation. 2. Plan the camera and presence strategy (encouraging cameras, using names). 3. Plan breakout-room activities with clear instructions. 4. Plan a check of learning at the end. 5. Plan for technical problems.
RULES	Keep instructions for activities very clear and posted in the chat, because online learners can't ask neighbours what to do.
OUTPUT	An interaction plan with timings, the breakout design, the check, then the tech backup.

PRO TIP Open with a chat question everyone can answer in 10 seconds. It gets people participating from minute one.

TRY IT WITH A 60-minute online tutorial on ECG basics for 40 MBBS students.

→ **NEXT #546** Slides for online teaching and webinars

685 Check learning during teaching

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT To know whether learners are following, before the end of the session.

PERSONA	You are an assessment-for-learning specialist.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Topic and objectives: [PASTE] Session format and group size: [brief]
TASK	1. Suggest 5 quick formative checks: hinge questions, a show of fingers (confidence 1–5), mini-whiteboards, polls and one-minute papers. 2. Write 3 hinge questions for my topic, with the misconception each wrong option reveals. 3. Explain how to adapt the teaching based on the responses.
RULES	Design the questions so that the wrong answers reveal specific misconceptions, because that is what makes a check useful.
OUTPUT	The checks list, the hinge questions with their distractor analysis, then the adaptation guide.

PRO TIP If more than a third of the group gets a hinge question wrong, re-teach before moving on.

TRY IT WITH Teaching acid-base disorders to interns.

→ **NEXT #698** An exit-ticket question

686 Build a teaching portfolio

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT For academic promotion, job applications, or to track your growth as a teacher.

PERSONA	You are an academic development mentor for clinician-educators.
MY CONTEXT	My teaching activities so far (seminars, lectures, bedside, simulation, mentoring): [list] Feedback or evaluations I have: [brief] Purpose of the portfolio: [promotion / application / personal growth]
TASK	1. Suggest the portfolio structure: teaching philosophy, activities log, evidence of effectiveness (feedback, learner outcomes), materials developed, and reflections. 2. Draft a 250-word teaching philosophy from my activities. 3. Create a teaching-activity log template. 4. Suggest how to collect better evidence going forward.
RULES	Include only activities and feedback I actually have, because portfolios are verified during academic review.
OUTPUT	The structure, the philosophy draft, the log template, then the evidence plan.

PRO TIP Save feedback forms and a copy of every teaching material. Evidence is the hardest part to reconstruct.

TRY IT WITH A second-year resident with 10 seminars, weekly intern teaching and one simulation session.

→ **NEXT #691** Peer observation of teaching

687 Respond to a hostile or aggressive question

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT When preparing for a talk where someone may challenge you aggressively.

PERSONA	You are a communication coach experienced in high-pressure academic settings.
MY CONTEXT	The likely hostile question or challenge: [paste] The relevant facts from my work: [brief]
TASK	1. Separate the emotion from the content of the question. 2. Write a calm response: acknowledge, clarify the substance, answer with evidence, and offer to continue afterwards. 3. Write what not to say. 4. Plan how to handle persistence (a second challenge).
RULES	Keep the response respectful and factual, and concede valid points, because the audience judges how you respond more than who is right.
OUTPUT	The analysis, the response script, the avoid list, then the persistence plan.

PRO TIP "That's a fair challenge" defuses most hostility, and it costs nothing.

TRY IT WITH "Your sample is tiny and your conclusions are meaningless. Why present this at all?"

→ **NEXT #673** Handle any question after my talk

688 Answer "I don't know" with confidence

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / MBBS STUDENT

USE IT When preparing for Q&A, rounds or vivas where you may not know an answer.

PERSONA	You are a senior clinician-teacher who models intellectual honesty.
MY CONTEXT	The setting (talk, rounds, viva, patient conversation): [brief] Examples of questions I might not know: [list]
TASK	1. Write 4 honest responses for not knowing, adapted to each setting. 2. Show how to reason aloud from first principles when partly unsure. 3. Show how to offer to follow up, and actually do it. 4. Explain when a guess is acceptable and how to label it.
RULES	Never recommend bluffing, especially about patient care, because a confident wrong answer is dangerous in medicine.
OUTPUT	The responses by setting, a reasoning-aloud example, then the follow-up guidance.

PRO TIP "I'm not sure. Let me reason it through..." often earns more respect than a quick right answer.

TRY IT WITH A viva question about a rare drug interaction.

→ **NEXT #673** Handle any question after my talk

689 Exercises for voice and pace

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / MBBS STUDENT

USE IT When you speak too fast, too softly or in a monotone.

PERSONA	You are a voice coach for professional speakers.
MY CONTEXT	My main voice issue: [fast / soft / monotone / accent clarity] Time available to practise daily: [minutes]
TASK	1. Give 5 daily exercises (breathing, projection, articulation, pausing and vocal variety). 2. Set a target pace (e.g. 130–150 words per minute) and explain how to measure it. 3. Explain how to mark scripts for pauses and emphasis. 4. Give a 2-week practice plan.
RULES	Keep the exercises safe for the voice (no strain) and practical in a small space, because busy residents practise in hostels and call rooms.
OUTPUT	The exercises table, the pace method, the script-marking guide, then the plan.

PRO TIP Record a 1-minute reading daily and time it. Pace problems become obvious and fixable.

TRY IT WITH A resident who speaks at over 180 words per minute when nervous.

→ **NEXT #674** Improve my voice, pace and presence

690 Body language and stage presence

STANDARD

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

USE IT When you want to look more confident while presenting.

PERSONA	You are a presentation coach specialising in non-verbal communication.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] My habits (e.g. pacing, hands in pockets, fidgeting): [list] Venue: [podium / open stage / small room / online]
TASK	1. Explain the effect of each habit on the audience. 2. Give a replacement behaviour for each. 3. Give guidance on stance, gestures, movement with purpose and using the pointer. 4. Give online-specific guidance (camera framing, eye contact with the lens).
RULES	Keep the advice natural rather than theatrical, because forced body language looks less confident than none.
OUTPUT	A habit-to-fix table, the general guidance, then the online tips.

PRO TIP Face the audience, not the screen. Glance at your laptop instead of turning to the projector.

TRY IT WITH A presenter who reads from the screen with their back to the audience.

→ **NEXT #674** Improve my voice, pace and presence

691 Peer observation of teaching

STANDARD

2.7 DELIVERY, Q&A & TEACHING · FACULTY / SENIOR RESIDENT

USE IT When a colleague observes your teaching, or you observe theirs.

PERSONA	You are a faculty-development specialist who runs peer-observation programmes.
MY CONTEXT	My specialty and level: [e.g. MD Medicine, year 2] Audience or purpose: [brief] The session type and the teacher's goals for feedback: [brief]
TASK	1. Create an observation form: objectives clarity, structure, engagement, questioning, use of time, clinical accuracy and learner participation. 2. Suggest how to agree the focus before the session. 3. Suggest how to record specific observations (time-stamped, behavioural). 4. Plan a supportive post-observation conversation.
RULES	Focus on observable behaviours and the teacher's chosen goals, because peer observation fails when it becomes judgemental.
OUTPUT	The observation form, the pre-meeting guide, then the conversation plan.

PRO TIP Observe for what the teacher asked you to look at. Unrequested critique rarely helps.

TRY IT WITH Peer observation of a resident's 30-minute bedside teaching session.

→ **NEXT #680** Give feedback that helps

QUICK PROMPTS · 2.7

692 Rephrase an audience question

QUICK

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT

Act as a speaking coach. Show me how to rephrase this audience question, [PASTE QUESTION], in one neutral sentence that confirms I understood it, buys thinking time and helps the rest of the audience hear it, before I answer. Flag anything you are unsure of so I can check it.

TRY IT WITH "Isn't this whole approach outdated now that the new guidelines have come out?"

693 Handle an off-topic question

QUICK

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

Act as a session chair. Write a polite 2-sentence response to this off-topic question, [PASTE QUESTION], after my talk on [TOPIC], that acknowledges it, bridges back to the talk if possible, and offers to discuss it afterwards. Flag anything you are unsure of so I can check it.

TRY IT WITH A question about hospital salaries after a talk on sepsis management.

694 Use pauses to sound confident

QUICK

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / MBBS STUDENT

Act as a speaking coach. Mark where to pause in this script, [PASTE SCRIPT], using "/" for a short pause and "//" for a long pause, before key numbers, after questions and before the conclusion, and explain in 2 lines why each long pause is there.

TRY IT WITH The opening and closing paragraphs of a seminar on stroke.

695 Fix wandering eye contact

QUICK

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT

Act as a presentation coach. Give me 5 practical techniques for better eye contact when presenting to [AUDIENCE SIZE] people in a [ROOM TYPE], including what to do when using notes and slides, and one exercise to practise before my next talk.

TRY IT WITH A 40-seat seminar room with the slides projected behind me.

696 Remind myself to slow down

QUICK

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / MBBS STUDENT

Act as a speaking coach. I speak too fast when nervous. Suggest 5 cues I can build into my slides, notes and routine for a [DURATION]-minute talk that will naturally slow me down, such as breathing points and planned audience questions.

TRY IT WITH A 15-minute case presentation.

697 Opening activity for a teaching session

QUICK

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

Act as a medical educator. Suggest 3 quick opening activities (under 3 minutes each) for a teaching session on [TOPIC] with [LEARNERS] that activate prior knowledge and show me what they already know. Flag anything you are unsure of so I can check it.

TRY IT WITH A tutorial on heart failure for final-year MBBS students.

698 An exit-ticket question

QUICK

2.7 DELIVERY, Q&A & TEACHING · PG RESIDENT / FACULTY

Act as an assessment-for-learning specialist. Write 3 exit-ticket questions for the end of my session on [TOPIC], each answerable in 1–2 minutes, that test the main objective [OBJECTIVE] and reveal the most common misconception. Flag anything you are unsure of so I can check it.

TRY IT WITH A session on interpreting chest X-rays for interns.

699 One sentence of specific feedback

QUICK

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Act as a clinical teacher. Turn this vague feedback, [VAGUE FEEDBACK], into one specific, behavioural sentence that names what the learner did, its effect, and exactly what to do next time, in a supportive tone. Flag anything you are unsure of so I can check it.

TRY IT WITH "Your presentation was a bit confusing."

700 Praise that reinforces the right behaviour

QUICK

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Act as a medical educator. Rewrite this generic praise, [GENERIC PRAISE], into specific praise for [LEARNER ACTION] that names the behaviour and why it mattered for the patient or the team, in one or two sentences. Flag anything you are unsure of so I can check it.

TRY IT WITH "Good job with that patient" after an intern calmly explained a diagnosis to a family.

Every prompt at a glance.

Q = Quick · S = Standard · M = Master

2.1 · SEMINAR PREPARATION

#401	Mixed-audience seminar deck, clinically rigorous	M
#402	From topic to seminar blueprint in one sitting	M
#403	Evidence pack: guidelines and trials, verified from my sources	M
#404	Full speaker script for a 45-minute seminar	M
#405	Case-based interactive seminar	M
#406	Mock moderator: grill me before the real seminar	M
#407	Turn a new guideline into a seminar	M
#408	Learning objectives with Bloom's verbs	S
#409	Minute-by-minute time plan	S
#410	Narrow a huge seminar topic to something teachable	S
#411	Pitch the same topic to different audiences	S
#412	Pathophysiology as a visual story	S
#413	Turn a classification into a clear table	S
#414	Diagnostic approach as an algorithm	S
#415	Management section, stepwise and practical	S
#416	Drug comparison table for a seminar	S
#417	Landmark trials in one table	S
#418	Controversies and grey zones	S
#419	Recent advances slide set	S
#420	The Indian perspective slide	S
#421	Compare two or more guidelines	S
#422	References slide and citation check	S
#423	End-of-seminar quiz	S
#424	Take-home messages that stick	S
#425	One-page handout for the audience	S
#426	Opening clinical vignette	S
#427	Basic science seminar with clinical hooks	S
#428	Seminar on a procedure or surgery	S
#429	Teaching session on interpreting a test	S
#430	Image-based seminar	S
#431	Turn a textbook chapter into a seminar	S
#432	Prepare a seminar with NotebookLM	S
#433	What have I missed?	S
#434	Feedback on my rehearsal script	S
#435	Make my seminar interactive	S
#436	Check the story and flow of my outline	S
#437	Adapt my seminar for nurses and allied staff	S
#438	Turn a seminar into a CME talk for practitioners	S
#439	Pro-con debate seminar	S
#440	Review my seminar slides like a moderator	S
#441	Build a seminar from our department's cases	S
#442	Collect feedback on my seminar	S
#443	A seminar title that's accurate and interesting	Q
#444	A one-line opening hook	Q
#445	Five learning objectives in a minute	Q

#446	The definition slide	Q
#447	A useful mnemonic for my seminar	Q
#448	"Why this matters" slide	Q
#449	How many slides do I need?	Q
#450	Key numbers slide	Q
#451	Summary slide from my outline	Q
#452	Transition lines between sections	Q
#453	A question to wake up the audience	Q
#454	An analogy for a hard concept	Q
#455	Three questions for the discussion	Q
#456	A short history slide	Q
#457	Abbreviations slide for my seminar	Q
#458	Future directions slide	Q
#459	What moderators always ask on this topic	Q
#460	Turn my paragraph into slide bullets	Q

2.2 · JOURNAL CLUB

#461	Journal club presentation: appraisal to bottom line	M
#462	Choose the right paper for journal club	M
#463	Deep appraisal of a randomised controlled trial	M
#464	Appraise a cohort or case-control study	M
#465	Appraise a diagnostic accuracy study	M
#466	Appraise a systematic review or meta-analysis	M
#467	Decode the statistics in my paper	M
#468	Five-line PICO summary of the paper	S
#469	Does the abstract match the paper?	S
#470	"Why was this study done?" slide	S
#471	Methods on two slides	S
#472	Key results, clearly presented	S
#473	Explain a forest plot to the audience	S
#474	Explain the survival curves	S
#475	Absolute effects: ARR, RRR, NNT and NNH	S
#476	Can we trust these results?	S
#477	Does this apply to our patients?	S
#478	Hunt for the hidden bias	S
#479	Confounding in an observational paper	S
#480	Are these the right outcomes?	S
#481	Appraise a non-inferiority trial	S
#482	Should we believe the subgroup finding?	S
#483	Detect spin in the discussion and conclusion	S
#484	Funding and conflicts: do they matter here?	S
#485	Compare the paper with its editorial and letters	S
#486	How does this fit with previous evidence?	S
#487	The clinical bottom line	S
#488	Questions to drive the discussion	S
#489	The faculty questions I'll face	S
#490	One-page appraisal handout	S
#491	Fill the CASP checklist for my paper	S

#492	Certainty of evidence using GRADE	S	#542	Choose the right chart for a slide	S
#493	Appraise a qualitative study	S	#543	Citing sources on slides	S
#494	Appraise an economic evaluation	S	#544	Title and opening slide design	S
#495	Appraise a prognostic or prediction-model study	S	#545	Section divider slides that guide the audience	S
#496	Appraise a clinical guideline (AGREE II)	S	#546	Slides for online teaching and webinars	S
#497	Check the trial registry for outcome switching	S	#547	Using images and figures legally	S
#498	Read a paper properly in 30 minutes	S	#548	Turn a slide title into a message	Q
#499	Explain the paper to MBBS students	S	#549	Check a slide for clutter	Q
#500	Quiz questions from the journal club	S	#550	Paragraph to one clean slide	Q
#501	Turn my critique into a letter to the editor	S	#551	A three-colour palette for my deck	Q
#502	Moderating a journal club well	S	#552	Font sizes for my room	Q
#503	This study's design in one line	Q	#553	Icon ideas for my concepts	Q
#504	Find the real primary outcome	Q	#554	Alt text for my slide images	Q
#505	Quick ARR and NNT	Q	#555	A caption for a figure on a slide	Q
#506	What does this hazard ratio mean?	Q	#556	Should these bullets be a diagram?	Q
#507	Intention-to-treat or per-protocol?	Q	#557	Design my final slide	Q
#508	Was allocation really concealed?	Q	#558	QR code slide content	Q
#509	Who was blinded, and does it matter?	Q	#559	Should this table be a chart?	Q
#510	Is this loss to follow-up a problem?	Q	#560	Does my slide pass the 6 × 6 rule?	Q
#511	The whole paper in one sentence	Q	2.4 · TALKS & ORAL PRESENTATIONS		
#512	Was this study adequately powered?	Q	#561	Invited CME talk or guest lecture, start to finish	M
#513	Is this outcome a surrogate?	Q	#562	Build any talk around a story	M
#514	Would this change my practice?	Q	#563	Health talk for patients or the community	M
#515	Explain a methods term from my paper	Q	#564	Prepare for a panel discussion (as panellist or moderator)	M
#516	Does the title overclaim?	Q	#565	The first 90 seconds of my talk	S
#517	Relative versus absolute risk for the audience	Q	#566	A closing the audience remembers	S
#518	Explain the key figure in three lines	Q	#567	A 5-minute lightning talk	S
#519	A sharp question to ask the presenter	Q	#568	A PechaKucha-style talk (20 slides × 20 seconds)	S
#520	Summarise the paper for colleagues	Q	#569	Motivational or career talk for students	S
2.3 · SLIDE DESIGN & STRUCTURE			#570	Deliver a talk in Hindi or a regional language	S
#521	Redesign my existing deck, slide by slide	M	#571	Using patient stories ethically in talks	S
#522	A personal slide design system for all my talks	M	#572	Health talk for school students	S
#523	Build a deck with AI tools, safely	M	#573	Chairing a scientific session	S
#524	Turn data and tables into clear slide visuals	M	#574	Introduce a guest speaker	S
#525	Rewrite slide titles as messages	S	#575	A vote of thanks that isn't boring	S
#526	Turn a text-heavy slide into a visual	S	#576	Host a medical webinar	S
#527	Simplify a table for a slide	S	#577	Script for a short educational video	S
#528	Build a flowchart slide	S	#578	Rehearsal plan for an important talk	S
#529	Adapt a guideline algorithm for a slide	S	#579	Manage stage fright before a talk	S
#530	Two conditions side by side	S	#580	Shrink or stretch a talk to a new time slot	S
#531	A timeline slide	S	#581	Turn a key paper into a teaching talk	S
#532	Clinical image slide with annotation	S	#582	Proposal and abstract for an invited talk	S
#533	Icons and simple graphics for concepts	S	#583	A short speaker bio	S
#534	Colours that work for everyone	S	#584	Learn from feedback on my talk	S
#535	Fonts and sizes that are readable	S	#585	An opening question that hooks the audience	Q
#536	When to use animation, and when not to	S	#586	Organise my talk into three parts	Q
#537	Cut my deck to fit the time	S	#587	My talk's big idea in one sentence	Q
#538	Write speaker notes for my slides	S	#588	Five questions about my audience	Q
#539	Separate the slides from the handout	S	#589	A safe icebreaker for a medical audience	Q
#540	A design brief for building slides in Canva	S	#590	Tell a story in 60 seconds	Q
#541	A prompt for AI slide generators	S	#591	Recap before moving on	Q

#592	A clear call to action	Q	#640	Present and justify the investigations	S
#593	A title for my invited talk	Q	#641	Present the management plan and its reasoning	S
#594	Clean filler words out of my script	Q	#642	ISBAR handover for a patient	S
#595	A better way to open the Q&A	Q	#643	Ward-round presentation (SOAP)	S
2.5 · POSTERS & VISUAL TEACHING AIDS					
#596	Clinical protocol poster for a ward or ICU	M	#644	Systems-based ICU rounds presentation	S
#597	Comic-style teaching infographic (AI image tools)	M	#645	Present a case to a senior on the phone	S
#598	Handwritten-style revision notes image	M	#646	Case presentation built around images	S
#599	Awareness poster series for a health day	M	#647	Teaching points from a case	S
#600	A medical infographic for teaching or social media	S	#648	A brief literature review for my case	S
#601	A patient-education leaflet	S	#649	A 10–12 slide case-presentation deck	S
#602	Pocket quick-reference card	S	#650	Case discussion on a management controversy	S
#603	A concept map for a complex topic	S	#651	Present a case with an ethical dilemma	S
#604	Plan a chalk-and-talk or whiteboard session	S	#652	Present a rare diagnosis without losing the audience	S
#605	Sketchnote summary of a topic	S	#653	Learning from a delayed or missed diagnosis	S
#606	Educational poster for a student competition	S	#654	The questions the head of department will ask	S
#607	Patient-facing poster for the OPD waiting area	S	#655	Root-cause analysis for an M&M case	S
#608	Pictorial medication schedule for patients	S	#656	Present a follow-up update on a previous case	S
#609	Brief for a medical illustration	S	#657	Turn a case into a teaching session for MBBS students	S
#610	Prompt an AI image tool for medical visuals	S	#658	Link a case to a recent paper	S
#611	Educational carousel for Instagram or LinkedIn	S	#659	Opening one-liner for my case	Q
#612	Low-cost teaching model or demonstration	S	#660	Summarise my case in three lines	Q
#613	Step-by-step procedure wall chart	S	#661	Which negatives matter?	Q
#614	Adapt a poster into several Indian languages	S	#662	Why is this case worth presenting?	Q
#615	Critique my poster or infographic	S	#663	Build the problem list	Q
#616	Print specifications for posters and charts	S	#664	Describe the vitals trend	Q
#617	Poster with QR codes to videos and resources	S	#665	Present the lab results concisely	Q
#618	Emergency algorithm card for the crash cart	S	#666	Hospital course as a timeline	Q
#619	A visual mnemonic poster	S	#667	The final diagnosis statement	Q
#620	Headline for my poster	Q	#668	One learning point from my case	Q
#621	Short slogans for a health campaign	Q	#669	SBAR script for a phone call	Q
#622	Icons for my visual aid	Q	#670	A title for my case presentation	Q
#623	Colours for a patient-facing poster	Q	2.7 · DELIVERY, Q&A & TEACHING		
#624	Check the reading level of my text	Q	#671	Plan a small-group teaching session	M
#625	Pictogram instructions for low literacy	Q	#672	Structured bedside teaching session	M
#626	Caption for my infographic post	Q	#673	Handle any question after my talk	M
#627	Poster size and font sizes	Q	#674	Improve my voice, pace and presence	M
#628	Fix a garbled AI image	Q	#675	The one-minute preceptor, applied	S
#629	Description for a visual post	Q	#676	Teach learners to present with SNAPPS	S
#630	Call to action for an awareness poster	Q	#677	Design a flipped-classroom session	S
2.6 · CASE PRESENTATIONS & GRAND ROUNDS					
#631	Departmental case presentation, start to finish	M	#678	Write a simulation scenario	S
#632	Grand rounds presentation	M	#679	Teach a clinical skill step by step	S
#633	Morbidity and mortality (M&M) presentation	M	#680	Give feedback that helps	S
#634	Short "interesting case" presentation with teaching	M	#681	Orientation teaching for new interns	S
#635	Presenting at a multidisciplinary team meeting or tumour board	M	#682	Engage a large lecture of 100+ students	S
#636	A three-line case summary	S	#683	Handle difficult small-group dynamics	S
#637	Organise a messy history for presentation	S	#684	Keep an online class engaged	S
#638	Present examination findings clearly	S	#685	Check learning during teaching	S
#639	Justify a differential diagnosis for a case	S	#686	Build a teaching portfolio	S
			#687	Respond to a hostile or aggressive question	S
			#688	Answer "I don't know" with confidence	S
			#689	Exercises for voice and pace	S

#690	Body language and stage presence	S
#691	Peer observation of teaching	S
#692	Rephrase an audience question	Q
#693	Handle an off-topic question	Q
#694	Use pauses to sound confident	Q
#695	Fix wandering eye contact	Q

#696	Remind myself to slow down	Q
#697	Opening activity for a teaching session	Q
#698	An exit-ticket question	Q
#699	One sentence of specific feedback	Q
#700	Praise that reinforces the right behaviour	Q

Which AI tool for which job.

JOB	BEST TOOL	WHY
Studying sources for a seminar	NotebookLM, Claude, ChatGPT (upload)	Answers and audio overviews grounded in your PDFs
Current evidence and recent trials	Perplexity (Academic), Consensus	Live search with clickable citations
Outlines, scripts and speaker notes	Claude, ChatGPT, Gemini	Strong structure and plain-language rewriting
Slide decks	PowerPoint (Copilot), Gamma, Canva	Fast first drafts from your verified outline
Posters, infographics and leaflets	Canva, ChatGPT images, Gemini images	Layouts and illustrations; proofread every word
Polls and live quizzes	Mentimeter, Slido, Google Forms	Turn a lecture into an interactive session
Rehearsal feedback	Phone recording + AI transcript review	See your pace, filler words and timing

TEACHING WITH AI, RESPONSIBLY

- › Verify every drug dose, guideline year and trial detail before it goes on a slide.
- › Never put identifiable patient data or images into AI tools or slides.
- › Credit figures you adapt, and use only images you have the right to use.
- › AI drafts the deck. You own the content, the delivery and the answers.

One last thing.

These prompts will not make you a great teacher. They will give you more time to become one: time to rehearse, to listen to your audience and to answer the question behind the question. The best presenters use AI to prepare and themselves to connect.

Prepare with AI. Teach with judgement. Leave the room better informed.

More AI in medicine every week: Instagram [@abhishekjbenur](#) · [paraboxai.com](#)

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